

Long-Term Care 8-Hour Course



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Long-Term Care Insurance

8-Hour Course

Course Description

As a significant portion of the U.S. population ages, the topic of long-term care is an increasingly hot topic being addressed by more and more individuals and families. This course takes an in-depth look at long-term care (LTC) and long-term insurance, ranging from a basic understanding of LTC to the complications that may arise in the absence of a policy. The advantages of policies and their features are weighed, and the differences between the policies offered by various institutions are discussed. It must be kept in mind that policy coverage, rules and regulations and conditions vary from company to company and state to state.

Learning Objectives

Upon completion of the course, the student will:

- Define the uses and purpose of an annuity.
- Recognize the investment function of an annuity.
- Discern the difference between an annuity and a retirement plan.
- Understand the differences between fixed and variable annuities, and the characteristics that distinguish them.
- Be familiar with settlement/payout options.
- Identify the benefits associated with annuities.

Lesson One

The Long-Term Care Landscape

What is Long-Term Care?

Long-term care is the medical and/or social services term for helping people who develop disabilities or chronic care needs. For example, there may come a time when you or a loved one need help walking, getting dressed, eating or bathing. It also includes the kind of care you would need if you had a severe cognitive impairment such as Alzheimer's disease.

Long-term care services may be brief with full recovery, or they can continue for years. They may be provided in a person's home, in the community, or in a residential facility (e.g., nursing home or assisted living facility).

What is Long-Term Care Insurance?

Long-term care insurance pays for long-term care services so you don't have to. Policies vary in terms of what they will cover and insurance companies do require that applicants qualify for the coverage.

Is Insurance the Right Solution?

Insurance may be the right solution, but it depends on circumstances. A long-term care expert once said that we should recognize where the need is greatest. For example:

- **If one has minimal assets**, long-term care insurance is inappropriate because the premiums would deplete those assets and they will likely qualify for Medicaid anyway.
- **If one is independently wealthy** and can pay for long-term care themselves, the coverage may be very attractive because the relatively

modest premium makes sense when compared to the high costs of the care.

- **If one has assets worth protecting**, but are not independently wealthy, they need to get the facts about long-term care insurance and consider this coverage as an alternative to paying for the care themselves.

Insurance is an important tool for protecting a person against risk. For instance, health insurance pays doctor and hospital bills should one get sick or injured. But how can they help protect themselves against the significant financial risk posed by the potential need for long-term care services, either in a nursing home or in their own home?

What About Americans & Long-Term Care?

Americans are living longer, yet more people can expect to have some sort of disability in their later years of life. Innovative approaches to long-term care, such as re-imagining nursing home care, may improve quality and provide more choices.

Long-term care includes a wide range of medical, social and personal care services that are provided to individuals with chronic illnesses or disabilities over an extended period of time. Services can include care provided in a nursing home, home or community, a hospice center or a respite center. Aging individuals as well as younger people with disabilities may need long-term care.

Long-term care is increasingly recognized as one of the most pressing issues facing our nation's health care system, due to a number of factors:

- Many of us have longer life expectancies, largely due to better medical care and technology, healthier lifestyles, and environmental improvements in recent decades.
- More Americans than ever before can expect to have some sort of disability in their later years of life.

Long-term care goes beyond medical care and nursing care to include all the assistance they could need if they ever have a chronic illness or disability that leaves them unable to care for themselves for an extended period of time. They can receive long-term care in a nursing home, assisted-living facility, or in their own home.

Need for Long-Term Care

You may never need long-term care. But about 19 percent of Americans aged 65 and older experience some degree of chronic physical impairment. Among those aged 85 or older, the proportion of people who are impaired and require long-term care is about 55 percent. In the year 2020, some 12 million older Americans are expected to need long-term care. Most will be cared for at home. Family members and friends are the sole caregivers for 70 percent of elderly people. A study by the U.S. Department of Health and Human Services indicates that people age 65 face at least a 40 percent lifetime risk of entering a nursing home sometime during their lifetime. About 10 percent will stay there five years or longer.

Baby Boomers

Long-term care is one of the most significant issues for our aging population in the 2000s and beyond. As medical science continues to allow Americans to live longer, as Baby Boomers enter the ranks of the senior citizen population, as economic pressures increase on federal and state budgets, and as the costs of providing long-term custodian care rise, we can expect LTC insurance to take an ever increasing role in the market place.

For those who engage in insurance and financial counseling to a clientele approaching middle age and older, long-term care insurance will play an increasing role. We can expect insurers to continue to improve their LTC product offerings and more companies will be entering this market.

Not long ago, the phrase long-term care was used when referring to a patient that required medical care and/or consistent assistance in an institutional nursing facility. In

the last several years, however, the issue of long-term care has received more specific and special attention from all levels of government. As an individual ages and becomes acutely aware of their impending maturity in a variety of ways, the once narrowly defined term no longer meets the reality, variety of care options, and the real needs of our general aging population.

In many states there continues to be growing pressure to implement strategies that will provide and improve long-term care services not only for nursing home patients, but also for the many individuals at all ages which need assistance in their homes and communities.

This growing social pressure is due in part to the large segment of our population referred to as the "Baby Boomers." Now in their late forties, fifties and early sixties, this generation of Americans is redefining the scope of and needs for long-term care.

Long-Term Care Insurance Act & Effective Date

One significant response to the pressure to serve Americans' long-term care needs properly for the short and longer terms is the NAIC "Long-Term Care Insurance Act." The LTC Model Act was originally adopted on December 1986.

The **purpose** of the "Act" is to promote the public interest, to promote the availability of long-term care insurance policies, to protect applicants for long-term care insurance, as defined, from unfair or deceptive sales or enrollment practices, to establish standards for long-term care insurance, to facilitate public understanding and comparison of long-term care insurance policies, and to facilitate flexibility and innovation in the development of long-term care insurance coverage. The purpose of the Act evidences legislative intent to protect the public while recognizing the need to permit flexibility and innovation with respect to long-term care insurance coverage.

Applicability and Scope

Except as otherwise specifically provided, this regulation applies to all long-term care insurance policies, including qualified long-term care contracts and life insurance policies that accelerate benefits for long-term care delivered or issued for delivery in this state on or after the effective date by insurers; fraternal benefit societies; nonprofit health, hospital and medical service corporations; prepaid health plans; health maintenance organizations and all similar organizations. Certain provisions of this regulation apply only to qualified long-term care insurance contracts, as noted.

Additionally, this regulation is intended to apply to policies having indemnity benefits that are triggered by activities of daily living and sold as disability income insurance, if:

- The benefits of the disability income policy are dependent upon or vary in amount based on the receipt of long-term care services;
- The disability income policy is advertised, marketed or offered as insurance for long-term care services; or
- Benefits under the policy may commence after the policyholder has reached Social Security's normal retirement age unless benefits are designed to replace lost income or pay for specific expenses other than long-term care services.

Today, individuals are very interested in long-term care insurance and many companies are providing insurance policies to meet this demand. However, the complexity of services and providers that exist today make it essential for agents to understand the mandates and regulations governing long-term care.

They must also understand the difference between long-term custodial care and clinically needed care. Custodial care can be offered at home, senior residential homes and nursing homes; but, for patients needing clinical care, a skilled nursing home is more appropriate and desirable.

Short Title

This Act may be known and cited as the "Long-Term Care Insurance Act."

Producer Training Requirements - NAIC LTCI Model Act

Professional well-trained producers are the key to sustaining a positive marketplace for Long-Term Care insurance. Most states now have or will soon have a specific training requirement for training of producers before their being able to sell LTC products.

An individual may not sell, solicit or negotiate long-term care insurance unless the individual is licensed as an insurance producer for accident and health or sickness or life including other lines of authority, as applicable, and has completed a one-time training course. The training must meet the requirements in the ACT.

An individual already licensed and selling, soliciting or negotiating long-term care insurance on the effective date of the Act in their state cannot continue to sell, solicit or negotiate long-term care insurance unless the individual has completed a one-time training course, within one year from the effective date of the state's LTC training legislation.

In addition to the one-time training course required, an individual who sells, solicits or negotiates long-term care insurance must complete ongoing training.

The training requirements may be approved as continuing education courses under applicable state law or regulation.

The one-time training required by this Section can be no less than eight (8) hours and the ongoing training must be no less than four (4) hours every 24 months.

The training required consists of topics related to long-term care insurance, long-term care services and, if applicable, qualified state long-term care insurance Partnership programs, including, but not limited to:

- State and federal regulations and requirements and the relationship between qualified state long-term care insurance Partnership programs and other public and private coverage of long-term care services, including Medicaid;
- Available long-term services and providers;
- Changes or improvements in long-term care services or providers;
- Alternatives to the purchase of private long-term care insurance;
- The effect of inflation on benefits & importance of inflation protection;
- Consumer suitability standards and guidelines.

The training required by this Section cannot include training that is insurer or company product specific or that includes any sales or marketing information, materials, or training, other than those required by state or federal law.

Insurers subject to this Act must obtain verification that a producer receives training required before a producer is permitted to sell, solicit or negotiate the insurer's long-term care insurance products, maintain records subject to the state's record retention requirements, and make that verification available to the commissioner upon request.

Insurers subject to this Act must maintain records with respect to the training of its producers concerning the distribution of its Partnership policies that will allow the state insurance department to provide assurance to the state Medicaid agency that producers have received the required training and that producers have demonstrated an understanding of the Partnership policies and their relationship to public and private coverage of long-term care, including Medicaid, in their state. These records must be maintained in accordance with the state's record retention requirements and must be made available to the commissioner upon request. The satisfaction of these training

requirements in any state will be deemed to satisfy the training requirements in this state.

Please return to the Course Player in order to take Lesson 1 Quiz.

Lesson Two

Long-Term Care Basics

The Chance of Needing Care

Getting old and needing long-term care isn't a subject that most of us care to dwell on. Generally, long-term care is the care people need when they can't perform the tasks of ordinary living independently without consistent assistance from others. However, the failure to discuss the realities of aging or plan for the high cost of long-term care is causing American families to be unprepared to meet the high costs of long-term care.

It has been reported that there is a 1 in 80 chance of ever using one's homeowner insurance, and a 1 in 40 chance of using one's automobile insurance, but there is a 60 percent, 6 in 10, chance of a person being in a nursing home after age 65.

Planning Ahead

If one has been ignoring or denying the need to plan for the risk of long-term care, they are not alone. Too many individuals are avoiding long-term care risk protection as part of their overall financial planning. As a result, families are faced with many dilemmas and decisions on how to finance long-term care if and when a need arises. Here are several excellent reasons to plan now, versus waiting for a crisis.

- **No age group is exempt from facing the risk of needing long-term care.** Waiting until one is 65 or older to think about long-term care issues can be too late. The population in need of long-term care is diverse in age and level of disability. A majority are over age 65 (57%), however 43% are below 65 years of age. As one ages, the probability they will need care, either at home or in an institution, increases. At ages 65-74,

12% of all people need help with care tasks. At age 85 and over, 70% of all people need help with activities of daily living and long-term care.

- **Planning ahead provides peace of mind.** It helps reduce fear and worry. It provides us with the security of knowing that our plans have been made and our wishes will more likely be followed.
- **More choices and options are typically available when we plan early.** The options and choices available to us tend to decrease with age and increased health risks. Planning early allows us time to research options and gather information about potential choices before making decisions.
- **Planning early allows us time to determine, clarify and prioritize our goals.** The decisions we make about how to finance long-term care should help us achieve our overall financial goals. Planning allows us to identify and share our financial goals with others and to seek their input, if we so choose.
- **Planning early may help reduce the potential for misunderstandings and conflict.** It allows us to discuss our plans with our spouse, our children, and others. It also allows us to discuss the potential role these people may have in our care and any limitations we may have.
- **Making plans early will reduce the burden for others.** By making plans while we are mentally competent, we will be able to choose our own direction rather than forcing someone else to make choices for us.

Long-Term Care is Many Things

Due to advances in our medical technology, our society has been blessed with an increasing average life span. Today, we are living almost twice as long as our ancestors did only 100 years ago. More recently, males born in the 1930's had a 50 percent chance of reaching age 65...slightly higher (60%) for females. Unfortunately, along with this increased longevity comes the drawback that the longer we live, the more likely we will need long-term care before we die.

Long-term care is so many things. It affects so many people: adult children, husbands, wives, employers, and friends. Long-term care means needing care for a long period of time, or at least greater than 90 days. A hospitalization for pneumonia, heart surgery, or kidney stones is not considered long-term care. A diagnosis of Parkinson's Disease, Alzheimer's Disease, Emphysema, a Stroke, or Lou Gehrig's Disease are diseases and conditions that at some point may require long-term care.

Today, long-term care is defined more broadly than it was previously. It includes care not only for the aged but also disabled children and adults as well as for AIDS patients and mentally impaired individuals. Long-term care has also become a family issue, a service provider issue, and a worker capability issue. Consequently, it is important for insurance agents to understand the specialized terminology and the varied levels of service involved.

When using the phrase "long-term care" in regard to organizations and delivery of services, the financing of it and the assistance with activities of daily living (ADLs) are all considered in the planning process. ADLs include many things we take for granted such as eating, bathing, dressing, moving freely about the house or even getting out of a chair. They are actually things that we have mastered by age 6. ADLs are a measure of a person's ability to function in their home and within the community. Simply stated, it includes any activities that enable one to remain independent.

The first loss of independence is the inability to walk. As a result, mobility is not as efficient any more. Before long, dressing and bathing cannot be accomplished without great effort or help. When a person needs assistance using the toilet, consistent full-time assistance is needed. An agent needs to understand how the whole host of ADLs and the assessment process determine benefit eligibility from the long-term care insurance policy.

Long-Term Care Policies

Types of Long-Term Care Policies

At a glance, several types of policies are available. Most are known as “indemnity” or “expense incurred” policies.

An indemnity or “per diem” policy pays up to a fixed benefit amount regardless of what you spend. With an expense-incurred policy, you choose the benefit amount when you buy the policy and you are reimbursed for actual expenses for services received up to a fixed dollar amount per day, week, or month.

Today, many companies also offer “integrated policies” or policies with “pooled benefits.” This type of policy provides a total dollar amount that may be used for different types of long-term care services.

There is usually a daily, weekly, or monthly dollar limit for your covered long-term care expenses. For example, say you purchase a policy with a maximum benefit amount of \$150,000 of pooled benefits. Under this policy you would have a daily benefit of \$150 that would last for 1,000 days if you spend the maximum daily amount on care. If, however, your care costs less, you would receive benefits for more than 1,000 days.

There are no policies that guarantee to cover all expenses. You usually have a choice of daily benefit amounts ranging from \$50 to more than \$300 per day for nursing home coverage. The daily benefit for at-home care may be less than the benefit for nursing home care. It’s important to keep in mind that you are responsible for your actual nursing home or home care costs that exceed the daily benefit amount you purchased.

Because the per-day benefit you buy today may not be enough to cover higher costs years from now, most policies offer inflation adjustments. In many policies, for example, the initial benefit amount will increase automatically each year at a specified rate (such as 5 percent) compounded over the life of the policy.

Some life insurance policies offer long-term care benefits. With these accelerated or living benefits provisions, under certain circumstances a portion of the life insurance benefit is paid to the policyholder for long-term care services instead of to the beneficiary at the policyholder's death. Some companies make these benefits available to all policyholders; others offer them only to people buying new policies.

What Long-Term Care Insurance Policies Cover

Long-term care services are provided when a person cannot perform certain "activities of daily living" (ADLs), or is cognitively impaired because of senile dementia or Alzheimer's disease. Most commonly the ADLs used to determine the need for services include bathing, dressing, transferring (getting from a bed to a chair), toileting, eating, and continence.

Today's policies cover skilled, intermediate, and custodial care in state-licensed nursing homes. Long-term care policies usually also cover home care services such as skilled or non-skilled nursing care, physical therapy, homemakers, and home health aides provided by state-licensed and/or Medicare-certified home health agencies. Many policies also cover assisted living, adult daycare and other care in the community, alternate care, and respite care for the caregiver.

"Alternate care" is nonconventional care and services developed by a licensed health care practitioner that serve as an alternative to more costly nursing home care. Benefits for alternate care may be available for special medical care and treatments, different sites of care, or medically necessary modifications to the insured's home, like building ramps for wheelchairs or modifications to a kitchen or bathroom. A health care professional develops the alternate plan of care, the insured or insurer may initiate the plan, and the insurer approves it.

You should know that the benefit amount paid for alternate care would reduce the maximum or lifetime benefit available for later confinement in a long-term care facility.

Policies may limit the expenses covered under this benefit (for instance, 60 percent of the lifetime maximum limit).

Alzheimer's disease and other organic cognitive disabilities are leading causes for nursing home admissions and worry for many older Americans. These conditions are generally covered under long-term care policies.

Cost

Long-term care can be very expensive and the real amount you will spend depends on the level of services and the length of time you need care. One year in a nursing home can average more than \$50,000. In some regions, it can easily cost twice that amount.

Home care is less expensive but it still adds up. Bringing an aide into your home just three times a week (two to three hours per visit) to help with dressing, bathing, preparing meals, and similar household chores can easily cost \$1,000 a month or \$12,000 a year. Add in the cost of skilled help, such as physical therapists, and these costs can be much greater.

The average monthly fee assisted living facilities charge is around \$2,000. This includes rent and most additional fees. Some residents in the facility may pay significantly more if their care needs are higher.

As the costs of long-term-care services continue to rise, policy-makers are seeking innovative approaches to meet increasing demand.

Availability of Long-Term Care Coverage

Although long-term care insurance is relatively new, more than 100 companies now offer coverage.

Long-term care insurance is generally available through groups and to individuals. Group insurance is typically offered through employers, and this type of coverage is

becoming a more common benefit. Just a few years back, more than 5,600 employers were offering a long-term care insurance plan to their employees, retirees, or both.

Individual long-term care insurance coverage is a good option if one is not employed, work for a small company that doesn't offer a plan, or are self-employed. Choosing a policy requires careful shopping because coverage and costs vary from company to company and depend on the benefit levels one chooses.

The Need for Long-Term Care

Chances Are Good

You may never need long-term care. But about 19 percent of Americans aged 65 and older experience some degree of chronic physical impairment. Among those aged 85 or older, the proportion of people who are impaired and require long-term care is about 55 percent. In the year 2020, some 12 million older Americans are expected to need long-term care. Most will be cared for at home. Family members and friends are the sole caregivers for 70 percent of elderly people. A study by the U.S. Department of Health and Human Services indicates that people aged 65 face at least a 40 percent lifetime risk of entering a nursing home sometime during their lifetime. About 10 percent will stay there five years or longer.

The American population is growing older, and the group over age 85 is now the fastest-growing segment of the population. The odds of entering a nursing home, and staying for longer periods, increase with age. In fact, statistics show that at any given time, 22 percent of those aged 85 and older are in a nursing home. Because women generally outlive men by several years, they face a 50 percent greater likelihood than men of entering a nursing home after age 65.

While certainly older people are more likely to need long-term care, your need for long-term care can come at any age. In fact, the U.S. Government Accountability Office

estimates that 40 percent of the 13 million people receiving long-term care services are between the ages of 18 and 64.

A Changing Society

We are certainly a changing society, both by ethnicity and age. This has changed how we view every facet of our daily lives in America, including long-term care. It will continue to require constant attention to these changes to meet the needs of our people.

The unique mix of people in America creates special challenges. A formal study done by one state's health and welfare agency in 1986 indicated that the demographics of that state were changing rapidly. Subsequently, another report stated that the population was increasing at a rate of 450,000 per year. More alarming was that the population of 65 and older was increasing more rapidly than any other age bracket below 65. To compound the problem, in 1988, statistics showed that people age 85 and older were increasing faster than any other age group.

Today, more than twenty years later, in just one state, there are nearly 4 million age 65 and over. Half a million of these people are considered to be the "oldest old" (i.e. those whose age is 85 and over). In 2011, the senior population in just one state is projected to grow by approximately 20% to 5 million. Between 2010 and 2020, the senior population there is expected to increase by 30% to 7 million. Furthermore, this state's senior population is becoming racially and ethnically more diverse. By 2020, an estimated 40% of its elders will be non-white. This is just one example of one state which has similarities by example of what is happening in all 50 states.

Overall, the current average stay in custodial care is four years today for both male and females combined. The length for males is less than this average and for females longer. The length of life in care is estimated to double in ten years! That's an increase in the average stay from four years to eight years in a very short period of time, relatively speaking. With the baby boomer population bubble moving into the over 65

age group over the next 17 years, this means that we will have millions more seniors that are staying longer in extended custodial care facilities. This creates a double duty pressure on our financial systems and facilities providing care for our seniors needing long-term care.

It has been said that one will incur 40% of their entire lifetime medical costs in the last 18 months of their life. The enormity of this problem becomes more apparent when this fact is realized that our oldest of old (85+) will require the most assistance, both socially and financially.

State's problems are considerable. The last U.S. Census Report showed that California, for example, has the greatest number of seniors more than any other state, including Florida. The problem is compounded because the oldest of the old require the most help in their activities of daily living. The next U.S. Census Report will undoubtedly show how much more crucial and unique these problems are for all states.

Factors Influencing Long-Term Care

The personal risk of needing long-term care depends on many factors. Some of those are determined by whether one is male or female, how long they live, their health history and whether they have a spouse or family member who can provide some of the care they may need.

Longevity

The longer one lives, the more likely it is that they will need long-term care. Those who live to be 95 years old or older are much more likely to have spent five or more years in a nursing home than those who die in their mid-70s. Much less is known about the use of home care services.

Gender

Women are at a much higher risk of needing to pay for formal long-term care for several reasons. Women not only have longer life spans, they often out-live their spouses.

When they need long-term care in their older years, there is often no one to care for them at home and they are more likely to need institutional care as a result of all these factors.

Married or Single

If one has a spouse or other family members who can provide care for them, they are more likely to be able to remain in their own home when they need care. If family members are unable to provide care, and they cannot pay someone to take care of them, then a nursing home may be the only available option. The condition that causes one to need care, and the severity of that condition, may determine whether they can be cared for at home or whether institutional care is the only option. For instance, a severe stroke could be so disabling that care at home is impossible, or an Alzheimer's patient may need constant supervision.

Health

Certain health conditions, like Alzheimer's or a stroke, can cause a need for long-term care. If one knows that certain health conditions run in their family, they may have a greater risk of needing long-term care than another person of the same age and gender. Unfortunately, it may be that this known health condition could also make them ineligible to buy this type of insurance.

Long-Term Care Insurance Regulations are Complex

The responsibility of understanding new regulations and explaining them to consumers rests on the shoulders of insurance brokers and agents. There are both penalties and fines for failing to do this accurately; therefore, agents must understand them thoroughly. Prospective customers should be made aware of the many long-term care policies available and the differences between them. Some important issues to be explained follow:

- Who the providers of care are

- Options in paying for long-term care
- Federal and State long-term care tax policies
- Nursing Home/Residential Care
- Consumer protection

The differences between federal tax-qualified insurance policies and state tax-qualified insurance policies caused special concern. If a consumer had a tax-qualified policy and itemize on their tax schedules, they could deduct any expenses connected with the policy; similar to regular health & accident policies that can be itemized on the federal income tax form. Most states have the same deductions.

Looking Ahead

This course, in part, highlights and explains the many long-term care issues so agents can better provide a comprehensive view of long-term care to their clients and consumers in general. It provides illustrations, charts, and statistics that provide for an easy-to-understand picture of this complex subject.

The aging population of “Baby Boomers” continues to spark a growing interest in long-term health care options. Insurance agents are responsible for explaining this important health issue to consumers so that they can make informed decisions about their future well being. Agents who fail to do this accurately can be subject to penalties and fines. This responsibility is regarded with so much importance that many states require agents to attend state-approved classes concerning long-term care. Some of the issues discussed in the following lessons will address mandated regulations, definitions of terms, and available services.

Long-term care is defined much differently today than a decade ago. Previously, it was thought of as help for the elderly, usually in the nursing home setting. Today, long-term care encompasses impaired children and adults, AIDS patients, and mentally disabled individuals. Most states have placed the responsibility of providing information to consumers on brokers and agents. Therefore, agents must understand the entire realm

of long-term care and have the knowledge and ability to educate prospective clients on the use of long-term care insurance.

Agents must make their prospective customers aware of the variety of long-term care insurance policies available and the differences between them. Among a whole host of information to be covered, some important issues to be covered in other lessons are:

Important issues to be covered	Some questions to be answered
NAIC Model LTC Regulations	What are the Activities of Daily Living?
Activities of Daily Living (ADLs)	How do the Activities of Daily Living trigger insurance benefits?
Skilled nursing care & Nursing home/Residential Care	When does the insured need help?
Benefit Triggers	When is the client eligible for insurance policy benefits?
Qualified and Non-Qualified Policies	Does a client have a policy allowing him to remain at home & still claim benefits?
LTC State Partnerships	What are the important features to have in my policy?
Consumer Protections	What should I look for in a LTC policy?
Policy Provisions	How am I protected from unethical agents?
LTC Services and Providers	
...and much more!	

These are just a few of the basic issues an agent must know to recommend the right long-term care insurance policy for a client.

Please return to the Course Player in order to take Lesson 2 Quiz.

Lesson Three

Providers and Services of Long-Term Care

The supply of beds are lagging behind the demand by an ever increasing number of individuals that are requiring consistent assistance in their activities of daily living and for assistance in coping with cognitive and mental impairment in varying degrees.

Some needs are easily taken care of at home, while others would be better cared for in a nursing home. Long-term care can be received in a variety of settings. The setting is usually determined by their support system (their family, attending physician, or someone qualified to develop a plan of care for them) and the reason that they need long-term care.

“Nursing homes” are being supplanted by a variety of new methods of care. Various states early response to future long-term care needs laid the foundation for the multitude of services offered today.

Care Levels

There are generally three levels of skilled nursing care:

- The first level is twenty-four hour care in a **skilled nursing facility**. This type of facility is generally licensed by a state’s department of health services. It can be paid for by Medicare, Medicaid, and private insurance. Medicare pays for it when one is hospitalized first. Hospitalization is a condition for which Medicare pays.
- The second level, **intermediate care**, is also licensed by a state’s department of health services. Medicaid certification is generally required, but the level of health-care is not as extensive as skilled nursing care. Intermediate care can also be paid for by Medicare, Medicaid, and private

insurance. Receiving intermediate care is another condition required if paid for by Medicare.

- The third level of care is **custodial care**. Under this type of care, a doctor prescribes a plan of care and a nurse aids in the care. Medicare does not cover it, but it can be covered by Medicaid.

Service Providers of Long-Term Care

The long-term care available in many states is comprised of several layers of care from the individual needing assistance in their home to institutionalized patients requiring 24-hour skilled nursing care. The services may be classified into two general categories: Community Based Services and Institutional Care Services. In addition to these two categories, there is also the Informal Network consisting of friends and family members.

Community-Based Care

Adult Day Health and/or Social Day Care are included in this category. These are only for the day outside of the home. This facility is supervised by licensed professionals who can provide medication management. The cost is from \$50-\$75 per day and is licensed by the state's Department of Health Services or similar agency. If eligible, Medicaid or private long-term care insurance will pay for this type of care.

Social Day Care is for support services needed by someone who has no one at home to talk to because children work all day. She can interact with others, play games, watch television and have meals with other people. This depends on location and is generally licensed by a state's department of social services.

Home Health Care is another community-based service, but is for skilled nursing in a person's home. It can include part-time, intermittent skilled nursing services, home health aid services, physical therapy, occupational therapy, speech therapy, audiology services and even medical social services by a social worker.

This type of care allows a person to live at home and still receive part-time medical help by a licensed person, a physician, or other professional who prescribes treatment. Services can include skilled nursing, part-time and intermittent skilled nursing services, and home health aid services. These services are provided by a home health agency and paid for by the patient, Medicare, Medicaid or private insurance, depending on the circumstances. The cost can be \$100 or more for a home visit by a licensed nurse, or \$110 or more per home visit by a medical social worker.

Many state's health and welfare department's policy statements emphasize the goal of allowing seniors to stay in their own home with the assistance to help them do that. They work hard towards that goal and today the options for seniors to stay in their homes are much greater than they were in the late '80s.

Informal Network

Family Members

Millions of seniors live with family members or others who provide needed services or access services in case of an emergency. Research has shown that an estimated 70% of the elderly disabled receive care informally from family and friends. Other studies report that there are millions of caregivers, usually consisting of spouses, daughters, and daughters-in-law. These caregivers report they usually help with shopping, transportation, and housekeeping. Families, friends and the community are providing more services than the former long-term care system.

Patient Paid Home Care

Long-term care needs vary widely among seniors. For some, assistance provided for them by others in their home is the most appropriate solution based on their needs. Home care is provided in the home by a person or professional company that is paid by the patient. Non-medical aid is provided and may include housekeeping, companionship, shopping, or cooking; in addition to daily living assistance. It can take place in the patient's home or in a relative's house. The cost varies for different

geographical areas, but is usually charged by the day and can range widely depending on the personal aid not living in the home. A 'live-in' companion/homemaker can cost up to \$155 or more per day. If a high level of skill is needed, the cost will increase.

Personal Homemaker Care

When possible, it is desirable for seniors to stay in an environment that is familiar to them as opposed to going into a care facility outside the home. Receiving care, while staying home for most, is preferable to an institution. The availability of home care services varies greatly from community to community, as does the cost of these services.

This type of service allows a patient to remain at home only with assistance that can include bathing, dressing and medical help following a plan of care prescribed by a doctor. A licensed nurse or a non-medical person who is paid either by the patient, or with certain qualifications, by Medicare, can supply this care. Some insurance plans may also cover this type of care. If a licensed nurse is needed for help, this costs approximately \$100-150 or more per visit. A professional has usually developed a plan of care for the patient.

Adult Boarding Care

Smaller group homes which provide care for seniors and frail elders have many names. Depending on where you live, they may be called board and care homes, assisted living, residential care homes for the elderly, personal care, adult foster care, adult group homes, adult family homes or boarding care homes. These residences offer room and board in a small home-like environment, generally housing 10 persons or fewer. However, some such facilities can accommodate 100 residents or more. They are often located in residential neighborhoods and offer a less institutional alternative to skilled nursing facilities (nursing homes) or hospitals.

These homes do *not* provide medically-oriented skilled nursing services. For this reason, they are not usually reimbursed by Medicaid. Some states have licensing

requirements for board and care homes which regulate services, facility environment, staffing, and reimbursement. Alternate means of reimbursement (e.g., supplemental security income - SSI) may be accepted for low-income residents.

Assisted Living Facility and/or Residential Care

For those seniors where the home is not the appropriate venue for them to have the level of care needed by them, there are alternatives available to deliver the type and amount of care they require. Assisted living facilities and residential care facilities and centers offer help for many seniors.

Assisted living facilities are stand alone or can be part of a larger facility. These facilities provide assistance with personal care, in addition to services provided in associated facilities. Residential care facilities provide room and board, personal care, and necessary supervision. This type of care is for people who need assistance with daily activities while maintaining some level of independence. Patients usually have their own room or apartment. If medical or other health service is needed, help can be provided. The patient may need various types of aid such as help with preparing meals, laundry, housecleaning or transportation. These services can be paid for by the client, by Medicaid, if qualified, or private long-term care insurance. Both of these facilities are licensed by state's departments of social service.

Continuing Care Retirement Community (CCRC)

There are many seniors who have the financial capacity to choose a retirement and long-term care arrangement that provides choices of retirement lifestyle as well as availability of a variety of "in house" or "on site" care options should they need them as they age and have varying degrees of need for care and assistance.

A CCRC is generally defined as a privately organized residential community where apartment maintenance and long-term care services are available on an 'as needed' basis. CCRC's have various degrees of health and activities of daily living requirements.

Residents pay a large one-time entry fee plus a monthly maintenance fee in exchange for the assurance of lifetime long-term care. Upon entering a CCRC, a resident is required to sign a legal binding contract. Since this contract has serious financial implications, this decision should be discussed with a financial planner and the contract should be reviewed by an attorney. If little or no care or help is needed, the residents have their own living quarters. If some assistance is needed, it can be provided for. As an added provision in the community, nursing home beds are there for anyone who needs complete care. The different levels of services are available as the need arises. It is funded by membership fees. Many are owned by churches and other private organizations. The nursing home and assisted living functions are generally licensed by the state's departments of social services and/or health services.

Although the contracts to enter a CCRC are legal and binding, this scenario was the right decision for many couples. After discussing the situation with their attorney, they move into a nice apartment. They have a garage for their car, take trips as they wish, visit their friends and relatives and are content. They have laundry services and they can either do their own cooking in their apartment or eat in the dining room. After living in a CCRC for a few years, one might suffer a stroke. As a result, the spouse could no longer help her husband with his dressing, bathing or eating. A personal aid came each day to help her with his daily living activities. The personal aid would also stay with him if she wanted to go outside the home. They had people to clean the apartment and a nurse to assist with his medication. This situation was fine for a year until the spouse had a heart attack. He was moved to the nursing care wing where he received full care. The spouse was able to visit him every day in a wheel chair with the help of his personal aid.

Special Care

Respite Care

Many seniors and some of us will face a terminal illness ourselves or face it with those we love in our families. Respite care offers temporary relief to caregivers for the terminally ill person.

This type of short-term care is for family members providing care or others taking care of an elder. It also applies to anyone providing full time care for Alzheimer's patients and the mentally impaired. Respite care can be provided in an institution, within the home or in a community-based program. Respite care can be paid by a private party or insurance. Medicare will pay for five days or less, if eligible. A person may also come into the home to stay with the patient. These facilities are generally licensed by state's departments of aging.

Hospice Services

When a person is diagnosed as being terminally ill, there many times is a need for physical, emotional, social, and spiritual support. This is where hospice services come into the picture.

This service is for patients with terminal illness. It provides any care needed by the patient as well as support services for the family. These services can be in a separate facility or in the home with a medical person who comes to administer any related health services. Care is provided by a skilled or unskilled person under a plan of care prescribed by a physician or a multidisciplinary team under medical direction. This is an outpatient service and therefore is not eligible for Medicare. It can, however, be paid for by private insurance. The cost will vary based on whether it is in the home, in a separate facility or with the aid of a medical professional.

Steve has been diagnosed with a terminal illness. He does not have the benefit of family and friends to assist him, so he chooses to live in a separate facility to be with other

people. His medical needs are provided through his doctor's prescribed plan of care. Long-term care insurance pays for his stay.

Institutional Care

Skilled Nursing Facilities (SNF)

When a person's condition gets to a point where they need levels of care that can only be provided by skilled nurses or non-skilled persons in a facility that specialized in delivering these levels of care, they may need the services provided by a skilled nursing facility. Most nursing home stays are relatively short. Seventy five percent of people who enter a nursing home stay one year or less, and 52 percent stay less than 3 months.

Skilled nursing homes are institutional facilities with services ranging from custodial care, intermediate care and skilled nursing care. ***Custodial care*** is determined by a professional who has planned the care needed by the patient. ***Intermediate care*** provides health care such as giving shots, medications and therapy. ***Skilled nursing care*** is provided by licensed nurses for patients needing more intensive care. Convalescence from surgeries or serious illness fall under this definition. SNF's are generally licensed by the state's departments of health services and funded by Medicare with certain qualifications, Medicaid, or private insurance. The cost can range from \$90 to \$300 and above per day or more, depending on the geographic area. The national median daily rate was \$193 in 2011.

The Brookings Institute estimates that the average 65 year old person has a 20% chance of incurring no long-term care expenses at home or in a nursing home; The remaining 80 percent have a 58% chance of incurring expenses between \$1 and \$50,000; a 9% chance of incurring expenses between \$50,000 and \$110,000; and almost a 13% chance of incurring expenditures of more than \$100,000 or more.

Facility Licensing Requirements

An important factor that consumers may not be aware of, is that many long-term care insurance policies require certain facilities to be licensed by the various states. Therefore, if a person requires care and would like to go to a special facility that is not licensed by their state, he or she will not be able to go there. The insurance agent must familiarize himself with their state facility licensing requirements and licensed facilities versus unlicensed facilities.

Generally, state departments of health services licensing and certification programs regulate licensed public and private health facilities throughout the state. These facilities include recovery hospitals, clinics, intermediate care facilities, skilled nursing facilities, home health agencies, referral agencies, adult day health care centers and intermediate care facilities for the developmentally disabled. These departments of health services also implement and enforce health standards and certifies all long-term care facilities including those that participate in Medicare and Medicaid programs.

Please return to the Course Player in order to take Lesson 3 Quiz.

Lesson Four

Paying for Long-Term Care

The costs of long-term care vary greatly depending on where one lives and the type of care they receive. Many people think that home care usually costs less than nursing home care. In fact, home care can cost just as much, if not more than nursing home care. It depends on what type of care they need in their home, and for how many hours per day.

Future Cost of Long-Term Care

When planning for one's future long-term care expenses it is important to not only note what the cost of care is today, but what it will be in the future. According to the General Accounting Office, the annual cost of care is growing by 5.8% per year. This means that in 20 years the cost of long-term care is expected to triple. So if one is 60 years old today, they need care at age 80, and they live in an area where the annual cost of care is \$150 per day or annually is \$54,385, the cost in 20 years would be \$167,954. Imagine what one's future expenses will be if they live in a higher cost area!

Incidental costs

Besides the normal daily rate that a nursing home charges, or the hourly rate that a home care agency charges, there are always going to be ancillary costs one should plan for. In a nursing home, additional expenses could be diapers, laundry, medications, and other incidentals. At home, there are additional expenses like safety devices (i.e. grab bars near the bathroom and toilet, and maybe a ramp for the stairs).

Personal costs

If a family member decides to provide the care rather than paying a professional, there are many human costs besides financial costs to consider. A family may or may not be aware of these, or be prepared for them. If their plans are to save the expense of paying

someone to care for themselves and rely on a family member instead, they need to consider the costs that they will pay - financially, physically, and emotionally. It is not unusual for the care-giving spouse to predecease the spouse being cared for, due to stress.

Over 25% of the adult population has provided care for a chronically ill, disabled, or aged family member or friend during the past year. Based on current census data, that translates into more than 54 million people.

Monetary Costs

When you rely on family members to take care of you, it is important to consider the financial costs they will incur. They may be more than willing to take care of you, but may be unaware there will probably be a financial loss to them. The financial loss to them could be very high. If they currently work, they may need to work fewer hours, or even give up their jobs altogether.

A 1999 study by a major insurer reported that care giving costs an individual an average of \$656,000 in lost wages, pensions, and Social Security, not to negative health impacts. Imagine that number today, over 10 years later.

Physical Costs

Providing long-term care is hard work. Family members report exhaustion from the long hours of care giving. If one had a stroke and could not bear their own weight it would be very difficult, if not impossible, for a family member to lift them in and out of bed. Especially if the caregiver is their spouse and they are both in their 80's! If they have Alzheimer's and require 24-hour care, it can be very exhausting for a family member to stay up all night and during the day. Their family caregiver couldn't do that for too many days in a row.

Emotional Costs

The emotional toll caregivers experience is another cost loved ones pay in providing care to them. If they are relying on a family member to take care of them, it is important to understand the type of care they may be asking them to provide. They may be thinking that the extent of the care is helping them in and out of bed, assisting them with meal preparation, or driving them to the doctor. But, what if they need more care than that? Will it be possible for the friend or family member to handle more extensive care?

Children can become clinically depressed while taking care of their parents. A majority of “intense” family caregivers that provide at least 21 hours of care a week have suffered from depression. Some studies have shown that caregiver stress inhibits healing.

Long-term care can be very expensive in most states. One year in a nursing home costs averages \$60,000 plus nationally. The average length of stay in a nursing home is 2.5-3.0 years. With the median head of household assets for retirement age people at \$73,471, just one and a half years in a nursing home could result in poverty for many Americans.

Medicare or Medicare Supplements to Pay for Long-term Care?

Many people assume that Medicare will pay for long-term care. But neither Medicare, private Medicare supplemental policies, nor general health insurance will pay for extended nursing home stays or home care services. Over half of all nursing home costs in most states are paid privately by individuals and their families. Medicare covers less than 5% of short-term skilled nursing home care following a hospital stay. Medicare does pay for some skilled in-home care, but only on a short-term basis. The remaining cost of long-term care in states is paid for by Medicaid which is a program designed to meet the health care needs low-income families. Assistance from Medicaid is provided immediately if one meets the eligibility requirements. Otherwise, they must “spend down” or deplete most of their assets to poverty levels to become eligible for Medicaid.

Private Long-term Care Payment Options

Three Basic Options

There are three basic options to cover long-term care expenses. They include (1) private pay out of current income, (2) pay out of assets, or “spend down” until they qualify for Medicaid, and (3) private long-term care insurance.

Insurance Policies and Other Methods

Many people run into situations in life where funds are needed to help them live out their remaining life in dignity and with funds to provide all sorts of things that come when one finds out they are terminally ill or need a major transplant. Life insurance companies, in an effort to meet these special life challenges for people, have created various options to be included in or attached to their life insurance policies to speak to these life situations at varied degrees.

Life insurance policies can be purchased with an “Accelerated Death Benefit” as part of the life insurance policy. This option allows one to ask their insurance company to pay a reduced amount of value on their insurance policy before their death, if they are diagnosed with a terminal illness or have a major organ transplant. Certain criteria must be met, but the proceeds may be used for any purpose, not just long-term care. If their current life insurance does not have this rider, they may wish to contact their insurance company to see if their policy can be modified.

Life insurance policies with Long-Term Care riders. As an alternative method to Accelerated Death Benefits in the previous section, long-term care benefits are occasionally sold as an additional rider (commonly called “advanced death benefit” or “living benefits”) to a life insurance policy. This rider allows lump sums or periodic payments through cash advances on the death benefit that can then be used to pay for specified long-term care services or any other purpose. The amount of the policy’s death benefit is reduced by the amount advanced under this rider.

Viatical Settlements. Non-life insurance companies also have created arrangements to help people receive advances on their death benefits from their life insurance policies. There are companies that will offer to buy one's existing life insurance policy at a discount (usually 50 to 80 percent) if they have a catastrophic or life-threatening illness. The company continues to pay the premium as it will be the beneficiary for the full amount when one dies. These companies must be licensed with the state's departments of insurance. If one is considering a Viatical Agreement, they should contact their insurance company to see if the medical information is kept confidential, if there may be tax consequences from entering into such a settlement, and if the company is properly licensed.

Annuities. Many people will save up their money in various financial instruments in order to have the funds available for future needs, including long-term care needs. Annuities are offered by most life insurance companies. They include two types of contracts, immediate and deferred. Immediate annuities provide for periodic payments tied to a specified event. For example, if you are placed in a nursing home, you can receive periodic payments toward the cost of your care. Deferred annuities accumulate tax-free until a specific event such as retirement when payments begin. They are comparable to certificates of deposit, but are not federally insured there are costs to have an annuity contract and the costs need to be weighed to determine its benefit relative to other avenues for providing funds for long-term care needs. A financial planner or tax advisor should be consulted before buying any type of annuity.

Home Equity Conversion or "Reverse Mortgages" were developed to help older people to tap into the value of their homes. Many people have a large portion of their asset base in the equity in their home. This equity represents a source of funds for meeting their needs for care in the future. A reverse mortgage allows one to remain in their home and receive payments for the equity. The amount of money they receive depends on their age, the value of their home and the cost of the loan. There are various types of home equity loans. Some require full-time residency and if they are unable to live in their home, the loan becomes due and payable. As with any complex

financial contract, one should discuss them with their financial advisor, accountant, or attorney before entering into one of these contracts.

Fraternal Organizations and Church Auxiliaries. Organizations and churches recognize the need for providing assistance for elders in general as well as their members in particular when it comes to the need for assistance with long-term care needs. Many of them have special funds to assist members of their organization who need long-term care assistance. Many religious groups have homes for the elderly that provide social, personal and medical services for members.

The Veterans Administration (VA) Honoring our veterans is important in all aspects of their lives as well as in providing for their needs for long-term care. The VA provides a wide variety of services for aging veterans through traditional hospital programs, VA nursing homes and the State Veterans Home. The VA also has non-institutional programs that include home care, adult day care, mental health clinics, day care at treatment centers and caregiver support programs. Not all veterans are eligible for these services, so it is important to inquire before you need assistance.

Private Long-Term Care Insurance is a clear and specialized way to provide the funds for our own and seniors' long-term needs. LTC insurance covers a wide range of services from home and community-based care to institutional care. A policy must be purchased before needing care. The younger a person is when the policy is purchased, the less expensive the premium will be. There are many options with private policies and there are significant differences between features, benefit options and eligibility criteria. In most states there are three types of long-term care insurance policies sold:

- Nursing Home/Residential Care Facility for the Elder
- Home Care Only
- Comprehensive long-term care (includes both home care and nursing facility)

The advantages and disadvantages of each policy must be explained to the client.

The cost of long-term care insurance varies with each policy, as does the coverage provided. For example, policies limited to nursing home/residential care are less expensive. Some of the factors that can influence the cost of long-term care insurance include:

- Your **age and health** at the time you apply for coverage;
- The **deductible and waiting period** you choose before the policy begins paying;
- The **daily or monthly benefit amount** you want the company to pay when you need care; and
- The **number of years** you want the company to pay benefits.

Note: Premium for tax-qualified policies may be a little less expensive because of the stricter eligibility standards.

To sum it up, the price of long-term care continues to increase each year. If a spouse must enter a nursing home, the other spouse can quickly become impoverished. Options exist such as fraternal and religious homes, reverse mortgages, and life insurance. Medicare and supplemental policies are not intended for long-term custodial care. Today's long-term care policies might be a good option to consider. Many people believe they will pay for the care as long as possible and then turn to Medicaid. This issue will be discussed further in the next section.

Medicaid as an Option

Most people do not know the difference between Medicaid and Medicare. As a result, people mistakenly believe that Medicare will provide for their custodial long-term care needs.

Medicare is a federal health care program funded by Social Security. Eligibility is based on a person's work history. Medicare will cover "medically necessary" or rehabilitative long-term care services.

Medicaid, on the other hand, funding comes from state and federal tax dollars. Eligibility is based on income and financial need.

The Medicaid program pays for custodial long-term care when people can no longer afford to pay for their own. Because long-term care costs are increasing at a rapid pace, many people are finding it necessary to apply for the Medicaid program.

Many people feel a stigma attached to Medicaid because they view it as welfare assistance. Medicaid can also be viewed as assistance to anyone in need, including the disabled, aged, or blind, as well as those experiencing financial difficulty that would otherwise not receive adequate care.

“Spend-Down” for Medicaid

Mr. and Mrs. Miller both worked hard to save money for their retirement. After only one year of retirement, Mr. Miller suffered a heart attack and required triple bypass surgery. Although Medicare paid 80% of hospital expenses, the remaining 20% significantly reduced their savings.

During Mr. Miller recovery he contracted pneumonia and was admitted to the hospital again. Mrs. Miller could no longer take care of him so he had to be placed in a nursing home. His health did not improve and he required additional care. After six months, Mr. Miller's health was still poor and Mrs. Miller had to apply for Medicaid.

Medicare only pays for a portion of nursing home costs. It does not pay for custodial care. A person must need skilled nursing and/or rehabilitation five days a week. Medicare covers nursing home care for 20 days. After 20 days, only a portion of skilled nursing care is covered for another 80 days.

To qualify for Medicaid, monthly income, personal property, and assets are considered. Because Mr. Miller had more income from his pension than he needed for his personal allowance, he had to give a “share-of-cost” for any treatment he received. This left

Mrs. Miller with \$2,739 a month to live on, but she was allowed to keep \$109,560 (2011) in assets and property. The maximum allowable to keep in income for his spouse would have been between \$1,821 and \$2,739 depending on certain expenses and geographic area. (2011 Figures)

The rules are different for single persons and married couples. If a single person needs long-term care, he or she can keep “approximately” \$21,912 (2011) depending on the state, in property and assets, but can only keep approximately \$60 a month from a pension or other income. The balance of their income will go towards the “share-of-cost”. These figures change every year. Therefore, it is important for agents to stay current on yearly property and asset and income allowances as Medicaid eligibility is based on these. The above allowances are valid for the year 2011, and change in July of each year.

There was a time when people thought they could transfer property and assets to their children or relatives to avoid ‘spend down’ requirements. However, there are criminal penalties for anyone attempting to do this in order to qualify for Medicaid. The law states, “whoever for a fee knowingly and willfully counsels or assists an individual to dispose of assets (including by any transfer to trust) in order for that individual to become eligible for medical assistance under a State plan under Title XIX; if disposing of the assets results in the imposition of a period of ineligibility for such assistance under Section 1917 (c) shall in (i) the case of such a statement, representation, concealment, failure or conversion by any person in connection with the furnishing (by that person) of items or services for which payment is or may be made under the program, be guilty of a felony and upon conviction fined not more than \$25,000 or imprisoned for not more than five years or both, or (ii) in the case of such a statement, representation, concealment, failure, conversion, or provision of counsel or assistance by any other person be guilty of a misdemeanor and upon conviction thereof fined not more than \$10,000 or imprisoned for not more than one year, or both” [Source: *Health Insurance Portability Accountability Act (HIPAA) 1996*].

Some people feel that because this is a program for the “medically needy”, one does not receive quality care. This is not true. Because nursing homes are licensed by the states, they are inspected for quality care.

Please return to the Course Player in order to take Lesson 4 Quiz.

Lesson Five

Long-Term Care Regulation

Federal Regulations Clarified Issues for States

How laws, both past and present, have addressed the environment and needs of state's residents is important to be aware of as producers go about assisting others who need their help and guidance... to be better versed in the legislative background and activities relating to the issues surrounding Long-Term Care.

In 1996, the Health Insurance Portability and Accountability Act (HIPAA) was passed at the federal level. HIPAA mandated regulations that were meant to clarify the characteristics of long-term care insurance, but in some ways, complicated it more. Consequently, states had to act quickly to analyze and modify their long-term care regulations to come in line with federal mandates.

When HIPAA was enacted, state policymakers began to address those issues that created problems for their state residents.

As a result of HIPAA, policies were divided into qualified long-term care insurance for tax purposes and non-qualified long-term care insurance not eligible for favorable tax treatment.

The two types of contracts resulted in differences in the benefit triggers for the activities of daily living.

Qualified and non-qualified plans have differences in eligibility time periods because federal rules stated that a health professional had to declare a client unable to perform two or more ADLs for a period of 90 days before care was considered long-term.

If a consumer wanted a tax-qualified policy, the guidelines HIPAA mandated were strictly enforced. One exception HIPAA made was for State-approved policies issued prior to 1/1/97. These policies were “grandfathered” as long as they were not altered from the guidelines HIPAA had stipulated.

The Health Insurance Portability and Accountability Act (HIPAA) created qualifications for long-term care insurance so that if a policy met the requirements, tax benefits for the owner of the policy were possible.

Most states have amended their tax laws relating to Long-Term Care Insurance so they are more in line with federal regulations, but there are still a few differences.

Differences between State and Federal Regulations

Benefit Triggers

The difference in benefit triggers between Federal and some state’s regulations was considerable. Benefit triggers, which allow the policyholder to begin receiving benefits, were more lenient for some states than for Federal policies.

A benefit trigger is the event that allows for a financial claim to be filed against the insurer issuing the insurance policy. For example, a fire policy's benefit trigger is a fire and an automobile policy's benefit trigger is a vehicle collision.

Benefit triggers for long-term care insurance involve the activities of daily living (ADLs). An insurance claim can be filed when specific ADL qualifications are met.

State and Federal ADLs

Before 1996, insurance companies in some states could not only include different activities of daily living to trigger a benefit but also could require a different number of deficiencies before policy benefits would be paid. Where one policy might have required the inability to do two of six ADLs, another policy might require three of seven. The

purpose of HIPAA was to standardize the inclusion of specific ADLs in policies and to define the number of ADL deficiencies that would trigger a policy's benefits.

When HIPAA became effective, insurance companies were required to include six ADLs in their policies. The policy benefits would be triggered if the insured could no longer perform two of the six ADLs. Prior to HIPAA, a few state's policy limit included seven ADLs. This did not present a problem, however, because the insurer had the freedom to impose a less stringent requirement. Federal mandates did not include any language prohibiting this practice.

As an example, under the federal tax-qualified definitions of ADLs, feeding oneself doesn't necessarily require the use of utensils. Barbara, age, 80, failed to meet federal ADL requirements for assisted eating because she still had use of her hands and fingers.

The federal definition for assisted eating is that a person can feed themselves by getting food into their mouth from a plate, cup, or table.

A few state's definition is slightly different. It states that, when eating, a person can "reach for, pick up, and get food on a utensil such as a fork and bring to the mouth".

Beverly met the state's definition of needing help because she could not put the food on to a fork and then bring it to her mouth. She could only use her fingers.

This is an example of how a tax-qualified long-term care insurance policy can prevent a person from receiving the help he or she needs. An agent must weigh these issues and consider the consequences as to which long-term care insurance policy is best for the client.

ADLs — Federal TQ (tax-qualified) vs. State

Federal TQ ADLs	State's ADLs
6 ADLs.	7 ADLs
Cognitive Impairment is a required trigger	Cognitive Impairment is a required trigger.
Inability to perform two ADLs.	Inability to perform two ADLs, but policy can impose less stringent requirement.
Regulations can be re-defined in future for eligibility requirements & other disability criteria.	Commissioner can approve other criteria.
Time period of 90 days & unable to perform two ADLs.	No time period imposed.
ADLs harder to meet.	ADLs easier to meet.

Not only do insurance agents need to know exactly which ADLs will trigger the insurance policy benefits, but they also need to know what the conditions are that might result in a change of care for their client. There are circumstances when a person having a chronic problem may develop a sudden illness or injury. They will need entirely different care for an acute illness in addition to a chronic condition.

Acute Versus Chronic Conditions

Individuals who have one or more physical or mental limitations that interfere with their activities of daily living require assistance ranging from custodial to skilled nursing care. These needs can change as more health problems arise. Therefore, it is important to understand the difference between acute health conditions and chronic health conditions. This is especially true of elderly patients.

Acute Care

Acute care is the type of care that is usually provided in hospitals and emergency rooms. Health insurance and Medicare will cover some or all of the expenses for acute care because it is usually skilled care. Acute care usually refers to conditions that are treatable and that one may fully recover from with the right medical attention. Acute care conditions usually develop rapidly and can strike suddenly. An acute condition is defined as a brief but severe illness such as. Some examples of this are stroke, heart attack, pneumonia, influenza, or injury.

Chronic Care

Chronic care, in contrast, is the type of care that is usually provided in the person's own home, nursing homes, and assisted living facilities. Health insurance and Medicare will usually not cover chronic care because it is not considered skilled care. Chronic care usually refers to conditions that are treatable, but generally not curable. A lot of times they are not noticed or are ignored in the initial stages. Examples of this are arthritis, diabetes, hypertension, heart disease, and Alzheimer's. A chronic condition is defined as an ailment or disease that continues for a long time or recurs frequently.

With the recognition that older patients with chronic conditions can function reasonably well for a long time, policy makers were compelled to reform the long-term care system. Thus, changes were made affecting both services and service providers.

Skilled Care vs. Non-skilled

Skilled Care

Skilled care is the type of care that is usually delivered in hospitals and skilled nursing homes, but can be received in the home, as well. If the skilled care meets strict criteria set forth by Medicare, then the facility providing it is referred to as a skilled nursing facility (SNF). Medicare defines skilled care as services and rehabilitation that require the skills of technical or professional personnel such as registered nurses, licensed practical nurses, and physical or occupational therapists.

For example, if one had a hip replaced, she is expected to recover fully. She needs physical therapy to be able to walk again. She was admitted to a skilled nursing facility to receive her therapy. She will remain there as long as she needs therapy on a daily basis and continues to improve and move towards getting better.

Non-Skilled Care

Non-skilled care is defined as the type of care that is provided to persons who need help or consistent assistance on a regular basis with their activities of daily living because of a physical limitation, chronic problem, or a cognitive problem. This type of care can be provided by a family member or by unskilled personnel. It does not have to be performed by a medical professional like skilled care does. Non-skilled care is also known as “custodial care”.

An example of this would be a person who is suffering from rheumatoid arthritis. She has difficulty dressing as well as getting in and out of the bathtub. Her daughter comes to her house in the morning and evening to take care of her mother’s needs.

Determining Assistance with Acute and Chronic Conditions

When and how a patient receives assistance depends on two elements. First, whether existing health conditions are chronic or acute. Second, whether ailments can fluctuate between acute and chronic conditions.

Long-Term Care Policies — Federal Vs. State

Tax deductibility of costs associated with medical care of all kinds helps to make the net cost for those expenses less than if they were not deductible. However, there are ‘trade-offs’ to be considered when one goes about deciding which type is most suitable for their particular situation. The following discussion examines this issue and provides insight into the evolution of dual approaches to tax-deductible and non-qualified long-term policies.

Congress passed legislation effective in 1997 that established the tax treatment of premiums paid for and the benefits paid by long-term care insurance policies that met certain federal standards. This legislation is called the Health Insurance Portability and Accountability Act or HIPAA.

Long-term care policies that use the federal standards to pay benefits are labeled as "Federally Tax-Qualified." Some or all of the premiums for these federally tax-qualified policies may be deductible as a medical expense on federal and state income tax returns (depending on their age and the amount of annual premium). In addition, the benefit payments are excluded from income.

Premiums paid for a tax-qualified policy, qualify as a medical expense. People who itemize medical expenses on their federal tax return and have total medical expenses greater than 7.5 percent of their adjusted gross income may be able to deduct all or some portion of the premiums paid for one of these policies. The benefits received from a tax-qualified long-term care policy are not taxable income on either the Federal or state tax returns.

Employers who provide tax-qualified long-term care insurance for their employees may deduct the premiums they pay as they do for other accident and health insurance policies. This employer contribution would not be considered income to the employees.

Policies sold as federally tax-qualified long-term care insurance use a standard of eligibility for benefits that may be stricter than the standards established in states for non-qualified policies. It may be easier to qualify for benefits from non-tax-qualified policies that use the standards established by the individual states. However, the premiums for these non-tax-qualified policies cannot be deducted and federal law is unclear on the tax treatment of the benefits paid.

If clients have questions about the tax status of a policy they own or one they are considering buying, you, if a qualified long-term care insurance agent, can advise them.

If they have specific questions pertaining to how the purchase of tax-qualified long-term care insurance will impact the deductions they take or the taxes they pay, they should talk to their tax advisor to see how it will affect their individual taxes.

With the evolution of long-term care needs, insurance coverage to meet those needs has changed significantly. The more familiar an agent is with these changes, the more informed a client will be about federal tax-qualified policies and state non-tax-qualified policies (State regulations can apply to *tax-qualified* long-term care insurance policies, if certain criteria are met).

The Health Insurance Portability and Accountability Act (HIPAA)

The Health Insurance Portability and Accountability Act of 1996 clarified federal laws on tax-qualified long-term care insurance contracts and expenses for qualified long-term care services. HIPAA provided favorable tax treatment for long-term care policies, contracts and services. Similar to routine medical insurance, long-term care premiums were now considered an eligible tax deduction on federal tax forms.

HIPAA outlined certain consumer protections that were tax-qualified on the federal level, but these same protections were not included in non-tax-qualified policies. Federal tax-qualified long-term care insurance policies must include the following elements:

- The insurance policy must be renewable and cannot have a cash value.
- It only provides coverage for qualified long-term care services.
- The policy cannot reimburse or pay for expenses paid by Medicare, unless Medicare is not the primary carrier.
- Refunds, other than those paid on the death of insured, cancellation or surrender, can only be used to increase future benefits or reduce premiums.
- The benefit triggers that activate a federal long-term care policy are more strict than each State's benefit triggers.

In addition to the above requirements, a qualified long-term care policy must follow requirements established by the National Association of Insurance Commissioners (NAIC). The NAIC stipulates that all prospective applicants must be given a Long-Term Care Shopper's Guide prior to completing an application or enrollment form (Many state's departments of aging have made this Shopper's Guide available for seniors. Policies that have living or accelerated benefits must have a policy summary instead of a shopper's guide.)

Changes and Improvements to Services and Facilities

There have been continuous changes and improvements between the old and newer insurance policies. Now consumers have more protections than before.

One example of this protection is that replacement policies are strictly monitored by requiring insurance companies to report the number of replacement policies written compared to newly written policies.

Consumers must have renewability clauses stated on page one of the policy. The first term of coverage, the conditions under which it may be renewed, and if premiums can be changed must also be included. All these conditions must be clearly stated. The insured must also sign a consent form if the policy has a right to increase premiums.

Limitations for pre-existing conditions must be labeled in a separate paragraph. This is also true for eligibility requirements that must state the number of days of confinement and description of limitations in the paragraph titled "Limitations or Conditions on Eligibility for Benefits."

Policies Include More Services

Benefit payments are now written into policies that include both Adult Day Care and Social Day Care insurance policies now allow home care without having a professional

declare that a client would need institutional care if they did not receive home care. Both of these programs do not require 24-hour care.

Generally, all insurance policies covering confinement in a nursing facility must also cover Residential Care Facilities for the Elderly (RCFE). The Insurance Code in many states requires coverage for assisted living care in a licensed residential care facility or a residential facility for the elderly. In most cases, the benefit amount payable for care may be no less than 70% of benefits for institutional confinement. All expenses incurred while in a residential care facility for long-term care services; such as diagnostic, preventative, therapeutic, treating, curing, mitigating, rehabilitative services, and including maintenance or personal care, etc., which are needed to assist the insured that cause him or her to be a chronically ill individual must be covered but not to exceed the maximum daily residential care facility benefit of the policy or certificate.

Home Care Benefits

"Home health care" is skilled nursing or other professional services in the residence, including, but not limited to, part-time and intermittent skilled nursing services, home health aid services, physical therapy, occupational therapy, or speech therapy and audiology services, and medical social services by a social worker.

State's Codes normally require that if home care or community-based services are written into the policy, they must provide:

- Adult Day Care
- Home Health Care
- Personal Care
- Homemaker Services
- Hospice Services
- Respite Care

Code Specifications and Restrictions

Many state's Codes specify that policy definitions cannot be more restrictive for **Adult Day Care (ADC)**. ADC is medical or non-medical care for less than 24 hours in a licensed facility outside the residence. Help is provided with activities of daily living and/or supervision that are in accord with each client's needs. **Home Health Care** is skilled nursing or other professional services in the home which include, but not limited to, part-time and intermittent skilled nursing services, home health aid services, physical therapy and medical social services by a social worker. **Personal Care** is assistance with activities of daily living and other aids developed by a professional medical worker in a plan of care. **Homemaker Services** are services that help the insured to stay in his or her own home under a doctor's a plan of care. **Hospice Services** are outpatient services that provide palliative care to alleviate the physical, emotional, social and spiritual discomforts of a person with a terminal disease. **Respite Care** is designed to relieve a primary care giver taking care of the patient and is a short-term rest either in the home or a special program.

State's codes generally also state that home care benefits will not be limited or excluded by the following:

- Requiring a need for care in a nursing home if home care services are not provided.
- Requiring that skilled nursing or therapeutic services be used before or with unskilled services.
- Requiring the existence of an acute conditional problem.
- Limiting benefits to services provided by Medicare certified providers or agencies.
- Defining an eligible provider in a manner that is more restrictive than that used to license that provider by the state where the service is provided.
- Requiring "medical necessity" or similar standard as a criteria for benefits.

Please return to the Course Player in order to take Lesson 5 Quiz.

Lesson Six

Definitions of Long-Term Care Insurance Benefits

One of the important roles of an agent in the long-term care Insurance arena is to be an educator for their clients in becoming competent in the framework and details of long-term care insurance policies. The more the client knows about the policy benefits and features, as well as the limitations and fine points, the more they will be confident in their decision to take action and to be confident after their purchase. They will have continued peace of mind and a greater sense of financial security.

The following is a brief discussion of the definition of those things that a client should know before they buy long-term care insurance. This lesson provides generally recognized definitions of benefits provided by the NAIC and also by various states's LTC systems. Since agents are ultimately responsible for explaining these benefits to consumers, this section will help agents guide consumers in making the best decisions to meet their health care needs.

According to the NAIC, "**Long-term care insurance**" means any insurance policy or rider advertised, marketed, offered or designed to provide coverage for not less than twelve (12) consecutive months for each covered person on an expense incurred, indemnity, prepaid or other basis; for one or more necessary or medically necessary diagnostic, preventive, therapeutic, rehabilitative, maintenance or personal care services, provided in a setting other than an acute care unit of a hospital.

The term includes group and individual annuities and life insurance policies or riders that provide directly or supplement long-term care insurance. The term also includes a policy or rider that provides for payment of benefits based upon cognitive impairment or the loss of functional capacity. The term also includes qualified long-term care insurance contracts.

Long-term care insurance may be issued by insurers; fraternal benefit societies; nonprofit health, hospital, and medical service corporations; prepaid health plans; health maintenance organizations or any similar organization to the extent they are otherwise authorized to issue life or health insurance.

Long-term care insurance will not include any insurance policy that is offered primarily to provide basic Medicare supplement coverage, basic hospital expense coverage, basic medical-surgical expense coverage, hospital confinement indemnity coverage, major medical expense coverage, disability income or related asset-protection coverage, accident only coverage, specified disease or specified accident coverage, or limited benefit health coverage.

With regard to life insurance, this term does not include life insurance policies that accelerate the death benefit specifically for one or more of the qualifying events of terminal illness, medical conditions requiring extraordinary medical intervention or permanent institutional confinement, and that provide the option of a lump-sum payment for those benefits and where neither the benefits nor the eligibility for the benefits is conditioned upon the receipt of long-term care.

Notwithstanding any other provision of this Act, any product advertised, marketed or offered as long-term care insurance will be subject to the provisions of the Act.

Location & Care Levels Benefits

Home Care - Home care refers to care given in the home by a person or professional company that the person must pay. Services include not only non-medical aid, daily living assistance, but also housekeeping, companionship, shopping, cooking, etc. This can be either in the senior's home or in a relative's home. A live-in companion/homemaker can also be provided, but the cost is higher and a higher level of skill is needed.

Nursing Facilities - Nursing homes include skilled nursing, intermediate nursing and custodial care. The custodial care is determined by a professional plan of care. Intermediate care provides shots, medications and therapy. Skilled nursing is for patients who need more complicated care provided by a licensed nurse. Convalescence from surgeries or serious illness fall under this definition.

Comprehensive Long-Term Care - This includes long-term care provided at home or in the community, as well as nursing facility care. These benefits are comprehensive because they provide all the services covered by home care and skilled nursing care

Long-term care insurance includes disability-based long-term care policies, but does not include insurance designed to provide Medicare supplement or major medical expense coverage.

Applicants can generally purchase Home Care Only, Nursing Home Care only, or Comprehensive Care.

Definition of Terms

Applicant means either of the following:

- In case of an individual long-term care insurance policy, the person who seeks to contract for benefits.
- In the case of a group long-term care insurance policy, the proposed certificate holder.

Certificate means any certificate issued under a group long-term care insurance policy, which policy to be delivered or issued for delivery in the state.

Exceptional increase means only those increases filed by an insurer as exceptional for

which the commissioner determines the need for the premium rate increase is justified:

- Due to changes in laws or regulations applicable to long-term care coverage in this state; or
- Due to increased and unexpected utilization that affects the majority of insurers of similar products.
- Except as provided in Section 20, exceptional increases are subject to the same requirements as other premium rate schedule increases.
- The commissioner may request a review by an independent actuary or a professional actuarial body of the basis for a request that an increase be considered an exceptional increase.
- The commissioner, in determining that the necessary basis for an exceptional increase exists, shall also determine any potential offsets to higher claims costs.

Incidental means that the value of the long-term care benefits provided is less than ten percent (10%) of the total value of the benefits provided over the life of the policy. These values will be measured as of the date of issue.

Qualified actuary means a member in good standing of the American Academy of Actuaries.

Similar policy forms means all of the long-term care insurance policies and certificates issued by an insurer in the same long-term care benefit classification as the policy form being considered. Certificates of groups that meet the definition of the NAIC Long-Term Care Model Act are not considered similar to certificates or policies otherwise issued as long-term care insurance, but are similar to other comparable certificates with the same long-term care benefit classifications. For purposes of determining similar policy forms, long-term care benefit classifications are defined as follows: institutional long-term care benefits only, non-institutional long-term care benefits only, or comprehensive long-term care benefits.

Policy Definitions

No long-term care insurance policy delivered or issued for delivery in this state will use the terms set forth below, unless the terms are defined in the policy and the definitions satisfy the following requirements:

- **"Activities of daily living"** means at least bathing, continence, dressing, eating, toileting and transferring.
- **"Acute condition"** means that the individual is medically unstable. Such an individual requires frequent monitoring by medical professionals, such as physicians and registered nurses, in order to maintain his or her health status.
- **"Adult day care"** means a program for six (6) or more individuals, of social and health-related services provided during the day in a community group setting for the purpose of supporting frail, impaired elderly or other disabled adults who can benefit from care in a group setting outside the home.
- **"Bathing"** means washing oneself by sponge bath; or in either a tub or shower, including the task of getting into or out of the tub or shower.
- **"Cognitive impairment"** means a deficiency in a person's short or long-term memory, orientation as to person, place and time, deductive or abstract reasoning, or judgment as it relates to safety awareness.
- **"Continence"** means the ability to maintain control of bowel and bladder function; or, when unable to maintain control of bowel or bladder function, the ability to perform associated personal hygiene (including caring for catheter or colostomy bag).
- **"Dressing"** means putting on and taking off all items of clothing and any necessary braces, fasteners or artificial limbs.
- **"Eating"** means feeding oneself by getting food into the body from a receptacle (such as a plate, cup or table) or by a feeding tube or intravenously.

- **"Hands-on assistance"** means physical assistance (minimal, moderate or maximal) without which the individual would not be able to perform the activity of daily living.
- **"Home health care services"** means medical and nonmedical services, provided to ill, disabled or infirm persons in their residences. Such services may include homemaker services, assistance with activities of daily living and respite care services.
- **"Medicare"** means "The Health Insurance for the Aged Act, Title XVIII of the Social Security Amendments of 1965 as Then Constituted or Later Amended," or "Title I, Part I of Public Law 89-97, as Enacted by the Eighty-Ninth Congress of the United States of America and popularly known as the Health Insurance for the Aged Act, as then constituted and any later amendments or substitutes thereof," or words of similar import.
- **"Mental or nervous disorder"** shall not be defined to include more than neurosis, psychoneurosis, psychopathy, psychosis, or mental or emotional disease or disorder.
- **"Personal care"** means the provision of hands-on services to assist an individual with activities of daily living.
- **"Skilled nursing care,"** "personal care," "home care," "specialized care," "assisted living care" and other services shall be defined in relation to the level of skill required, the nature of the care and the setting in which care must be delivered.
- **"Toileting"** means getting to and from the toilet, getting on and off the toilet, and performing associated personal hygiene.
- **"Transferring"** means moving into or out of a bed, chair or wheelchair.
- All providers of services, including but not limited to "skilled nursing facility," "extended care facility," "convalescent nursing home," "personal care facility," "specialized care providers," "assisted living facility," and "home care agency" shall be defined in relation to the services and facilities required to be available and the licensure, certification, registration or degree status of those providing or supervising the

services. When the definition requires that the provider be appropriately licensed, certified or registered, it shall also state what requirements a provider must meet in lieu of licensure, certification or registration when the state in which the service is to be furnished does not require a provider of these services to be licensed, certified or registered, or when the state licenses, certifies or registers the provider of services under another name.

Definition of Policy Terms - Additional

To ensure that all policies abide by the same terms and definitions, policy makers have defined the exact meaning for each. The terms and definitions must be defined in the policy and the definitions must be as follows:

“Medicare” is generally defined as “The Health Insurance for the Aged Act, Title XVIII of the Social Security Amendments of 1965 as then constituted or later amended”, or “Title I, Part I, of Public Law 89-97, as enacted by the 89th Congress of the United States of America and popularly known as the Health Insurance for the Aged Act, as then constituted and any later amendments or substitutes thereof,” or words of similar import.

“Skilled nursing care”, “intermediate care,” “home health care,” and other services are to be defined in relation to the level of skill required, the nature of the care and the setting which is required to be delivered.

The first level is twenty-four hour care in a skilled nursing facility. This type of facility is licensed by the department of health services and Medicaid certification is required. It can be paid for by Medicare, Medicaid and private insurance.

The second level, intermediate care, is also licensed by the departments of health services. Medicaid certification is required, but the level of health-care is not as extensive as skilled nursing care. Medicare, Medicaid and private insurance can also pay for intermediate care.

The third level of care is custodial care. Under this type of care, a doctor prescribes a plan of care and a nurse's aid in the care. Medicare does not cover it, but it can be covered by Medicaid.

Group Insurance

Group LTC insurance is a policy that is delivered or issued for delivery in the the various states and generally issued to any of the following:

Professional, trade or occupational association, employer, labor organization, or trust if it is composed of individuals actively engaged in the same profession or trade, or has been maintained in good faith for purposes other than obtaining insurance.

Associations or Trusts must be established or maintained for the benefit of members of one or more associations. Prior to advertising, marketing, or offering of policy or certificate, the association or insurer must file evidence with the insurance commissioner that the association has a minimum of 100 persons and has been organized and maintained in good faith for a purpose other than that of obtaining insurance, have been in active existence for at least one year, have a constitution and bylaws which provide all of the following and provide evidence that following have been consistently implemented (a) the association hold annual meetings (b) except for credit unions, the association or associations collect dues (c) the members have voting privileges and representation on the governing board and committees.

Associations must satisfy the above organizational requirements thirty days after filing, unless the commissioner makes a finding that the association does not satisfy those organization requirements.

Discretionary group is a group other than as described above and subject to all of the findings by the state's commissioner:

- The issuance of the group policy or certificate is not contrary to the best interest of the public.
- The issuance of the group policy will result in economies of acquisition or administration.
- The benefits are reasonable in relation to the premiums charged.
- The use of the true or fictitious name of the group, or group master policyholder, group policy, certificate, or any trust or other entity created or used for the marketing of the group policy or certificates is not deceptive or misleading with regard to the status, character, or proprietary or representative capacity of the insurer, group, trust, or other entity.
- The group's main revenue source is not related to the marketing of the insurance.
- The group's outreach method to obtain new members is not related to the solicitation of insurance.
- The group provides benefits or services other than insurance, of significant value to its members.

Unique Specifications

Some unique specifications of group insurance are that plans can not only include spouses, but also parents, retirees and parents in-law. The cost is usually less because the ages of the employees are younger. They do have a waiting period and pre-existing conditions, and benefits are the same as in individual policies. Inflation protection is offered as are nonforfeiture options. If a policy is replaced, the benefits must be the same; they cannot require different waiting periods or determine benefits on health or disability of person. They must also be guaranteed a renewable clause.

Example: Barry is a member of the Union Trust Association. He is interested in their group insurance policy, so he does some research to make sure they qualify. The Union Trust has been organized for five years and has 173 members all engaged in the same trade. Larry makes sure they have filed evidence with the insurance commissioner, have a constitution and bylaws, hold annual meetings and collect dues. The members have voting privileges and representation on the governing board and committees. The UnionTrust has been organized for the purpose of making sure their members have a valid contract with the trade company they work for so they are eligible for raises, sick leave, etc. Therefore, this group qualifies to provide insurance for their members. Barry was able to buy insurance for his family through their group policy.

Example: Wilbur belongs to a group called the “Workman’s Shopping Clinic” which is considered a “discretionary group.” This company has been organized for the purpose of getting lower prices for its members for cars, appliances, furniture, etc. The Clinic has a group policy, is not contrary to the best interest of the public, the benefits are fair in relation to the premium for membership and provides services of significant value to the members. This company also qualifies for the state’s definition of group insurance policies. Therefore, Wilbur can also buy a group policy for his family through the Workman’s Shopping Clinic.

The more the client knows about the policy benefits and features, as well as the limitations and fine points, the more they will be confident in their decision to take action and to be confident after their purchase.

Policy Definitions

"Policy" means, for the purposes of this Act, any policy, contract, subscriber agreement, rider or endorsement delivered or issued for delivery in this state by an insurer; fraternal benefit society; nonprofit health, hospital, or medical service corporation; prepaid health plan; health maintenance organization or any similar organization.

"Qualified long-term care insurance contract" or "federally tax-qualified long-term care insurance contract" means an individual or group insurance contract that meets the requirements of Section 7702B(b) of the Internal Revenue Code of 1986, as amended, as follows:

- The only insurance protection provided under the contract is coverage of qualified long-term care services. A contract shall not fail to satisfy the requirements of this subparagraph by reason of payments being made on a per diem or other periodic basis without regard to the expenses incurred during the period to which the payments relate;
- The contract does not pay or reimburse expenses incurred for services or items to the extent that the expenses are reimbursable under Title XVIII of the Social Security Act, as amended, or would be so reimbursable but for the application of a deductible or coinsurance amount. The requirements of this subparagraph do not apply to expenses that are reimbursable under Title XVIII of the Social Security Act only as a secondary payor. A contract shall not fail to satisfy the requirements of this subparagraph by reason of payments being made on a per diem or other periodic basis without regard to the expenses incurred during the period to which the payments relate;
- The contract is guaranteed renewable, within the meaning of section 7702B(b)(1)(C) of the Internal Revenue Code of 1986, as amended;
- The contract does not provide for a cash surrender value or other money that can be paid, assigned, pledged as collateral for a loan, or borrowed except as provided in [insert reference to state law equivalent to Section 4G(1)(e) of the Long-Term Care Insurance Model Act];
- All refunds of premiums, and all policyholder dividends or similar amounts, under the contract are to be applied as a reduction in future premiums or to increase future benefits, except that a refund on the event of death of the insured or a complete surrender or cancellation of the contract cannot exceed the aggregate premiums paid under the contract; and

- The contract meets the consumer protection provisions set forth in Section 7702B(g) of the Internal Revenue Code of 1986, as amended.
- "Qualified long-term care insurance contract" or "federally tax-qualified long-term care insurance contract" also means the portion of a life insurance contract that provides long-term care insurance coverage by rider or as part of the contract and that satisfies the requirements of Sections 7702B(b) and (e) of the Internal Revenue Code of 1986, as amended.

Please return to the Course Player in order to take Lesson 6 Quiz.

Lesson Seven

Activities of Daily Living

Triggers and Federal / State Differences

Long-term care policies require that one's physical or mental abilities be limited before benefits will be paid. These standards are often called Benefit Triggers. Many policies also require that additional conditions be met before they will receive payment. These "conditions" are events that must occur (or documents they must submit) after they meet the "benefit triggers" and before benefits will be paid.

One of the first questions a "lay" person will ask when attempting to understand how a LTC policy works is, "Under what circumstances does the insurance company begin paying benefits"? The answer is fairly straightforward. However, there are differences when considering tax deductible and non-tax deductible policies (qualified vs. non-qualified). We take a closer look at the trigger issue here.

Standards for Benefit Triggers

According to NAIC Model Regulations, "a long-term care insurance policy shall condition the payment of benefits on a determination of the insured's ability to perform activities of daily living and on cognitive impairment. Eligibility for the payment of benefits shall not be more restrictive than requiring either a deficiency in the ability to perform not more than three (3) of the activities of daily living or the presence of cognitive impairment".

Activities of daily living must include at least the following as defined in the policy:

- Bathing;
- Continence;
- Dressing;

- Eating;
- Toileting; and
- Transferring.

Insurers may use activities of daily living to trigger covered benefits in addition to those above as long as they are defined in the policy.

An insurer may use additional provisions for the determination of when benefits are payable under a policy or certificate; however the provisions must not restrict, and are not in lieu of, the requirements.

For purposes of this section the determination of a deficiency will not be more restrictive than:

- Requiring the hands-on assistance of another person to perform the prescribed activities of daily living; or
- If the deficiency is due to the presence of a cognitive impairment, supervision or verbal cueing by another person is needed in order to protect the insured or others.

Assessments of activities of daily living and cognitive impairment must be performed by licensed or certified professionals, such as physicians, nurses or social workers.

Long-term care insurance policies must include a clear description of the process for appealing and resolving benefit determinations.

The **Non-Tax-Qualified Benefit Triggers** for many states generally include 7 ADLs, but not limited to are:

- **Eating**, which shall mean reaching for, picking up and grasping a utensil and cup; getting food on a utensil and bringing food, utensil and cup to mouth; manipulating food on plate; and cleaning face and hands as necessary following a meal.

- **Bathing**, which shall mean cleaning the body using a tub, shower, or sponge bath, including getting a basin of water, managing faucets, getting in and out of tub or shower, and reaching head and body parts for soaping, rinsing and drying.
- **Dressing**, which shall mean putting on, taking off, fastening, and unfastening garments, undergarments and special devices such as back or leg braces, corsets, elastic stocking or garments and artificial limbs or splints.
- **Toileting**, which shall mean getting on and off a toilet or commode and emptying a commode, managing clothing and wiping and cleaning the body after toileting, and using and emptying a bedpan and urinal.
- **Transferring**, which shall mean moving from one sitting or lying position to another sitting or lying position; for example, from bed to or from a wheelchair or sofa, coming to a standing position, or repositioning to promote circulation and prevent skin breakdown.
- **Continence**, which shall mean the ability to control bowel and bladder as well as use ostomy or catheter receptacles, and apply diapers and disposable barrier pads.
- **Ambulating**, which shall mean walking or moving around inside or outside the home.

Generally, the insured needs to have impairment in 2 out of 6 or 7 ADLs for benefits to begin. There is also a benefit trigger which is “impairment of cognitive ability”. The term “impairment” means that the insured needs human assistance or continual substantial supervision; and “impairment of cognitive ability” means deterioration or loss of intellectual capacity due to organic mental disease, including Alzheimer’s disease or related illnesses that require continual supervision to protect oneself or others.

Most states need only a written notice from a doctor or other health professional to qualify for benefit triggers to begin.

The policy or certificate may provide for the lesser eligibility criteria. Insurance commissioners have the discretion to approve other criteria if the insurer demonstrates the interest of the insured is better served.

The Six Federally Tax-Qualified Benefit Triggers are:

The Federal Government has strict definitions for a “chronically ill person” which includes the inability to do at least two of the six benefit triggers listed below (for 90 days), or having a severe impairment in cognitive ability.

- ***Eating***, which means feeding oneself by getting food in the body from a receptacle (such as a plate, cup or table) or by a feeding tube or intravenously.
- ***Bathing***, which means washing oneself by sponge bath or in a tub or shower, including the act of getting into or out of a tub or shower.
- ***Continence***, which means the ability to maintain control of bowel and bladder functions; or when unable to maintain control of bowel or bladder functions, the ability to perform associated personal hygiene, including caring for a catheter or colostomy bag.
- ***Dressing***, which means putting on and taking off all items of clothing and any necessary braces, fasteners or artificial limbs.
- ***Toileting***, which means getting to and from the toilet, getting on or off the toilet, and performing the associated personal hygiene.
- ***Transferring***, which means the ability to move into or out of a bed, a chair or wheelchair.

The term “cognitive impairment” is defined as needing “substantial assistance, severe cognitive impairment, and substantial supervision”. IRS Notice 97-31 clarifies these terms as meaning:

- The standard for benefit triggers for qualified long-term insurance according to the National Association of Insurance Commissioners

(NAIC), is determined on the insured's inability to (a) perform two of the above listed ADLs or (b) having a severe impairment in cognitive ability and having a licensed health practitioner (independent of insurer) write a certification for the insured. This certification must be renewed every 12 months and submitted to the insurer and the insurer cannot deduct the cost of the certification from the policy *(Source: Long-Term Care Model Regulation, National Association of Insurance Commissioners)*.

The licensed health practitioner must be independent of the insurer, must submit a written plan of care after personally examining the insured. The definition of "independent of the insurer is defined as: "shall not be an employee of the insurer and not be compensated in any manner linked to the outcome of the certificate". The practitioner's assessments must be unhindered by financial considerations". Applies only to federally tax-qualified long-term insurance contract. A licensed health care practitioner is defined as a medical doctor, a registered nurse and licensed social worker.

Case management as it refers to "chronically ill certification" allows the insured to have a second assessment in the event the first practitioner feels the insured does not meet the definition of "chronically ill". This does not apply if the first practitioner meets the qualifications, who meets the requirements of this section, and who personally examined the insured.

Prior to the payment of benefits for any care covered by the terms of the policy, any insurer offering long-term care insurance may obtain a written declaration by a physician, independent needs assessment agency, or any other source of independent judgement suitable to the insurer that services are necessary.

Note: Please see an extended NAIC Model Regulation section in the Appendix section for more detailed information on this "Qualified" issue.

Assistance Levels

Substantial Assistance which means hands-on assistance and standby assistance.

Hands-on Assistance which means the physical assistance of another person without which the individual would not be able to perform the ADL.

Standby Assistance which means the presence of another person within arm's reach of the person that is necessary to prevent, by physical intervention, injury to the person while the individual is performing the ADL (such as being ready to catch the person if he or she falls while getting in or out of the bathtub or shower as part of bathing, or being ready to remove food from the person's throat if he chokes while eating).

Severe Cognitive Impairment which means a loss or deterioration in intellectual capacity that is comparable to and includes Alzheimer's disease and similar forms of irreversible dementia, and measured by clinical evidence and standardized tests that reliably measure impairment in the person's short or long-term memory, orientation to people, place or time and deductive or abstract reasoning.

Substantial Supervision, which means continual supervision (which may include cuing by verbal prompting, gestures or other demonstrations by another person that is necessary to protect the severely cognitively impaired individual from threats to his or her health or safety, such as wandering).

Federal and State Benefit Trigger Differences

Many states have seven activities of daily living for non-qualified policies, whereas federal law contains six for tax-qualified policies. At first glance, it would appear that federal ADLs are easier to trigger, but this is not necessarily true. Federal ADLs are more restrictive in that they require the policyholder to need assistance for 90 days or longer before benefits are paid, whereas states do not require this.

A point to emphasize is that federal law does not allow a less restrictive requirement for the inability to perform two of the ADLs if tax-qualification is desired. Federal law will also not allow anything other than severe cognitive impairment as an alternative trigger to the two ADLs requirement. In addition, a licensed health care professional must prepare a new certified plan of care every 12 months.

In contrast, state's non-qualified policies do not have the 90 day requirement. Activities of daily living are easier to meet, as well. If the Federal Government expands their benefit triggers requirements in any way, the state's departments of insurance must issue emergency regulations.

NOTE: It is extremely important for agents to understand the difference between federal and state benefit triggers so as to be able to explain them to their client. If the client does not understand these differences, he or she will be unable to make a decision as to which policy best suites his or her needs.

Despite fewer ADLs by the federal government, they are more restrictive and difficult to meet than state's ADL requirements. (More on this in Lesson 14)

Please return to the Course Player in order to take Lesson 7 Quiz.

Lesson Eight

Regulations and General Provisions

In addition to many of the specific categories relative to the fundamentals and characteristics of long-term care treated in other sections of your course, there are several other key areas that legislation and regulation addresses. Your acquaintance with and awareness of these following issues will continue to enlarge your competency levels with the many areas of long-term care and long-term care insurance. Check out several following from the NAIC Long-Term Care Insurance Act.

Authority to Promulgate Regulations

The commissioners will issue reasonable regulations to promote premium adequacy and to protect the policyholder in the event of substantial rate increases, and to establish minimum standards for producer education, marketing practices, producer compensation, producer testing, penalties and reporting practices for LTC insurance.

Administrative Procedures

Regulations adopted pursuant to the Act will be in accordance with the provisions of state insurance code relating to the adoption and promulgation of rules and regulations of the state's administrative procedures act.

Severability

If any provision of the Act or the application to any person or circumstance is for any reason held to be invalid, the remainder of the Act and the application of such provision to other persons or circumstances will not be affected.

Penalties

In addition to any other penalties provided by the laws of a state, any insurer and any producer found to have violated any requirement of the state relating to the regulation of

long-term care insurance or the marketing of LTC insurance will be subject to a fine of up to three (3) times the amount of any commissions paid for each policy involved in the violation or up to \$10,000, whichever is greater.

Disclosure and Performance Standards for Long-Term Care Insurance

The commissioner may adopt regulations that include standards for full and fair disclosure setting forth the manner, content and required disclosures for the sale of long-term care insurance policies, terms of renewability, initial and subsequent conditions of eligibility, non-duplication of coverage provisions, coverage of dependents, pre-existing conditions, termination of insurance, continuation or conversion, probationary periods, limitations, exceptions, reductions, elimination periods, requirements for replacement, recurrent conditions and definitions of terms.

This regulation permits the adoption of regulations establishing disclosure standards, renewability and eligibility terms and conditions, and other performance requirements for long-term care insurance. Regulations under this subsection should recognize the developing and unique nature of long-term care insurance and the distinction between group and individual long-term care insurance policies.

No long-term care insurance policy may:

- Be cancelled, non-renewed or otherwise terminated on the grounds of the age or the deterioration of the mental or physical health of the insured;
- Contain a provision establishing a new waiting period in the event existing coverage is converted to or replaced by a new or other form within the same company, except with respect to an increase in benefits voluntarily selected by the insured individual or group policyholder; or
- Provide coverage for skilled nursing care only or provide significantly more coverage for skilled care in a facility than coverage for lower levels of care.

Required Disclosure Provisions

Renewability

Individual long-term care insurance policies must contain a renewability provision. The provision must be appropriately captioned, must appear on the first page of the policy, and must clearly state that the coverage is guaranteed renewable or non-cancellable. This provision will not apply to policies that do not contain a renewability provision, and under which the right to non-renew is reserved solely to the policyholder.

A long-term care insurance policy/certificate, other than one the insurer does not have the right to change the premium, will include a statement that premium rates may change.

Riders and Endorsements

Except for riders or endorsements by which the insurer effectuates a request made in writing by the insured under an individual long-term care insurance policy, all riders or endorsements added to an individual long-term care insurance policy after the date of issue or at reinstatement or renewal that reduce or eliminate benefits or coverage in the policy will require signed acceptance by the individual insured. After the date of policy issue, any rider or endorsement which increases benefits or coverage with a concomitant increase in premium during the policy term must be agreed to in writing and signed by the insured, except if the increased benefits or coverage are required by law. Where a separate additional premium is charged for benefits provided in connection with riders or endorsements, the premium charge will be shown in the policy, rider or endorsement.

Payment of Benefits

A long-term care insurance policy that provides for the payment of benefits based on standards described as "usual and customary," "reasonable and customary" or words of similar import must include a definition of these terms and an explanation of the terms in its accompanying outline of coverage.

Limitations

If a long-term care insurance policy or certificate contains any limitations with respect to pre-existing conditions, the limitations must appear as a separate paragraph of the policy or certificate and must be labeled as "Pre-existing Condition Limitations."

Other Limitations or Conditions on Eligibility for Benefits

A long-term care insurance policy or certificate containing any limitations or conditions for eligibility other than those prohibited in state law corresponding to Section 62D(2) of the Long-Term Care Insurance Model Act must set forth a description of the limitations or conditions, including any required number of days of confinement, in a separate paragraph of the policy or certificate and must label such paragraph "Limitations or Conditions on Eligibility for Benefits."

Disclosure of Tax Consequences

With regard to life insurance policies that provide an accelerated benefit for long-term care, a disclosure statement is required at the time of application for the policy or rider and at the time the accelerated benefit payment request is submitted that receipt of these accelerated benefits may be taxable, and that assistance should be sought from a personal tax advisor. The disclosure statement must be prominently displayed on the first page of the policy or rider and any other related documents. This subsection does not apply to qualified long-term care insurance contracts.

Benefit Triggers

Activities of daily living and cognitive impairment must be used to measure an insured's need for long-term care and will be described in the policy or certificate in a separate paragraph and must be labeled "Eligibility for the Payment of Benefits." Any additional benefit triggers must also be explained in this section. If these triggers differ for different benefits, explanation of the trigger must accompany each benefit description. If an attending physician or other specified person must certify a certain level of functional dependency in order to be eligible for benefits, this too must be specified.

A qualified long-term care insurance contract must include a disclosure statement in the policy and in the outline of coverage that the policy is intended to be a qualified long-term care insurance contract under Section 7702B(b) of the Internal Revenue Code of 1986, as amended.

A nonqualified long-term care insurance contract must include a disclosure statement in the policy and in the outline of coverage that the policy is not intended to be a qualified long-term care insurance contract.

Required Disclosure of Rating Practices to Consumers

This applies to any long-term care policy or certificate issued in a state.

Other than policies for which no applicable premium rate or rate schedule increases can be made, insurers must provide all of the information required to the applicant at the time of application or enrollment, unless the method of application does not allow for delivery at that time. In such a case, an insurer must provide all of the information listed in this section to the applicant no later than at the time of delivery of the policy or certificate.

A statement that the policy may be subject to rate increases in the future;

- An explanation of potential future premium rate revisions, and the policyholder's or certificate holder's option in the event of a premium rate revision;
- The premium rate or rate schedules applicable to the applicant that will be in effect until a request is made for an increase;
- A general explanation for applying premium rate or rate schedule adjustments that will include:
 - A description of when premium rate or rate schedule adjustments will be effective (e.g., next anniversary date, next billing date, etc.); and
 - The right to a revised premium rate or rate schedule if the

premium rate or rate schedule is changed;

- Information regarding each premium rate increase on this policy form or similar policy forms over the past ten (10) years for this state or any other state that, at a minimum, identifies:
 - The policy forms for which premium rates have been increased;
 - The calendar years when the form was available for purchase; and
 - The amount or percent of each increase. The percentage may be expressed as a percentage of the premium rate prior to the increase, and may also be expressed as minimum and maximum percentages if the rate increase is variable by rating characteristics.
- The insurer may, in a fair manner, provide additional explanatory information related to the rate increases.
- An insurer will have the right to exclude from the disclosure premium rate increases that only apply to blocks of business acquired from other nonaffiliated insurers or the long-term care policies acquired from other nonaffiliated insurers when those increases occurred prior to the acquisition.
- If an acquiring insurer files for a rate increase on a long-term care policy form acquired from nonaffiliated insurers or a block of policy forms acquired from nonaffiliated insurers on or before the later of the effective date of this section or the end of a twenty-four-month period following the acquisition of the block or policies, the acquiring insurer may exclude that rate increase from the disclosure. However, the nonaffiliated selling company will include the disclosure of that rate increase.
- If the acquiring insurer files for a subsequent rate increase, even within the twenty-four-month period, on the same policy form acquired from nonaffiliated insurers or block of policy forms acquired from nonaffiliated insurers, the acquiring insurer shall make all disclosures required,

including disclosure of the earlier rate increase.

Information about past rate increases needs to be reviewed carefully. If the insurer expects to provide additional information (such as a brief description of significant variations in policy provisions if the form is not the policy form applied for by the applicant or information about policy forms offered during or before the calendar years of forms with rate increases), the commissioner should be satisfied that the additional information is fairly presented in relation to the information about rate increases.

States should be aware of and review situations where a group policy is no longer being issued but new certificates are still being added to existing policies.

- An applicant must sign an acknowledgement at the time of application, unless the method of application does not allow for signature at that time, that the insurer made the disclosure required. If due to the method of application the applicant cannot sign an acknowledgement at the time of application, the applicant must sign no later than at the time of delivery of the policy or certificate.
- An insurer must provide notice of an upcoming premium rate schedule increase to all policyholders or certificate holders, if applicable, at least forty-five (45) days prior to the implementation of the premium rate schedule increase by the insurer.

Initial Filing Requirements

An insurer must provide the information listed below to the commissioner 30 days prior to making a long-term care insurance form available for sale.

States should consider whether a time period other than 30 days is desirable. An alternative time period would be the time period required for policy form approval in the applicable state regulation or law.

- A copy of the disclosure documents

- An actuarial certification consisting of at least the following:
 - A statement that the initial premium rate schedule is sufficient to cover anticipated costs under moderately adverse experience and that the premium rate schedule is reasonably expected to be sustainable over the life of the form with no future premium increases anticipated;
 - A statement that the policy design and coverage provided have been reviewed and taken into consideration;
 - A statement that the underwriting and claims adjudication processes have been reviewed and taken into consideration;
 - A complete description of the basis for contract reserves that are anticipated to be held under the form, to include:
 - Sufficient detail or sample calculations provided so as to have a complete depiction of the reserve amounts to be held;
 - A statement that the assumptions used for reserves contain reasonable margins for adverse experience;
 - A statement that the net valuation premium for renewal years does not increase (except for attained-age rating where permitted); and
 - A statement that the difference between the gross premium and the net valuation premium for renewal years is sufficient to cover expected renewal expenses; or if such a statement cannot be made, a complete description of the situations where this does not occur;

When the difference between the gross premium and the renewal net valuation premiums is not sufficient to cover expected renewal expenses, the description provided

could demonstrate the type and level of change in the reserve assumptions that would be necessary for the difference to be sufficient.

- An aggregate distribution of anticipated issues may be used as long as the underlying gross premiums maintain a reasonably consistent relationship;
- If the gross premiums for certain age groups appear to be inconsistent with this requirement, the commissioner may request a demonstration based on a standard age distribution; and
- A statement that the premium rate schedule is not less than the premium rate schedule for existing similar policy forms also available from the insurer except for reasonable differences attributable to benefits; or
- A comparison of the premium schedules for similar policy forms that are currently available from the insurer with an explanation of the differences.

It is not expected that the insurer will need to provide a comparison of every age and set of benefits, period of payment or elimination period. A broad range of expected combinations is to be provided in a manner designed to provide a fair presentation for review by the commissioner.

The commissioner may request an actuarial demonstration that benefits are reasonable in relation to premiums. The actuarial demonstration must include either premium and claim experience on similar policy forms, adjusted for any premium or benefit differences, relevant and credible data from other studies, or both.

The commissioner may wish to have the actuarial demonstration reviewed by an independent actuary in those instances where the demonstration does not address fully the questions that triggered the request for the actuarial demonstration.

Extraterritorial Jurisdiction - Out-of-State Products

If another state offers group Long-Term Care insurance to residents of your state, that LTC insurance must follow the same restrictions and regulations that apply to the latter

state's rules. So, discretionary groups and association groups must have another purpose other than that of selling insurance and must follow state regulations.

Group Long-Term Care Insurance

No group long-term care insurance coverage may be offered to a resident of a state under a group policy issued in another state to a group, unless the state or another state having statutory and regulatory long-term care insurance requirements substantially similar to those adopted in this state has made a determination that such requirements have been met.

Disclosure of Tax-Qualified or Non-Tax-Qualified Policy

States that have tax-qualified policies

All policies must disclose whether they are tax-qualified or non-tax-qualified, according to both Federal and State rules and regulations and the disclosure must follow the exact sequence as below:

Disclosure must be in clear and concise form that indicates in large print (generally, no smaller than 12-point and preferably in bold) that the company has two types of LTC insurance that meet the requirements of the Health Insurance Portability and Accountability Act of 1996, Public Law 104-191, plus any other requirements enacted into the law.

There must be a notice separate from the basic coverage that states the company has two types of insurance policies and/or certificates of LTC that meet the qualification standards for both Federal and State tax benefits. A copy must be given to applicant.

If this is a group policy, generally, the employer must furnish all employees with this informal notice:

IMPORTANT NOTICE

THIS COMPANY OFFERS TWO TYPES OF LONG-TERM CARE INSURANCE
IN Name of State :

LONG-TERM CARE POLICIES (OR CERTIFICATES) INTENDED TO QUALIFY
FOR FEDERAL AND STATE OF Name of State TAX BENEFITS

AND

LONG-TERM CARE POLICIES (OR CERTIFICATES) THAT MEET Name of
State STANDARDS AND ARE NOT INTENDED TO QUALIFY FOR FEDERAL
OR STATE OF Name of State TAX BENEFITS, BUT WHICH MAY MAKE IT
EASIER TO QUALIFY FOR HOME CARE BENEFITS

Each of the Three Products Must Be Clearly Identified on Policy or Certificate

In most states, every policy or certificate that is “federally tax-qualified” according to the above requirements must have the words “**Federally Tax-Qualified**” prominently displayed on page one of the form and outline of coverage.

Frequently, any policy or certificate in which benefits are limited to the provision of institutional care must be called a “nursing facility and residential care facility only” policy or certificate and the words “**Nursing Facility and Residential Care Facility Only**” must be prominently display on page one of the form and the outline of coverage. The state’s commissioner may approve alternative working if it is more descriptive of the benefits.

Similarly, if there are limitations such as home care services, which include community-based services, it must be called a “**Home Care Only**” policy or certificate and this must be prominently displayed on page one of the form and outline of coverage.

Only those policies or certificates that have benefits for both institutional care and home care can be called “**Comprehensive Long-Term Care**” insurance and prominently displayed on page one of the form and outline of coverage.

The following chart illustrates the qualifying and non-qualifying of Federal and State policies and benefits.

Federal Tax-Qualifying Policy and Benefits	State Non-Qualifying Tax Policy and Benefits
If you cannot do two out of the following six activities of daily living and your doctor certifies you will be unable to do them for at least 90 days, then you qualify for home care services.	If you cannot do two out of the following seven activities of daily living, you qualify for home care services without needing the 90 day certification. The State does not have a 90 day qualification.
Eating, dressing, continence, toileting, transferring or bathing.	Eating, dressing, continence, toileting, transferring, bathing or ambulating.
The other condition with which you can get help is if you have <u>severe</u> cognitive impairment.	You can receive help if you have cognitive impairment, but it need not be as severe as with the federal stipulation.
Federal tax-qualifying policies do not pay for any services covered by Medicare, Medicare deductibles or co-payments.	Benefits for Medicare services can be used without limitations.

(Note: Insurers must give chart to applicant and employers must also give chart to their employees)

Post Claims Underwriting

The definition of Post Claims Underwriting is that an insurer attempts to get information about the health and/or functional capacity of the insured after a claim is filed. This is in order for the insurer to deny a claim or rescind a policy. **This is strictly prohibited.**

(Source: Section 9, National Association Insurance Commissioners, NAIC)

Prohibition Against Post-Claims Underwriting

All applications for long-term care insurance policies or certificates except those that

are guaranteed issue must contain clear and unambiguous questions designed to ascertain the health condition of the applicant.

If an application for long-term care insurance contains a question that asks whether the applicant has had medication prescribed by a physician, it must also ask the applicant to list the medication that has been prescribed.

If the medications listed in the application were known by the insurer, or should have been known at the time of application, to be directly related to a medical condition for which coverage would otherwise be denied, then the policy or certificate must not be rescinded for that condition.

Except for policies or certificates which are guaranteed issue:

The following language must be set out conspicuously and in close conjunction with the applicant's signature block on an application for a long-term care insurance policy or certificate:

Caution: If your answers on this application are incorrect or untrue, [company] has the right to deny benefits or rescind your policy.

The following language, or language substantially similar to the following, must be set out conspicuously on the long-term care insurance policy or certificate at the time of delivery:

Caution: The issuance of this long-term care insurance [policy] [certificate] is based upon your responses to the questions on your application. A copy of your [application] [enrollment form] [is enclosed] [was retained by you when you applied]. If your answers are incorrect or untrue, the company has the right to deny benefits or rescind your policy. The best time to clear up any questions is now, before a claim arises! If, for any reason, any of your answers are incorrect, contact the company at this address: [insert address]

Prior to issuance of a long-term care policy or certificate to an applicant age eighty (80) or older, the insurer must obtain one of the following:

- A report of a physical examination;
- An assessment of functional capacity;
- An attending physician's statement; or
- Copies of medical records.

A copy of the completed application or enrollment form (whichever is applicable) must be delivered to the insured no later than at the time of delivery of the policy or certificate unless it was retained by the applicant at the time of application.

Every insurer or other entity selling or issuing long-term care insurance benefits must maintain a record of all policy or certificate rescissions, both state and countrywide, except those that the insured voluntarily effectuated and will annually furnish this information to the insurance commissioner in the format prescribed by the National Association of Insurance Commissioners (See Appendix 11).

Applications

Health questions, Warning, Rescission Evidence, Delivery of Copy

Companies have different underwriting provisions, but all must follow their application regulations. Some companies require a stand alone application while others may require applications accompanied by a physician recommendation. Likewise, some companies need specifications such as a medical examination, while others only need a telephone interview.

There is one common denominator for federal and state applications. Health questions must be asked so only a “yes” or “no” answer is necessary.

There must be a clear warning on the application that states if there are any misstatements or false information or incomplete, this may result in rescission or withdrawal of the policy. Generally, the wording is as follows:

“CAUTION: If your answers on this application are misstated or untrue, or incomplete the insurer may have the right to deny benefits or rescind your coverage.”

Applications for long-term insurance should include a checklist that enumerates each of the specific documents which are to be given to the applicant at the time of solicitation. The documents and notices to be listed in the checklist include, but are not limited to:

- Important Notice Regarding Policies Available
- Outline of coverage
- Long-term Care Insurance Shopper’s Guide.
- “Long-Term Care Insurance Personal Worksheet”
- “Notice to Applicant Regarding Replacement of Accident and Sickness or long-term Care Insurance”, if applicable

If an insurer does not complete medical underwriting and resolve all reasonable questions arising from information submitted on or with an application before issuing the policy or certificate, an insurer may only rescind the policy or certificate or deny an otherwise valid claim, upon clear and convincing evidence of fraud or material misrepresentation of the risk by applicant. The evidence should:

- Pertain to the condition for which benefits are sought.
- Involve a chronic condition or involve dates of treatment before date of application.
- Be material to the acceptance for coverage.

No long-term care policy or certificate may be field issued. The contestability period is generally two years.

A copy of the completed application should be delivered to the insured at the time of delivery of the policy or certificate.

Every insurer must maintain a record of all policy or certificate rescissions, both state and countrywide, except those voluntarily initiated by the insured, and must annually furnish this information to their insurance commissioner in a format prescribed by that commissioner.

Limitations on LTC Insurance

There are certain limitations passed by most state legislatures that must be strictly followed by insurers. Generally we see that long-term care insurance may not:

- Be canceled, nonrenewable or otherwise terminated because of the age or deterioration of the mental or physical health of the insured.
- Have a provision for a new waiting period if the existing coverage is converted to, replaced by a new or other form within the same insurer, except there may be an increase in benefits if the insured individual or group policyholder consents to it.
- Provide coverage for skilled nursing care only or provide more coverage for skilled care in a facility than coverage for lower levels of care.
- Provide through a long-term care policy or certificate for payment of benefits based on a standard described as “usual and customary”, “reasonable and customary,” or words of similar import.
- Terminate a policy, certificate, or rider or contain a provision allowing premium for an in-force policy, certificate or rider, to be increased due to divorce of policyholder.
- Include additional benefit for service with known market value other than the statutorily required home & community-based service benefits, assisted living benefit or nursing facility benefit unless the additional benefit provides for the payment of at least five times the daily benefit and

dollar value and additional benefit is disclosed in schedule page of policy to explain maximum benefits.

Additional Requirements to Various Insurance Codes

No New Pre-existing Conditions on Replacement Policies

In most states, a new policy that replaces an old one has stringent rules that must be followed. For example, the insurer who is replacing the old policy must waive time periods that have already been met under the old policy. (Pre-existing condition is defined as any health problems that an applicant disclosed on original application. The time limitation is no later than 6 months after date of coverage of application.) If this has already been met, it cannot be included again on the new policy.) New pre-existing conditions are covered by state mandate through a paragraph in the policy, PRE-EXISTING CONDITIONS LIMITATIONS". The Code is generally quoted similarly and states the following:

"If a policy or certificate replaces another long-term care policy or certificate, the replacing insurer shall waive any time periods applicable to pre-existing conditions and probationary periods to the extent that similar exclusions have been satisfied under the original policy or certificate."

No Benefit Reductions for Out-Of-Pocket Expenditures

LTC insurance benefits may not be reduced because the insured or a family member or any other individual pays out-of-pocket expenditures for the insured. This means that if a family member or anyone else pays an expense for the insured, the insured is still allowed the full benefits of the LTC insurance policy. This could be an issue if the insurance company feels, if someone else has paid for a covered benefit expense, they should be able to take away that benefit from the insured.

Outline of Coverage (OOC)

An outline of coverage must be presented to the prospective applicant at the time when he or she is first solicited by the agent. This outline clearly states the purpose of the document and its contents. The OOC is very exact and must generally follow specific language.

This is true not only for individual policies, but also for group policies and provides for the following [*Required by State Law & National Association of Insurance Commissioners*] (NAIC):

Requirements:

- In the case of agent solicitations, an agent will deliver the outline of coverage prior to the presentation of an application or enrollment form.
- In the case of direct response solicitations, the outline of coverage will be presented in conjunction with any application or enrollment form.
- The outline of coverage will be a freestanding document using no smaller than 10-point type.
- The outline of coverage will contain no material of an advertising nature.
- Use of the text and sequence of the text of the outline of coverage set forth in this section is mandatory, unless otherwise specifically indicated. (Minor changes are O.K.)
- Text that is capitalized or underscored in the outline of coverage may be emphasized by other means that provide prominence equivalent to capitalization or underscoring.

The Outline of Coverage Includes:

- A description of the principal benefits and coverage provided in the policy;
- A statement of the principal exclusions, reductions and limitations contained in the policy;

- A statement of the terms under which the policy or certificate, or both, may be continued in force or discontinued, including any reservation in the policy of a right to change premium. Continuation or conversion provisions of group coverage must be specifically described;
- A statement that the outline of coverage is a summary only, not a contract of insurance, and that the policy or group master policy contains governing contractual provisions;
- A description of the terms under which the policy or certificate may be returned and premium refunded;
- A brief description of the relationship of cost of care and benefits; and
- A statement that discloses to the policyholder or certificate holder whether the policy is intended to be a federally tax-qualified long-term care insurance contract under 7702B(b) of the Internal Revenue Code of 1986, as amended.

Outline of Coverage Form

[COMPANY NAME]

[ADDRESS - CITY & STATE]

[TELEPHONE NUMBER]

LONG-TERM CARE INSURANCE

OUTLINE OF COVERAGE

[Policy Number or Group Master Policy and Certificate Number]

[Except for policies or certificates which are guaranteed issue, the following caution statement, or language substantially similar, must appear as follows in the outline of coverage.]

Caution: The issuance of this long-term care insurance [policy] [certificate] is based upon your responses to the questions on your application. A copy of your [application] [enrollment form] [is enclosed] [was retained by you when you applied]. If your answers are incorrect or untrue, the company has the right to deny benefits or rescind your policy. The best time to clear up any questions is now, before a claim arises! If, for any reason, any of your answers are incorrect, contact the company at this address: [insert address]

1. This policy is [an individual policy of insurance]([a group policy] which was issued in the

[indicate jurisdiction in which group policy was issued]].

2. **PURPOSE OF OUTLINE OF COVERAGE.** This outline of coverage provides a very brief description of the important features of the policy. You should compare this outline of coverage to outlines of coverage for other policies available to you. This is not an insurance contract, but only a summary of coverage. Only the individual or group policy contains governing contractual provisions. This means that the policy or group policy sets forth in detail the rights and obligations of both you and the insurance company. Therefore, if you purchase this coverage, or any other coverage, it is important that you **READ YOUR POLICY (OR CERTIFICATE) CAREFULLY!**

3. **FEDERAL TAX CONSEQUENCES.**

This [POLICY] [CERTIFICATE] is intended to be a federally tax-qualified long-term care insurance contract under Section 7702B(b) of the Internal Revenue Code of 1986, as amended.

OR

Federal Tax Implications of this [POLICY] [CERTIFICATE]. This [POLICY] [CERTIFICATE] is not intended to be a federally tax-qualified long-term care insurance contract under Section 7702B(b) of the Internal Revenue Code of 1986 as amended. Benefits received under the [POLICY] [CERTIFICATE] may be taxable as income.

4. **TERMS UNDER WHICH THE POLICY OR CERTIFICATE MAY BE CONTINUED IN FORCE OR DISCONTINUED.**

(a)[For long-term care health insurance policies or certificates describe one of the following permissible policy renewability provisions:

(1) Policies and certificates that are guaranteed renewable shall contain the following statement:] **RENEWABILITY: THIS POLICY [CERTIFICATE] IS GUARANTEED RENEWABLE.** This means you have the right, subject to the terms of your policy, [certificate] to continue this policy as long as you pay your premiums on time. [Company Name] cannot change any of the terms of your policy on its own, except that, in the future, **IT MAY INCREASE THE PREMIUM YOU PAY.**

(2)[Policies and certificates that are non-cancellable shall contain the following statement:] **RENEWABILITY: THIS POLICY [CERTIFICATE] IS**

NONCANCELLABLE. This means that you have the right, subject to the terms of your policy, to continue this policy as long as you pay your premiums on time. [Company Name] cannot change any of the terms of your policy on its own and cannot change the premium you currently pay. However, if your policy contains an inflation protection feature where you choose to increase your benefits, [Company Name] may increase your premium at that time for those additional benefits.

- (b) [For group coverage, specifically describe continuation/conversion provisions applicable to the certificate and group policy;]
- (c) [Describe waiver of premium provisions or state that there are not such provisions.]

5. TERMS UNDER WHICH THE COMPANY MAY CHANGE PREMIUMS.

[In bold type larger than the maximum type required to be used for the other provisions of the outline of coverage, state whether or not the company has a right to change the premium, and if a right exists, describe clearly and concisely each circumstance under which the premium may change.]

6. TERMS UNDER WHICH THE POLICY OR CERTIFICATE MAY BE RETURNED AND PREMIUM REFUNDED.

- (a) [Provide a brief description of the right to return—"free look" provision of the policy.]
- (b) [Include a statement that the policy either does or does not contain provisions providing for a refund or partial refund of premium upon the death of an insured or surrender of the policy or certificate. If the policy contains such provisions, include a description of them.]

7. THIS IS NOT MEDICARE SUPPLEMENT COVERAGE. If you are eligible for Medicare, review the Medicare Supplement Buyer's Guide available from the insurance company.

- (a) [For agents] Neither [insert company name] nor its agents represent Medicare, the federal government or any state government.
- (b) [For direct response] [insert company name] is not representing Medicare, the federal government or any state government.

8. LONG-TERM CARE COVERAGE. Policies of this category are designed to provide coverage for

one or more necessary or medically necessary diagnostic, preventive, therapeutic, rehabilitative, maintenance, or personal care services, provided in a setting other than an acute care unit of a hospital, such as in a nursing home, in the community or in the home.

This policy provides coverage in the form of a fixed dollar indemnity benefit for covered long-term care expenses, subject to policy [limitations] [waiting periods] and [coinsurance] requirements. [Modify this paragraph if the policy is not an indemnity policy.]

9. BENEFITS PROVIDED BY THIS POLICY.

- (a) [Covered services, related deductibles, waiting periods, elimination periods and benefit maximums.]
- (b) [Institutional benefits, by skill level.]
- (c) [Non-institutional benefits, by skill level.]
- (d) Eligibility for Payment of Benefits

[Activities of daily living and cognitive impairment shall be used to measure an insured's need for long-term care and must be defined and described as part of the outline of coverage.]

[Any additional benefit triggers must also be explained. If these triggers differ for different benefits, explanation of the triggers should accompany each benefit description. If an attending physician or other specified person must certify a certain level of functional dependency in order to be eligible for benefits, this too must be specified.]

10. LIMITATIONS AND EXCLUSIONS.

[Describe:

- (a) Pre-existing conditions;
- (b) Non-eligible facilities and provider;
- (c) Non-eligible levels of care (e.g., unlicensed providers, care or treatment provided by a family member, etc.);
- (d) Exclusions and exceptions;
- (e) Limitations.]

[This section should provide a brief specific description of any policy provisions which limit, exclude, restrict, reduce, delay, or in any other manner operate to qualify payment of the benefits described in Number 6 above.]

THIS POLICY MAY NOT COVER ALL THE EXPENSES ASSOCIATED WITH YOUR LONGTERM CARE NEEDS.

11. RELATIONSHIP OF COST OF CARE AND BENEFITS. Because the costs of long-term care services will likely increase over time, you should consider whether and how the benefits of this plan may be adjusted. [As applicable, indicate the following:

- (a) That the benefit level will not increase over time;
- (b) Any automatic benefit adjustment provisions;
- (c) Whether the insured will be guaranteed the option to buy additional benefits and the basis upon which benefits will be increased over time if not by a specified amount or percentage;
- (d) If there is such a guarantee, include whether additional underwriting or health screening will be required, the frequency and amounts of the upgrade options, and any significant restrictions or limitations;
- (e) And finally, describe whether there will be any additional premium charge imposed, and how that is to be calculated.]

12. ALZHEIMER'S DISEASE AND OTHER ORGANIC BRAIN DISORDERS.

[State that the policy provides coverage for insured's clinically diagnosed as having Alzheimer's disease or related degenerative and dementing illnesses. Specifically describe each benefit screen or other policy provision which provides preconditions to the availability of policy benefits for such an insured.]

13. PREMIUM.

- [(a) State the total annual premium for the policy;
- (b) If the premium varies with an applicant's choice among benefit options, indicate the portion of annual premium which corresponds to each benefit option.]

14. ADDITIONAL FEATURES.

[(a) Indicate if medical underwriting is used;

(b) Describe other important features.]

15. CONTACT THE STATE SENIOR HEALTH INSURANCE ASSISTANCE PROGRAM IF YOU HAVE GENERAL QUESTIONS REGARDING LONG-TERM CARE INSURANCE. CONTACT THE INSURANCE COMPANY IF YOU HAVE SPECIFIC QUESTIONS REGARDING YOUR LONG-TERM CARE INSURANCE POLICY OR CERTIFICATE.

Group Policy Certificates Required Disclosure

Certificates are long-term care coverage for groups. Many of the benefits and coverage are identical, but there are some differences. A certificate issued for group long-term care insurance has some of the same required disclosure rules as the Outline of Coverage for individual policies:

There must be a description of the principle benefits and coverage which includes:

- A statement of the principle exclusion, reductions and limitations in the policy.
- A statement of the terms under which the policy or certificate or both may be continued in force, or continued, including any reservation in the policy of the right to change premiums.
- A statement that the group master policy determines the governing contractual provisions.
- An explanation of the policyholder's rights regarding continuation, conversion, or replacement.

Requirement to Deliver Buyer's Guide

A long-term care insurance buyer's guide in the format developed by the National Association of Insurance Commissioners, or a guide developed or approved by the

commissioner, must be provided to all prospective applicants of a long-term care insurance policy or certificate.

In the case of agent solicitations, an agent must deliver the shopper's guide prior to the presentation of an application or enrollment form.

In the case of direct response solicitations, the shopper's guide must be presented in conjunction with any application or enrollment form.

Life insurance policies or riders containing accelerated long-term care benefits are not required to furnish the above-referenced guide, but must furnish the policy summary.

Definition for Pre-existing Conditions

No long-term care insurance policy or certificate, other than a group policy or certificate, should or do use a definition of pre-existing condition which is more restrictive than a condition for which medical advice or treatment was recommended by, or received from a provider of health care services, within six months preceding the effective date of coverage of an insured person.

Generally, long-term care insurance policies or certificates cover pre-existing conditions that are disclosed on the application no later than six months following the effective date of the coverage of an insured, regardless of the date the loss or confinement begins.

The definition of pre-existing condition does not prohibit an insurer from using an application form designed to elicit the complete health history of an applicant and on the basis of the complete health history of the answers on that application, from underwriting in accordance with insurer's established underwriting standards. Further, unless otherwise provided in the policy or certificate, a pre-existing condition, regardless of whether it is disclosed on the application, need not be covered until the waiting period described expires. Unless a waiver or rider has been specifically approved by the commissioner, no long-term insurance policy or certificate normally exclude or use

waivers or riders of any kind to exclude, limit, or reduce coverage or physical conditions beyond the waiting period.

Not all insurers have this restriction. If there is a restriction or limitation clause, those states mandate that it must be clearly written in a paragraph in the policy as “pre-existing conditions limitations.”

Insurers may ask health questions on an application so as to determine the true health condition of the applicant. Most insurance commissions have the option to extend the limitation period for specific age group categories in a specific policy if there are findings that the extension is in the best interest of the public.

Prior Hospitalization or Institutionalization Eliminated

States do not allow long-term care insurance to be issued that is based on the following:

- Preconditions the availability of benefits on prior hospitalization.
- Conditions eligibility for benefits provided in an institutional care setting on the receipt of a higher level of institutional care.
- Preconditions the availability of benefits for community-based care, home health care or home care on prior institutionalization.
- Conditions eligibility for noninstitutional benefits, other than those above on a prior institutional stay of more than 30 days.

Policy Summary

In many states, if an application for a long-term care insurance contract or certificate is approved, the issuer shall deliver the contract or certificate of insurance to the applicant no later than thirty (30) days after the date of approval.

At the same time, in many states, at the time of policy delivery, a policy summary must be delivered for an individual life insurance policy that provides long-term care benefits within the policy or by rider. In the case of direct response solicitations, the insurer will

deliver the policy summary upon the applicant's request, but regardless of request will make delivery no later than at the time of policy delivery. In addition to complying with all applicable requirements, the summary must also include:

- An explanation of how the long-term care benefit interacts with other components of the policy, including deductions from death benefits;
- An illustration of the amount of benefits, the length of benefit, and the guaranteed lifetime benefits if any, for each covered person;
- Any exclusions, reductions and limitations on benefits of long-term care;
- A statement that any long-term care inflation protection option required by state's inflation protection option requirement comparable to the Long-Term Care Insurance Model Regulation is not available under this policy;
- If applicable to the policy type, the summary must also include:
 - A disclosure of the effects of exercising other rights under the policy;
 - A disclosure of guarantees related to long-term care costs of insurance charges; and
 - Current and projected maximum lifetime benefits.

The provisions of the policy summary listed above may be incorporated into a basic illustration required to be delivered in accordance with the state's basic illustration requirement comparable to Sections 6 and 7 of the Life Insurance Illustrations Model Regulation or into the life insurance policy summary which is required to be delivered in accordance with the state's life insurance policy summary requirement comparable to Section 5 of the Life Insurance Disclosure Model Regulation.

Life Insurance LTC Benefit Report

Any time a long-term care benefit, funded through a life insurance vehicle by the acceleration of the death benefit, is in benefit payment status, a monthly report must be

provided to the policyholder. The report must include:

- Any long-term care benefits paid out during the month;
- An explanation of any changes in the policy, e.g. death benefits or cash values, due to long-term care benefits being paid out; and
- The amount of long-term care benefits existing or remaining.

Regulations for Loss-Ratio Standards

Insurance commissioners may adopt regulations establishing loss ratio standards for long-term care insurance policies provided a specific reference to long-term care insurance policies is contained in the regulation. If the expected loss ratio is at least 60% in relation to the premiums, this is considered reasonable if consideration is given to factors such as:

- Interest
- Claim fluctuations
- Policy reserves
- Statistical credibility of incurred claims experience and earned premiums.
- The period for which rates are computed to provide coverage
- Experience refunds, adjustments or dividends.
- Renewability features
- All appropriate expense factors
- Experimental nature of coverage
- Mix of business by risk classification
- Concentration of experience, deductibles, and high maximum limits

Any regulation adopted by the commissioners generally substantially reflect the loss ratio standards contained in Section 10 of the National Association of Insurance Commissioners Long-Term Insurance Model Regulations.

Incontestability Period

For a policy or certificate that has been in force for less than six (6) months an insurer may rescind a long-term care insurance policy or certificate or deny an otherwise valid long-term care insurance claim upon a showing of misrepresentation that is material to the acceptance for coverage.

For a policy or certificate that has been in force for at least six (6) months but less than two (2) years an insurer may rescind a long-term care insurance policy or certificate or deny an otherwise valid long-term care insurance claim upon a showing of misrepresentation that is *both* material to the acceptance for coverage *and* which pertains to the condition for which benefits are sought.

After a policy or certificate has been in force for two (2) years it is not contestable upon the grounds of misrepresentation alone; such policy or certificate may be contested only upon a showing that the insured knowingly and intentionally misrepresented relevant facts relating to the insured's health.

- A long-term care insurance policy or certificate may be field issued if the compensation to the field issuer is not based on the number of policies or certificates issued.
- For purposes of this section, "field issued" means a policy or certificate issued by a producer or a third-party administrator pursuant to the underwriting authority granted to the producer or third party administrator by an insurer and using the insurer's underwriting guidelines.

If an insurer has paid benefits under the long-term care insurance policy or certificate, the benefit payments may not be recovered by the insurer in the event that the policy or certificate is rescinded.

In the event of the death of the insured, this section will not apply to the remaining death benefit of a life insurance policy that accelerates benefits for long-term care. In this

situation, the remaining death benefits under these policies will be governed by a state's life insurance incontestability clause. In all other situations, this section will apply to life insurance policies that accelerate benefits for long-term care.

No Obligation Thirty-Day Review

A prospective client for a long-term care insurance policy or certificate (not group certificate) has the right to review the policy for thirty days in most states. If he or she wishes to return it within that time, the insurer must refund the premiums paid.

The return of the policy normally voids the policy or certificate as if it had never been issued. A notice must be printed on the first page of the policy or certificate stating the conditions described above.

Right to Return - Free Look

According to NAIC Model Regulations, Long-term care insurance applicants must have the right to return the policy or certificate within thirty (30) days of its delivery and to have the premium refunded if, after examination of the policy or certificate, the applicant is not satisfied for any reason. Long-term care insurance policies and certificates must have a notice prominently printed on the first page or attached stating in substance that the applicant will have the right to return the policy or certificate within thirty (30) days of its delivery and to have the premium refunded if, after examination of the policy or certificate, other than a certificate issued pursuant to a policy issued to a group defined in Section 4E(1) of this Act, the applicant is not satisfied for any reason. This subsection will also apply to denials of applications and any refund must be made within thirty (30) days of the return or denial.

Penalties

In addition to any other penalties provided by the laws of the individual states, any insurer and any agent found to have violated any requirement of the states relating to the regulation of long-term care insurance or the marketing of such insurance will be

subject to a fine of up to three (3) times the amount of any commissions paid for each policy involved in the violation or up to \$10,000, whichever is greater.

The intent of this component is to authorize separate fines for both the company and the agent in the amounts suggested above.

Permitted Compensation Arrangements

According to the NAIC Model Regulations, an insurer or other entity may provide commission or other compensation to an agent or other representative for the sale of a long-term care insurance policy or certificate only if the first year commission or other first year compensation is no more than 200 percent of the commission or other compensation paid for selling or servicing the policy or certificate in the second year or period.

The commission or other compensation provided in subsequent (renewal) years must be the same as that provided in the second year or period and must be provided for a reasonable number of renewal years.

No entity may provide compensation to its agents or other producers and no agent or producer may receive compensation greater than the renewal compensation payable by the replacing insurer on renewal policies.

For purposes of this section, "compensation" includes pecuniary or non-pecuniary remuneration of any kind relating to the sale or renewal of the policy or certificate including but not limited to bonuses, gifts, prizes, awards and finder's fees.

The NAIC recognizes that long-term care insurance is in an evolutionary stage. The product market needs to be able to develop in order to be responsive to the needs of consumers. In addition, since long-term care insurance and long-term care insurance regulations are continually changing, a state should consider the fact that not all replacements are improper.

The NAIC also recognizes that currently, long-term care insurance products are being primarily sold to the senior citizen market, a market that has been identified as being susceptible to abusive marketing practices. In response, the NAIC has adopted consumer protection amendments in its model regulation for Medicare supplement insurance. The Medicare supplement insurance model regulation limits agents' compensation in order to address the potential for marketing abuses resulting from the large difference between first year and renewal commissions.

If a state believes that there is evidence that the long-term care insurance market is experiencing similar abuses, it may wish to consider adopting the optional agent compensation provision above.

In considering these agent compensation limitations, states should recognize the emerging nature of the long-term care insurance market. Long-term care insurance is evolving along both health insurance indemnity and life insurance lines. A state may want to consider that, since life insurance products usually contain nonforfeiture and cash value accumulation features and are normally targeted to a younger age group than long-term care indemnity products, such life insurance products could be exempted from these compensation limitation requirements.

The compensation provision such as provided above should not be enacted in lieu of the penalty and other consumer protection provisions contained in the regulation, but in addition to them.

Additional Information for Agents

In most states, producers must be aware that newer policies cannot require the use of a state-licensed provider unless the state also requires a license for those providers. Insurance companies can make exceptions when care specified in the policy can be delivered and often for less money even though in a place not specifically described in the contract.

Producers must be taught to read older policies and understand why the services may be more restrictive than those described in the newer policies. They should be able to explain this when an older policy is replaced and to accurately identify the reason for replacement and whether it constitutes a material improvement in the agent certification statement on the application.

Please return to the Course Player in order to take Lesson 8 Quiz.

Lesson Nine

Minimum Standards for Home Care

In the early 1990's, long-term care insurance covering care at home was not prevalent. Most early coverage focused entirely on nursing home benefits. With the high percentage of people needing home care as a real alternative to nursing home care, the landscape of policy coverages changed to where they are today. Many alternatives to nursing home care have evolved even above and beyond home care.

Mandated Elements of Home Care

"Long-term care insurance" is normally defined to include any insurance policy, certificate or rider advertised, marketed, offered, solicited or designed to provide coverage for diagnostic, preventive, therapeutic, rehabilitative, maintenance or personal care services which are provided in a setting other than an acute care unit of a hospital. Long-term care insurance includes all products containing any of the following benefit types: coverage for institutional care, including care in a nursing home, convalescent facility, extended care facility, custodial care facility, skilled nursing facility, or personal care home; home care coverage including home health care, personal care, homemaker services, hospice or respite care; or community-based coverage including adult day care, hospice, or respite care.

Long-term care insurance includes disability based long-term care policies, but does not include insurance designed primarily to provide Medicare supplement or major medical expense coverage.

Insurance Commissioners review and approve individual and group policies, certificates, riders, and outlines of coverage. Long-term care benefits are designed to provide coverage of 12 months or more that are contained in or amended to Medicare supplement or other disability policies and certificates.

The mandated services below are described by and have been issued as policies by most states and must be included and described in clear detail in any long-term care insurance policy.

Six Commonly Mandated Elements of Home Care

Home Health Care is skilled nursing or other professional services in the residence, including, but not limited to, part-time and intermittent skilled nursing services, home health aide services, physical therapy, occupational therapy or speech therapy, audiology services and medical social services by a social worker or social work assistant.

Adult Day Care is medical or nonmedical care provided on a less than 24-hour basis in a licensed facility outside the residence. It is for persons in need of personal services, supervision, protection or assistance in sustaining daily needs, including eating, bathing, dressing, ambulating, transferring, toileting and taking medications.

Personal Care is assistance with the activities of daily living, including instrumental activities of daily living, provided by a skilled or unskilled person under a plan of care developed by a physician or a multidisciplinary team under medical direction. Instrumental activities include using the telephone, managing medication, moving around outside, shopping, preparing meals, laundry and light housekeeping.

Homemaker Services is assistance with activities necessary for a person to remain in his or her residence. The service is provided by skilled or unskilled personnel under the direction of a physician or multi-disciplinary medical team.

Hospice Services are outpatient services not paid by Medicare that are designed to provide palliative care to alleviate the physical, emotional, social and spiritual discomforts of an individual who is experiencing the last phases of life due to the existence of a terminal disease. The service also provides support to the primary

caregiver and family. Care may be provided by a skilled or unskilled person under a plan of care developed by a physician or a multidisciplinary team under medical direction.

Respite Services is short-term care provided in an institution, in the home or in a community-based program, that is designed to relieve a primary caregiver in the home. This is a separate benefit with its own condition for eligibility and maximum benefit levels.

In every long-term policy or certificate that provides home care benefits, the threshold establishing eligibility for home care benefits is that the insured will qualify if either one of two criteria are met:

- Impairment in two activities of daily living
- Impairment of cognitive ability

Policies may provide for the lesser, not the greater eligibility criteria.

Minimum Standards for Home Health and Community Care Benefits

In many states, home care benefits cannot be limited by any of the following:

- By requiring that the insured or claimant would need care in a skilled nursing facility if home health care services were not provided;
- By requiring that the insured or claimant first or simultaneously receive nursing or therapeutic services, or both, in a home, community or institutional setting before home health care services are covered;
- By limiting eligible services to services provided by registered nurses or licensed practical nurses;
- By requiring that a nurse or therapist provide services covered by the policy that can be provided by a home health aide, or other licensed or certified home care worker acting within the scope of his or her licensure

or certification;

- By excluding coverage for personal care services provided by a home health aide;
- By requiring that the provision of home health care services be at a level of certification or licensure greater than that required by the eligible service;
- By requiring that the insured or claimant have an acute condition before home health care services are covered;
- By limiting benefits to services provided by Medicare-certified agencies or providers; or
- By excluding coverage for adult day care services.

A long-term care insurance policy or certificate, if it provides for home health or community care services, shall provide total home health or community care coverage that is a dollar amount equivalent to at least one-half of one year's coverage available for nursing home benefits under the policy or certificate, at the time covered home health or community care services are being received. This requirement will not apply to policies or certificates issued to residents of continuing care retirement communities.

Home health care coverage may be applied to the non-home health care benefits provided in the policy or certificate when determining maximum coverage under the terms of the policy or certificate.

Comprehensive Policy Defined

A comprehensive long-term care policy has both institutional care and home care. It has daily, weekly or monthly benefit payment maximums. A maximum benefit payment for home care is at least 50% of the maximum benefit payment for institutional care, but home care benefits are not to be paid at less than a specific amount per day, for example fifty dollars (\$50). Residents in continuing care retirement communities are generally exempt from this provision.

A policy or certificate that covers a nursing facility reimbursement can also cover and reimburse for per diem expenses, as well as costs for ancillary supplies and services up to the maximum lifetime daily facility benefit of the policy.

State-Licensed Providers

In most states, long-term care providers that are eligible to be state-licensed are:

- Skilled nursing facilities
- Intermediate care nursing facilities
- Nursing Home/Residential facility

The responsibility of the agent and insurer is to know what facilities must be licensed, the certification requirements, and their relationship to coverages and benefits specified in the long-term care insurance policy that they sell.

Policies cannot require the use of a state-licensed provider unless the state has also required a license for that provider. Namely, the policy can't require that a home health care agency be licensed unless the state has licensed home health care agencies.

Use of Terms Being Defined in the Policy

No long-term care insurance policy delivered or issued for delivery in most states may use the terms below, unless the terms are defined in the policy and the definitions satisfy the following requirements:

- "Medicare" is defined as "The Health Insurance for the Aged Act Title XVIII of the Social Security Amendments of 1965 as then constituted or later amended, "Title I, Part I of Public Law 89-97, as enacted by the 89th Congress of the United States of America and popularly known as the Health Insurance for the Aged Act, as then constituted and any later amendments or substitutes thereof", or words of similar import.

- “Cognitive impairment” may not be defined to include more than neurosis, psychoneurosis, psychopathy, psychosis, or mental or emotional disease or disorder.
- “Skilled nursing care”, “intermediate care”, home health care”, and other services must be defined in relation to the level of skill required, the nature of the care and the setting in which care is required to be delivered.
- All providers of services, including, but not limited to, skilled nursing facilities, intermediate care facilities, home health agencies must be defined in relation to the services and facilities required to be available and the licensure or degree status of those providing or supervising the services. The definition may require that the provider be appropriately licensed or certified.

Terms must be defined precisely in the policy because definitions, other than those specified, can be misinterpreted by the consumer.

Potential Tax Consequences if Pre -1997 Insured Requests “Material Modification”

When the federal government passed the Health Insurance Portability and Accountability Act (HIPAA) in 1996 it stated that all long-term policies issued before January 1, 1997 that met State requirements would be considered qualified and had the same tax advantages as those issued after January 1, 1997. There was one stipulation that stated if the contract had any “material change” it would automatically be considered a new contract. This new contract had to meet all of the qualifications and requirements or it would be considered non-tax-qualified.

The producer should discuss this with the client to make sure he or she understands the potential loss of benefits if there are material changes as defined by HIPAA. Material changes are defined as anything that changes the amount or timing of a benefit or premium (including nonforfeiture benefits). Any of these changes are considered a new

contract with the exception of an option or right a policyowner had under contract as of December 31, 1996.

In some states, if the producer does not adequately explain the ramifications of material modification to the client, that producer can be fined \$100 a day per client if the LTC policies do not meet the qualifications of both HIPAA and the NAIC Model Act and Regulations Act.

Insurers May Verify Necessity with Any Independent Judgement

Insurers have the option to get a written declaration by a physician, an independent needs assessment agency or any other source of independent judgement before paying benefits for any care covered by the policy.

Benefits for LTC Contracts

There are two benefit period designs:

The first type is for a certain **Time Period** in months or years, i.e. 1, 2, 5 years. There is also an unlimited benefit period for policies, but the longer the benefit period the higher the cost. This type pays a certain amount per day, i.e. \$150. Benefits stop at end of the period of time. The amount is different for nursing home care, home health care, adult day care, etc. per policy specifications.

The second type of benefit period is not for months or years, but a **Monetary Amount**, i.e., \$54,750 which is \$150 a day for 365 days. There is the specific aggregate “pool” amount of money reserved regardless of any time period. Benefits are paid until the money is used up. The benefit amount is not specified based on the level of care received, but only on the total aggregate amount of money reserved for the insured. If the insured requires only \$125 a day, the \$25 a day surplus makes money available beyond the 1-year period and can be used until the \$54,750 is used up.

NOTE: For some states, there is a word of caution for the comprehensive long-term care policy that has a specified time or maximum duration for institutional care, with a limited length of time for benefits received. There must be a similar maximum duration for home care that is at least 50% of the length of time for institutional care.

Reserve Standards

When long-term care benefits are provided through the acceleration of benefits under group or individual life policies or riders to such policies, policy reserves for the benefits will be determined in accordance with standard valuation laws for life insurance, which contains a section referring to "special benefits" for which tables must be approved by the commissioner. Claim reserves will also be established in the case when the policy or rider is in claim status.

Reserves for policies and riders subject to this section should be based on the multiple decrement model utilizing all relevant decrements except for voluntary termination rates. Single decrement approximations are acceptable if the calculation produces essentially similar reserves, if the reserve is clearly more conservative, or if the reserve is immaterial. The calculations may take into account the reduction in life insurance benefits due to the payment of long-term care benefits. However, in no event may the reserves for the long-term care benefit and the life insurance benefit be less than the reserves for the life insurance benefit assuming no long-term care benefit.

In the development and calculation of reserves for policies and riders subject to these regulations, due regard must be given to the applicable policy provisions, marketing methods, administrative procedures and all other considerations which have an impact on projected claim costs, including, but not limited to, the following:

- Definition of insured events;
- Covered long-term care facilities;
- Existence of home convalescence care coverage;
- Definition of facilities;

- Existence or absence of barriers to eligibility;
- Premium waiver provision;
- Renewability;
- Ability to raise premiums;
- Marketing method;
- Underwriting procedures;
- Claims adjustment procedures;
- Waiting period;
- Maximum benefit;
- Availability of eligible facilities;
- Margins in claim costs;
- Optional nature of benefit;
- Delay in eligibility for benefit;
- Inflation protection provisions; and
- Guaranteed insurability option.

Any applicable valuation morbidity table must be certified as appropriate as a statutory valuation table by a member of the American Academy of Actuaries.

When long-term care benefits are provided other than as in the section above, reserves will be determined in accordance with state law equivalent to the most recent version of the NAIC Minimum Reserve Standards for Individual and Group Health Insurance Contracts.

HIPAA applies the reserve method to qualified long-term care contracts that is applied to all insurance contracts except life insurance contracts, annuity contracts, or non-cancellable accident and health contracts.

Loss Ratio

Generally, in relation to premiums, benefits for individuals are reasonable if the expected loss ratio is at least 60%, calculated in a manner that provides for adequate

reserving of the long-term care risk. To evaluate the expected loss ratio, consideration will be given to the following:

- Period for which rates are computed to provide coverage.
- Experience and project trends.
- Concentration of experience within early policy duration.
- Expected claim fluctuation.
- Experience refunds, adjustments or dividends.
- Renewability features.
- All appropriate expense factors.
- Interest.
- Experimental nature of coverage.
- Policy reserves.
- Mix of business by risk classification.
- Product features, such as long elimination periods, high deductibles and high maximum limits.

In other words, this is making sure there will be adequate funds available for all losses no matter what unforeseen circumstances may arise. It has been established that expected loss ratio is 60% in relation to premiums, which provides for a reserve for long-term care risk.

Premium Rate Schedule Increases

An insurer must provide notice of a pending premium rate schedule increase, including an exceptional increase, to the commissioner generally at least 30 days prior to the notice to the policyholders and must include a number of requirements too numerous for discussion here. (These can be found in Appendix 14).

Filing Requirement

Prior to an insurer or similar organization offering group long-term care insurance to a resident of this state pursuant to cite state law equivalent to Section 5 of the Long-Term

Care Insurance Model Act, it must file with the commissioner evidence that the group policy or certificate has been approved by a state having statutory or regulatory long-term care insurance requirements substantially similar to those adopted in this state.

Elimination Periods

Both group and individual long-term care policies/certificates have elimination or waiting periods (deductible in an individual policy). There are 30, 60, 90, etc. elimination days. In some policies, if no medical care is received for a day, that day is not counted even though it is in the middle of a time frame for which care is given for a medical event. This is important for a policyholder to know as the number of elimination days has to be met to trigger a benefit. Group policies usually have one choice. These waiting or deductible periods often can be different for nursing home care, home care or community-care. The shorter the waiting period, the higher the premium...and vice versa.

Case Management

Case management, or managed care as it is called today, is described in the early long-term care policies as: "...a complex of services which include an individual, comprehensive client assessment, service arrangements for clients, monitoring and follow-up." States may often provide these services through their agencies on aging or similar agencies in each community. These services include comprehensive assessment; individual care planning, coordination, and monitoring of care plans. Case management can be itemized for tax purposes in qualified policies.

Case management or managed care is important in the long-term care arena because most clients do not know all the options regarding various providers and the levels of health care that can be offered. A producer must familiarize himself with all healthcare levels because provider networks can be established to give the client the level of care needed. A good case manager can develop a plan that will have an economical cost benefit for both provider and client.

The case manager will merge the needs of the insured to that of a suitable provider. This puts a responsibility on the insurance producer to decide if the type of managed care offered will benefit his client and how different types of policies will be affected by managed care, especially if his client has a tax-qualified policy.

The requirements for a plan of care must take the best interests of the insured into consideration. The best health care providers must be considered, as well as how economically feasible it is for both insured and provider.

Managed care with a skilled manager who knows where to get inexpensive and reliable care will help control long-term care costs. Producers should be knowledgeable about managed long-term care, how the policies they are selling can be affected by managed care, and if this care will meet the needs of the client.

If the managed care is a 'fee-for-service' situation, an independent assessment is made. If the client has a tax-qualified policy, certain fees and costs can be deducted on Schedule A if the policy meets the IRS conditions. If a taxpayer does not itemize on his tax return, the tax benefits will not be an advantage. Many people fall in the low income tax bracket; therefore, a tax-qualified policy would not be necessary.

Optional Benefits

Insurers offer various benefits with long-term care policies. Not everyone is interested in these specialized options. It would be wise for the producer to question his or her client to see if they are interested in these extras. Optional benefits by various insurance companies are presented below. Some can be purchased and others cannot (i.e., return of premium).

Indemnity Contracts/Integrated Contracts

Long-Term Care Indemnity Contracts, referring to the loss of health of an older individual can have several different levels of coverage such as home care, nursing

care and residential care. Nursing care and residential care are integrated into one contract and the policy or certificate must have “Nursing Facility and Residential Care Facility Only” prominently displayed on page one of the policy form and the outline of coverage. These are also called comprehensive policies. Policies or certificates with the limited provision of home care only, in other words not an integrated contract, must have the words “Home Care Only” prominently displayed on page one of the policy and outline of coverage.

Bed Reservation Benefit

This is an optional benefit, if desired by the client, available for an additional cost. This benefit applies if a nursing home resident must leave the nursing home to be hospitalized. A bed can be reserved until such time as they can return. Otherwise, their long-term care at the nursing home may cease and the bed will be lost and another facility would have to be found.

Nursing Home/Residential Care Facility for the Elderly Benefit

Prospective clients buying a long-term care policy covers confinement in a nursing facility. Some states have Nursing Home/Residential Care Facilities for the Elderly. Formerly, some insurance codes required that he be offered a policy in assisted living, but now he or she may be offered the Residential Care Facility for the Elderly Benefit. These facilities must meet applicable license standards and offer ongoing care in related services that cover the needs from impairment of activities of daily living or impairment in cognitive ability. They must have 24-hour care with trained employees at all times. Three meals must be provided, special dietary needs, medical care with physicians and nurses, and proper medication.

Generally, this coverage for residential care must pay a minimum benefit not less than 70% of the maximum benefit for a nursing facility care.

Many states define a “chronically ill individual” and regulations as necessary for services and care. There are to be no restrictions on who provides services as long as there is no conflict with favorable tax considerations.

Generally, if such an individual is confined in a residential care facility for long-term care services, all expenses that are needed by a “chronically ill individual” are to be covered and payable up to, but to exceed maximum daily residential care facility benefit of policy.

In the event a policy is not intended to be federally qualified, the care in a residential care facility is no more restrictive than for home care benefits and activities of daily living and the restrictions for cognitive ability are same as for home care benefits.

If the policy is federally qualified, eligibility for care in a residential care facility is no more restrictive than for home care benefits and the same for cognitive impairment.

Restoration of Benefit

There is a stipulation that will restore benefits under certain situations for Long-term Care policyowners who do not have a lifetime benefit or an unlimited period of time. After a certain claim-free period, a provision will restore the time used up in benefit status. Suppose a person goes into a nursing home for six months. After a certain claim-free period, a provision will restore the time used up in benefit status. An example is if the person has a LTC policy that pays nursing home benefits for three years. He or she goes into a nursing home for six months, is released and has no more claims for whatever time period specified in the policy (this could be three, six or twelve months). At that time, the entire 3-year benefit is returned to the client.

Home Modification

A home modification benefit can be purchased that allows the insured to modify his or her home for both safety and flexibility, if this allows them to be independent. This

includes such things as wheelchairs ramps, bathroom grab bars, widening of doorway for wheelchair access, etc.

Caregiver Training

Most long-term care policies do not provide coverage for care by a family member. Another option benefit available to the insured is caregiver training so the caregiver can provide more extensive aid to the insured. This option is highly specialized, as most policyholders only need caregivers that have already been trained.

Return of Premium

If a client purchases a LTC insurance policy and returns it within 30 days (free look) for any reason, the premium will be returned and it will be as if the policy had never been issued. Another optional benefit that is available from some companies as a rider at an additional cost would return all premiums paid into the policy if an insured meets certain premium payment time requirements stated in the benefit language. This is for people who are concerned that they might pay the premiums for a long period of time and end up not ever using the benefits.

No Termination of Coverage During a Claim

Most states do not allow a policy to be terminated during an active claim. If long-term care coverage would have been terminated for some reason, and the insured is in an active claim (such as being in a nursing home or receiving formal care at home), the policy benefits must continue for as long as the benefit period lasts or up to the maximum benefit limit in the policy.

Many state's statutes cite this issue as the following: Termination of long-term care insurance will be without prejudice to any benefits payable for institutionalization if that institutionalization began while the long-term care insurance was in force and continues without interruption after termination. This extension of benefits beyond the period the long-term care insurance was in force may be limited to the duration of the benefit

period, if any, or to payment of the maximum benefits and may be subject to any policy waiting period, and all other applicable provisions of the policy.

Maximum Lifetime Benefit Definition & Limitations on Specific Covered Benefits

Usually, every long-term care policy or certificate must define the maximum lifetime benefit as a single dollar amount that can be used for any home and community based services or assisted living benefit or institutional care covered by policy.

There is often no limit on specific covered benefits except for a daily, weekly, or monthly limit set for home and community-based care and for no limit for institutional care. Nothing in this section can be construed as prohibiting limitations for reimbursement of actual expenses and incurred expenses up to daily, weekly and monthly limit.

Please return to the Course Player in order to take Lesson 9 Quiz.

Lesson Ten

Company Stability and Ratings

Especially in these times, when it comes to giving advice to the client regarding the selection of a long-term care insurance policy, it is quite the same counsel you might offer regarding any insurance purchase.

One should look for an insurance company that has been around a while and looks like it will still be there when they need it. They will want to review the financials of the company and check out their ratings with Moody's, Standard and Poor's, A.M. BEST, Dunn and Bradstreet, Duff and Phelps, and any other information published regarding the company. Talking to people who have done business with them may be helpful. The claims paying record of a company is important to be aware of to avoid any difficulties with an insurance company when claim time comes around. It is one of the many factors to consider in the buying process.

For the public, selecting a producer who will be giving advice should be a high priority! Long-term care insurance, like most other coverage, can be complex and the choices difficult. They look for the honest, knowledgeable, and caring producer who will assist them in understanding and selecting exactly the right base policy and mix of optional benefits.

In addition, a company that has been in the long-term care field for a number of years may be better founded than one who just introduced their product six months ago. Over time, a company may learn that the product has been under-priced and the client could be in for sticker shock. Or they may decide to get out of the long-term care insurance business after a while, leaving the client with a policy for which they can get no replacement parts or service. Again, this is not an absolute, but is certainly one factor for them in the consideration process.

Price shopping only has never been a wise choice. Regardless of the product, one usually gets exactly what they pay for. Nevertheless, there appears to be some price differential between good companies. When comparison-shopping, it is important to read all the literature on the product and examine a sample contract.

Companies that provide insurance carrier ratings evaluate the stability of insurance companies to determine their financial ability to pay their insurance policies and contracts in accordance with their terms. They evaluate the factors affecting the overall performance of an insurance company, operating performance, and ability to meet its obligations to policyholders.

Several of the major players have been rating insurance companies for many years such as Standard & Poor's, A.M. Best, Duff & Phelps, and Moody's.

Long-term care insurance rating organizations use slightly different rating definitions, so it might be difficult to compare one company to another unless the ratings are explained. For example:

Standard & Poor rates an insurance company with definitions such as AAA and AA+. An insurance company that is rated AAA is considered to have VERY STRONG financial security characteristics. These ratings are a current opinion confirming the company's ability to pay its insurance obligations. However, this opinion does not take into account deductibles, surrenders, cancellations, penalties or timeliness of payment.

A.M. Best has a rating of A++, which is very superior, and A+ that is superior. This company evaluates factors affecting the overall performance of an insurance company to provide an opinion of the company's financial stability and strength, operating performance and ability to meet its obligations.

Duff & Phelps also uses AAA that is considered very superior and AA+ signifies a superior rating. The AA+ definition, according to Duff & Phelps, means the protection

factors are strong and risk is modest, but may vary slightly over time due to economic and/or underwriting conditions.

Moody's has a designation of Aaa as very superior and Aa1 as superior. Moody's Insurance Financial Strength Ratings are opinions of the ability of insurance companies to punctually repay senior policyholder claims and obligations. Specific obligations are not considered to be rated, unless individually rated because the standing of a particular insurance obligation would depend on an assessment of its relative standing under those laws governing both the obligation and the insurance company.

Carrier Ranking and Stability

Standard & Poor's	AM Best	Moody's	Duff & Phelps
AAA	A++	Aaa	AAA
AA+	A+	Aa1	AA+
AA	A	Aa2	AA

If a particular company does not have one of the above ratings but instead has a "Not Assigned" status, it could mean the company did not score even a "fair" rating or there was not enough information available for any kind of rating. If this were the case, it would be necessary to research further to get the best information.

There are other sources that rate the financial stability and overall standings of insurance companies. This information can be accessed in most public libraries. The ratings in a given rank are not equivalent to one another. But, the company with the highest rating will be in first place with the next company in second place and so forth.

Agents bear the responsibility of explaining what type of a company it is to prospective clients; that is, if it is financially stable or not. There may be cases where the LTC insurance policy has a lower premium than another company but this company is not

rated as high. The customer must have this information so he or she can make a sensible decision. If the company is not rated very high, it may not have enough financial stability to meet the obligations of a LTC policy when the need arises. Producers must explain this.

Please return to the Course Player in order to take Lesson 10 Quiz.

Lesson Eleven

Administration and Enforcement

Oversight and monitoring of producers activities in the long-term marketplace is essential to maintaining quality and consistency in providing consumers with appropriate education and counseling in evaluating their needs and providing solutions to those needs. In this segment we take a look at the penalties and enforcement of the regulations so all are aware of the significance of the issues and the consequences for inappropriate actions by producers.

As continually reinforced throughout this course, insurers and producers bear a great responsibility for following the federal and state codes, rules and regulations. It is also the agent's responsibility to understand the codes well enough to explain the ramifications and different provisions. If she does not, there are penalties for violations.

The commissioners issue reasonable regulations, amendments and additions that may be necessary. The commissioners have the administrative authority to assess penalties for any violation of any provision against insurers, brokers, producers and other entities that are engaged in the insurance business.

Violation

If there is a violation of regulations in a civil action, a court can assess the penalties described in state regulations. The court may award reasonable attorney's fees and costs to a plaintiff who violates the provisions of state's code regulations.

The Attorney General, a district attorney or city attorney, on behalf of the people of a state, may bring to superior court any violation of any provision in the regulations. The court in addition to reasonable attorney's fees and costs may provide actions for

penalties, relief, damages or restitution in law for a plaintiff who violates the state's provisions.

If an agent, broker, or other entity (not the insurer) engaged in the insurance business is found to have violated the provisions they will be fined an administrative penalty. As an example, one state has the following fines structure of not less than:

- Two hundred and fifty dollars (\$250) for a first violation.
- One thousand dollars (\$1,000) for a second violation.
- Not more than twenty-five thousand (\$25,000) for each subsequent violation.
- The penalty for inappropriate replacement of long-term care coverage is five thousand dollars (\$5,000) for each violation.
- Insurers who commit a violation will be charged an administrative penalty of five thousand dollars (\$5,000) for the first penalty and subsequent knowing violations will be ten thousand dollars (\$10,000) each.
- If there is a trend of general business practice violations, the penalty for the magnitude of the violations against the public interest shall be ten thousand dollars (\$10,000) and not more than five hundred thousand dollars (\$500,000).
- The penalties are generally will be paid to the state's Insurance Fund.
- Non-penalty Remedies are Suspension or Revoking Licenses, Cease Marketing orders.
- There are also non-penalty remedies that can be taken in addition to the fines assessed. The commissioner can take the actions of suspending or revoking the license of any broker, agent or other producer licensed by the department.
- The commissioner can also suspend an insurer's certificate of authority to transact disability insurance.
- He can also order any broker, agent, insurer, or other entity engaged in the business of insurance to cease marketing in the state a particular

policy of long-term care insurance, to cease marketing any long-term care insurance or take any actions as are necessary to comply.

Notice and Hearing

An agent, broker, insurer or other entity that is charged with a violation is generally afforded due process in most states through proper notice and public hearing, if requested before a penalty is assessed.

The process commonly must be a written notice, served by registered mail, and generally includes all or most of the following:

- Summary of the facts establishing reasonable cause that a violation has occurred.
- Citation of the code section or other standard violated.
- A statement of the commissioner's intent to assess a penalty with amount of penalty or to seek other remedy.
- A statement of the respondent's right to elect any of the following:
 - To accept assessment of the penalty or other remedy as stated in the notice.
 - To respond to the charge in writing after which the commissioner may issue a final order or set a hearing.
 - To request within 10 days of receipt of notice, a public hearing.

Also, if there is a timely request by the respondent or one ordered by the commissioner, a public hearing before an administrative law bureau of the state will be held within 30 days after notice is served. Within 20 days after the hearing, the administrative judge will issue findings of act and a proposed order.

Commissioners many times issue their final orders or their proposed orders become the final order of the commissioner within 30 working days after the hearing unless consideration is granted for good cause by the administrative judge.

In addition, the commissioners may suspend the insurer's certificate of authority. If a commissioner finds after notice and hearing that the insurer has violated regulations, or that the insurer has knowingly allowed others to do so, he may allow an insurer to pay a fine, for example, of not more than say \$55,000 to avoid suspension of the insurer's certificate of authority.

Please return to the Course Player in order to take Lesson 11 Quiz.

Lesson Twelve

Consumer Protection

In matters that deal with our children and elders, there is an added measure of concern that they are protected from things that can hurt them or unfairly take advantage of their natural trusting nature, their potentially more frail stature, and their additional measure of inability to deal with the more complex aspects of everyday life.

Producers in the long-term care arena have a special responsibility to provide extra special care and attention to this as we go about working with people in their effort to learn, appreciate, and take action on their long-term care financial matters and insurance. Insurers, brokers and agents must be honest and operate with complete integrity when dealing with prospective clients and policyholders. State insurance codes have put a direct obligation upon insurers, brokers, and producers.

With regard to long-term care insurance, all insurers, brokers, agents and others engaged in the business of insurance owe a policyholder a duty of honest, and a duty of good faith and fair dealing.

The conduct of an insurer, broker during the offer and sale of a policy previous to the purchase is relevant to any action alleging a breach of the duty of honest, and a duty of good faith and fair dealing.

Advertisements Must Be Filed

In general, any advertisements to be used in most states for long-term care policies must be reviewed by their commissioners at least 30 days before it is publicly announced or published. It must comply with the laws in those states and the insurer must generally keep a copy in his files in some states for at least 3 years.

If the advertisement is for leads, in many states there must be a sentence reading “an insurance agent will contact you.” If it is a cold lead device, and the agent or broker received information that way, this fact must be disclosed to the consumer.

Filing Requirements for Advertising

According to the NAIC Model Regulations, every insurer, health care service plan or other entity providing long-term care insurance or benefits in this state must provide a copy of any long-term care insurance advertisement intended for use in a state whether through written, radio or television medium to the Commissioner of Insurance of the state for review or approval by the commissioner to the extent it may be required under state law. In addition, all advertisements must be retained by the insurer, health care service plan or other entity for at least three (3) years from the date the advertisement was first used.

The commissioner may exempt from these requirements any advertising form or material when, in the commissioner's opinion, this requirement may not be reasonably applied.

Marketing Standards

Educating the client on the contents of his or her long-term care insurance policy is the responsibility of brokers, insurers, and producers. The policy, benefits, restrictions, limitations and any another facts must be explained in a clear manner. The prospective consumer must be thoroughly educated before purchasing a policy. To accomplish this, each insurer, broker, and producer must be familiar with the product. Many states require agents selling long-term care policies to attend a continuing education class that has been approved by the state's department of insurance.

Marketing Procedures

Marketing procedures for long-term care insurance requires the following in most states:

- Every insurer of long-term care insurance must provide policy comparisons that are fair and accurate by its agents or other producers.

- Establish marketing procedures to assure excessive insurance is not sold or issued.
- Submit to their commissioners a list of all agents or other insurer representatives authorized to solicit individual consumers for the sale of long-term care insurance and these submissions to be updated at least semi-annually.

Standards for Marketing

The NAIC Model Regulations states the standards for marketing as follows:

Every insurer, health care service plan or other entity marketing long-term care insurance coverage in all states, directly or through its producers, must:

- Establish marketing procedures and agent training requirements to assure that:
 - Any marketing activities, including any comparison of policies, by its agents or other producers will be fair and accurate; and
 - Excessive insurance is not sold or issued.
- Display prominently by type, stamp or other appropriate means, on the first page of the outline of coverage and policy the following:

"Notice to buyer: This policy may not cover all of the costs associated with long-term care incurred by the buyer during the period of coverage. The buyer is advised to review carefully all policy limitations."

- Provide copies of the disclosure forms to the applicant.
- Inquire and otherwise make every reasonable effort to identify whether a prospective applicant or enrollee for long-term care insurance already has accident and sickness or long-term care insurance and the

types and amounts of any such insurance, except that in the case of qualified long-term care insurance contracts, an inquiry into whether a prospective applicant or enrollee for long-term care insurance has accident and sickness insurance is not required.

- Every insurer or entity marketing long-term care insurance must establish auditable procedures for verifying compliance.
- If the state in which the policy or certificate is to be delivered or issued for delivery has a senior insurance counseling program approved by the commissioner, the insurer will, at solicitation, provide written notice to the prospective policyholder and certificate holder that the program is available and the name, address and telephone number of the program.
- For long-term care health insurance policies and certificates, use the terms "non-cancellable" or "level premium" only when the policy or certificate conforms to this regulation.
- Provide an explanation of contingent benefit upon lapse and, if applicable, the additional contingent benefit upon lapse provided to policies with fixed or limited premium paying periods.

In addition to the practices prohibited in state's unfair trade practices act, the following acts and practices are prohibited:

- **Twisting.** Knowingly making any misleading representation or incomplete or fraudulent comparison of any insurance policies or insurers for the purpose of inducing, or tending to induce, any person to lapse, forfeit, surrender, terminate, retain, pledge, assign, borrow on or convert any insurance policy or to take out a policy of insurance with another insurer.
- **High pressure tactics.** Employing any method of marketing having the effect of or tending to induce the purchase of insurance through force, fright, threat, whether explicit or implied, or undue pressure to

purchase or recommend the purchase of insurance.

- **Cold lead advertising.** Making use directly or indirectly of any method of marketing which fails to disclose in a conspicuous manner that a purpose of the method of marketing is solicitation of insurance and that contact will be made by an insurance agent or insurance company.
- **Misrepresentation.** Misrepresenting a material fact in selling or offering to sell a long-term care insurance policy.

With respect to the obligations set forth in this section, the primary responsibility of an association, as defined in the NAIC Long-Term Care Insurance Model Act, when endorsing or selling long-term care insurance will be to educate its members concerning long-term care issues in general so that its members can make informed decisions.

Associations must provide objective information regarding long-term care insurance policies or certificates endorsed or sold by such associations to ensure that members of such associations receive a balanced and complete explanation of the features in the policies or certificates that are being endorsed or sold.

The insurer must file with the insurance department the following material:

- The policy and certificate,
- A corresponding outline of coverage, and
- All advertisements requested by the insurance department.

The association must disclose in any long-term care insurance solicitation:

- The specific nature and amount of the compensation arrangements (including all fees, commissions, administrative fees and other forms of financial support) that the association receives from endorsement or sale of the policy or certificate to its members; and
- A brief description of the process under which the policies and the

insurer issuing the policies were selected.

- If the association and the insurer have interlocking directorates or trustee arrangements, the association must disclose that fact to its members.
- The board of directors of associations selling or endorsing long-term care insurance policies or certificates must review and approve the insurance policies as well as the compensation arrangements made with the insurer.

The association must also:

- At the time of the association's decision to endorse, engage the services of a person with expertise in long-term care insurance not affiliated with the insurer to conduct an examination of the policies, including its benefits, features, and rates and update the examination thereafter in the event of material change;
- Actively monitor the marketing efforts of the insurer and its agents; and
- Review and approve all marketing materials or other insurance communications used to promote sales or sent to members regarding the policies or certificates.
- The above section does not apply to qualified long-term care insurance contracts.
- The materials specified for filing must be filed in accordance with a state's filing due dates and procedures.
- No group long-term care insurance policy or certificate may be issued to an association unless the insurer files with the state insurance department the information required in this subsection.
- The insurer may not issue a long-term care policy or certificate to an association or continue to market such a policy or certificate unless the insurer certifies annually that the association has complied with the requirements set forth in this subsection.

- Failure to comply with the filing and certification requirements of this section constitutes an unfair trade practice in violation of insert citation to corresponding section of state unfair trade practices act.

“Training” Requirement

In many states and in an increasing number over the past few years, it is the responsibility of the insurer to inform an agent, broker or anyone selling or marketing long-term care insurance that in order to sell long-term policies, he or she must have completed an approved long-term training course. This course consists of topics related to long-term care services and long-term insurance, including, but not limited to, that state’s regulations and requirements, available long-term care services and facilities, changes or improvements in services or facilities, and alternatives to the purchase of private long-term care insurance.

The long-term training course also is required to discuss differences in eligibility for benefits and tax treatment between policies intended to be federally qualified and those not intended to be federally qualified, the effect of inflation in eroding the value of benefits and the importance of inflation protection, and NAIC consumer suitability standards and guidelines.

In most states, licensees must complete the initial training requirements before being authorized to solicit individual consumers for the sale of long-term care insurance. The following section illustrates the training requirement from the NAIC LTC Model Regulations.

Producer Training Requirements - NAIC LTCI Model Act

An individual may not sell, solicit or negotiate long-term care insurance unless the individual is licensed as an insurance producer for accident and health or sickness or life [include other lines of authority as applicable] and has completed a one-time training course. The training must meet the requirements in the ACT.

An individual already licensed and selling, soliciting or negotiating long-term care insurance on the effective date of this Act in a state may not continue to sell, solicit or negotiate long-term care insurance unless the individual has completed a one-time training course, within one year from the effective date of the state's LTC training legislation.

In addition to the required one-time training course, an individual who sells, solicits or negotiates long-term care insurance must also complete ongoing training.

The training requirements may be approved as continuing education courses under applicable state law or regulation.

The one-time training required by this Section must be no less than eight (8) hours and the ongoing training must be no less than four (4) hours every 24 months.

The training required consists of topics related to long-term care insurance, long-term care services and, if applicable, qualified state long-term care insurance Partnership programs, including, but not limited to:

- State and federal regulations and requirements and the relationship between qualified state long-term care insurance Partnership programs and other public and private coverage of long-term care services, including Medicaid;
- Available long-term services and providers;
- Changes or improvements in long-term care services or providers;
- Alternatives to the purchase of private long-term care insurance;
- The effect of inflation on benefits and the importance of inflation protection; and
- Consumer suitability standards and guidelines.

The training required by this Section may not include training that is insurer or company product specific or that includes any sales or marketing information, materials, or training, other than those required by state or federal law.

Insurers subject to this Act must obtain verification that a producer receives training required before a producer is permitted to sell, solicit or negotiate the insurer's long-term care insurance products, maintain records subject to the state's record retention requirements, and make that verification available to the commissioner upon request.

Insurers subject to this Act must maintain records with respect to the training of its producers concerning the distribution of its Partnership policies that will allow the state insurance department to provide assurance to the state Medicaid agency that producers have received the training and that producers have demonstrated an understanding of the Partnership policies and their relationship to public and private coverage of long-term care, including Medicaid, in their state. These records must be maintained in accordance with the state's record retention requirements and must be made available to the commissioner upon request.

The satisfaction of these training requirements in any state will be deemed to satisfy the training requirements in a particular state.

Fair Trade Practices

Notice to Buyer

To protect consumers, in many states a statement similar to the following statement must be included on page one of any policy, certificate, or outline of coverage:

“Notice to buyer”:

This policy may not cover all of the costs associated with long-term care incurred by the buyer during the period of coverage. The buyer is advised to review carefully all policy limitations.”

In addition, the producer or representative of the insurer should make a reasonable effort to:

- Determine whether a prospective applicant or enrollee for long-term care insurance already has accident and sickness or long-term care insurance, what types and amounts of any such insurance.
- Every insurer marketing long-term care insurance should have auditable procedures for verifying compliance.
- Every insurer marketing long-term care insurance should provide a copy of the long-term care insurance shopper’s guide developed by their state, if available, to each prospective applicant prior to the presentation of an application or enrollment form for insurance.

Commonly Prohibited Practices

In addition to other unfair trade practices, including those identified in most state’s codes, the following acts and practices are commonly prohibited:

- **Twisting.** To knowingly present any misleading information or incomplete and fraudulent comparison of any insurance policy or insurer for the purpose of inducing, or intending to induce, any person to lapse, forfeit, surrender, terminate, retain, pledge, assign, borrow on, convert any insurance policy or to take out a policy of insurance with another insurer.

- **High Pressure Tactics.** Using any method of marketing for inducing the purchase of insurance through force, fright, and threat whether explicit or implied or undue pressure for the purchase of insurance.
- **Cold Lead Advertising.** Using any method of marketing, directly or indirectly, which fails to disclose it as solicitation of insurance and that contact will be made by an insurance agent or insurance company.

Suitability

Suitability of Long-Term Care Insurance for Clients

The following items are designed and intended to alert insurers and agents once again of the responsibility they have to insure the applicant has the proper long-term care insurance, ability to pay for it and to also enable the client to compare insurance rates. Because it is so important, it is quoted in its entirety.

In most states, agents must make reasonable efforts to determine the appropriateness of a recommended purchase or replacement. The insurer or anyone else marketing long-term care insurance should:

- Develop and use suitability standards to determine whether the purchase or replacement of long-term care insurance is appropriate for the needs of the applicant.
- Train its agents in the use of its suitability standards.
- Maintain a copy of its suitability standards and make them available for inspection upon request by the commissioner.
- The agent and insurer will develop procedures that take into consideration, when determining whether the applicant meets the following standards developed by the insurer, the following:

To determine whether the applicant meets the standards developed by the issuer, the agent and issuer must develop procedures that take the following into consideration:

- Does the applicant have the ability to pay for proposed coverage and what affect it would have on other financial obligations the applicant might have?
- The applicant's goals and needs with respect to long-term care and the advantages or disadvantages of insurance to meet those goals or needs.
- Finally, does the applicant have other insurance, and if so, how does it compare to the values, benefits and cost of the recommended purchase or replacement?
- The issuer and agent should make reasonable efforts to obtain the information set out above. Their efforts should include presentation to the applicant at or prior to application some form of a "Long-Term Care Insurance Personal Worksheet".
- A good worksheet contains the information from the NAIC in not less than 12-point type. The insurer may request an applicant to provide additional information to comply with its suitability standards.
- In the premium section of the personal worksheet, the insurer should disclose all rate increases and rate increase requests for all policies, whether issued by insurer or purchased or acquired from another insurer, in the United States on or after January 1, 1990.

The premium section should include a statement that reads something like the following:

- "A rate guide is available that compares the policies sold by different insurers, the benefits provided in those policies, and sample premiums. The rate guide also provides a history of the rate increases, if any, for the policies issued by different insurers in each state they do business in. Agents can generally obtain a copy of this rate guide in most states by calling their department of insurance's toll-free telephone by accessing the state's department of insurance's internet web site (www.insurance.yourstate.gov)."

- A copy of the issuer's personal worksheet should be filed and approved by their state's insurance commissioner. A new personal worksheet should be filed and approved each time there is a rate increase and each time a new policy is filed for approval by the commissioner. The new personal worksheet should disclose the amount of the rate increase and all prior rate increases in a state as well as all prior rate increases and rate increase requests or filings in any other state.
- A completed personal worksheet should be returned to the issuer prior to the issuer's consideration of the applicant for coverage, except the personal worksheet need not be returned for sale of employer group long-term care insurance to employees and their spouses and dependents.
- The sale or dissemination outside the company or agency by the issuer or agent of information obtained through the personal worksheet is generally prohibited.
- The issuer should use the suitability standards it has developed in determining whether issuing long-term insurance coverage to an applicant is appropriate.
- Agents should use the suitability standards develop by insurer in marketing long-term care insurance.
- At the same time as the personal worksheet is provided to the applicant, the disclosure form entitled "Things You Should Know Before You Buy Long-Term Care Insurance" must be provided. The form will be in the format contained in the Appendix 3 in not less than twelve (12) point type.
- If the issuer determines that the applicant does not meet its financial suitability standards, or if the applicant has declined to provide the information, the issuer may choose to reject the application.
- The insurer should report annually to the state's commissioners the total number of applications received from residents of their state, the number of those who declined to provide information on the personal worksheet, the number of applicants who did not meet the suitability standards and the number who chose to conform after receiving a suitability letter.

Replacement of Policies (No unnecessary replacement of policies)

Why is this procedure watched so closely and why are there harsh penalties if violated?

In the past, there were some agents, who for the sake of making more commissions had written unnecessary replacement policies that did not benefit the consumer. When this was noticed, regulations were written into various states insurance codes to protect the insured. It emphasizes that insurers or agents are prohibited from replacing a LTC insurance policy that will either cost more money for the insured or result in a decrease of benefits.

For example, if any policy sold to a client that is a third or greater policy within a year's period it is considered to be unnecessary with one exception. Many say, however, if a policy is replacing several policies with one insurer, it is allowed.

This is an important part of consumer protection. State's insurance codes frown on unnecessary replacement of policies. Many of their codes state that:

No insurer, broker agent, or other person may cause a policyholder to replace a long-term care insurance policy unnecessarily. Nothing in their statutes should be construed to allow an insurer, broker, agent or other person to cause a policyholder to replace a long-term care insurance policy that will result in a decrease in benefits and an increase in premium.

How Are Replacement Policies Monitored?

In many states, insurers must maintain records for each producer and of that producer's amount of replacement sales as a percent of the agent's total annual sales and the amount of lapses of long-term care insurance policies sold by the agent as a percent of the agent's total annual sales.

In some states, each year by a certain date, their insurers must report the top 10 percent of agents with the greatest percentages of replacements and lapses as measured.

Many insurers must also report annually the number of lapses policies as a percent of its total annual sales in a state, as a percent of its total number of policies in force in that state, and as a total number of each policy forms in the states as of the end of the preceding calendar year.

Similarly, most insurers must report annually the number of replacement policies sold as a percent of its total annual sales in the states and as a percent of its total number of policies in force in the states as of the end of the preceding calendar year.

Reported replacement and lapse rates do not alone constitute a violation of insurance laws or imply wrongdoing. The reports are generally for the purpose of reviewing agent activities regarding the sale of long-term care insurance more closely.

Prohibition Against Pre-existing Conditions and Probationary Periods in Replacement Policies or Certificates

If a long-term care insurance policy or certificate replaces another long-term care policy or certificate, the replacing insurer must waive any time periods applicable to pre-existing conditions and probationary periods in the new long-term care policy for similar benefits to the extent that similar exclusions have been satisfied under the original policy.

New Benefits, New Eligibility or New Policies

Many policies contain a provision that in the event the insurer develops new benefits or benefit eligibility or new policies with new benefits or benefit eligibility not included in the previously issued policy, the insurer will grant current holders of its policies who are not

in benefit or within the elimination period the following rights:

- The policyholder will be notified of the availability of the new benefits or benefit eligibility or new policy within 12 months. The insurer's notice will be filed with the department at the same time as the new policy or rider.

Typically, the insurer offers the policyholder new benefits or benefits eligibility in one of the following ways:

- By adding a rider to existing policy and paying a separate premium for the new benefit or benefit eligibility based on insured's attained age. The premium for the existing policy will remain unchanged based on insured's age at issuance.
- By replacing the existing policy, or
- By replacing the existing policy or certificate with a new policy or certificate in which case consideration for past insured status shall be recognized by setting the premium for the replacement policy or certificate at the issue age of policy or certificate being replaced.

The insured may be required to undergo new underwriting but the underwriting generally can be no more restrictive than if the policyholder or certificate holder were applying for a new policy or certificate.

The insurer of a group policy offers the group policyholder the opportunity to have the new benefits and provisions extended to existing certificate holders, but the insurer is relieved of the obligations if the holder of the group policy declines the issuer's offer.

Availability of New Services or Providers

The NAIC Model Regulations puts it this way:

An insurer must notify policyholders of the availability of a new long-term policy series that provides coverage for new long-term care services or providers material in nature

and not previously available through the insurer to the general public. The notice must be provided within twelve (12) months of the date of the new policy series is made available for sale in this state.

New long-term care services or providers that are material in nature may not include changes to policy structure; or benefits or provisions that are minor in nature. Examples of when notification need not be provided include: changes in elimination periods, benefit periods and benefit amounts.

Notification is not required for any policy issued prior to the effective date of this Section or to any policyholder or certificate holder who is currently eligible for benefits, within an elimination period or on a claim, or who previously had been in claim status, or who would not be eligible to apply for coverage due to issue age limitations under the new policy. The insurer may require that policyholders meet all eligibility requirements, including underwriting and payment of the required premium to add such new services or providers.

The insurer must make the new coverage available in one of the following ways:

- By adding a rider to the existing policy and charging a separate premium for the new rider based on the insured's attained age;
- By exchanging the existing policy or certificate for one with an issue age based on the present age of the insured and recognizing past insured status by granting premium credits toward the premiums for the new policy or certificate. The premium credits will be based on premiums paid or reserves held for the prior policy or certificate;
- By exchanging the existing policy or certificate for a new policy or certificate in which consideration for past insured status will be recognized by setting the premium for the new policy or certificate at the issue age of the policy or certificate being exchanged. The cost for the new policy or certificate may recognize the difference in reserves between the new policy or certificate and the original policy or certificate; or
- By an alternative program developed by the insurer that meets the intent

of this Section if the program is filed with and approved by the commissioner.

An example of an acceptable alternative program is underwriting concessions.

An insurer is not required to notify policyholders of a new proprietary policy series created and filed for use in a limited distribution channel.

"Limited distribution channel" means through a discrete entity, such as a financial institution or brokerage, for which specialized products are available that are not available for sale to the general public.

Policyholders that purchased such a new proprietary policy must be notified when a new long-term care policy series that provides coverage for new long-term care services or providers material in nature is made available to that limited distribution channel. These policies issued will be considered exchanges and not replacements.

Where the policy is offered through an employer, labor organization, professional, trade or occupational association, the required notification in Subsection A above shall be made to the offering entity. However, if the policy is issued to a group defined in the Long-Term Care Insurance Model Act, the notification must be made to each certificate holder.

Nothing prohibits an insurer from offering any policy, rider, certificate or coverage change to any policyholder or certificate holder. However, upon request any policyholder may apply for currently available coverage that includes the new services or providers. The insurer may require that policyholders meet all eligibility requirements, including underwriting and payment of the required premium to add such new services or providers.

This does not apply to life insurance policies or riders containing accelerated long-term care benefits.

Premium Credits in Replacement Sales

Generally, if an insurer replaces a policy or certificate that it has previously issued, the insurer must recognize past-insured status by granting premium credits toward the premiums for the replacement policy of policyholder. How do these premium credits work? Two examples follow to illustrate this.

Betty has had a long-term policy for 15 years for home care and would like a comprehensive policy covering both home care and nursing home care which will cost her \$3000.00 a year. He paid \$2000 a year for 15 years for the old policy. (50% of \$2000 is \$1000). Her state's code says she is allowed 50% of \$2000 which is \$1000. So she reduces her new comprehensive policy of \$3000.00 by \$1000.00. She is allowed 50% of original policy to be applied to the new policy. ($5\% \times 10 \text{ years} = 50\%$) This premium credit helps Betty because she is now 15 years older than when she purchased the original policy. Also, her agent might be discouraged to replace an older policy simply for the sake of commission.

Joe has had and paid for a long-term policy for 5 years with a premium of \$2,000 per year. He wants to replace his policy with a new one with the same company that has a premium of \$3,000 per year. In this case, Joe is allowed a premium credit on his newer policy for a portion of the premiums he had paid on his original policy. Since his state regulations requires a credit that must equal 5% of the original premium of \$2,000 for the number of 5 years that he paid on it, his credit toward the new \$3,000 premium each year will be as follows: $5\% \text{ times } 5 \text{ years} = 25\% \text{ times } \$2,000 = \$500 \text{ Credit}$; which when subtracted from the new premium of \$3,000 = \$2,500. This \$2,500 amount would be Joe's new premium on his newer policy.

Generally, in these sample cases, the premium credits must equal five percent of the annual premium of the prior policy or certificate for each full year the prior policy was in force. The premium credit must be applied toward all future premium payments for the replacement policy or certificate, but the cumulative credit allowed need not exceed 50 percent. No credit need be provided if a claim was filed under the original policy.

The cumulative credits allowed need not reduce the premium for the replacement policy or certificate to less than the premium of the original policy or certificate.

In many states, any time long-term care coverage is replaced, the sales commission paid by the insurer and that represents the percentage of the sale normally paid for the first year sales of long-term care policies or certificates is to be calculated based on the difference between the annual premium of the replacement coverage and that of the original coverage. If the premium on the replacement product is less than or equal to the premium for the product being replaced, the sales commission are to be limited to the percentage of sale normally paid for renewal of long-term care policies or certificates. Replacement must be contingent upon the insurer's declaration that the replacement policy materially improves the position of the insured. These provisions do not generally apply to replacement coverage which is group insurance.

For purposes of this ruling, "commission or other compensation" generally includes pecuniary or non-pecuniary remuneration of any kind relating to the sale or renewal of the policy including, not limited to, bonuses, gifts, prizes, awards and finder's fees.

This simply means that an insurer cannot pay an agent commissions on the sale of a replacement long-term care policy that is based on the replacement coverage alone. The coverage replaced must also be factored into the commission. Therefore an agent who ordinarily would receive a first-year commission of \$600 on the sale of a particular long-term care policy if it were to a new policyholder will received only a percentage if this amount if it were a replacement. This is to ensure that a replacement is necessary and not simply to get commissions.

Most long-term care insurers must file with their commissioners its commission structure or an explanation of the insurer's compensation plan.

The above regulations attempt to discourage insurers and agents from writing replacement policies for no legitimate reason. If a policy is replaced, the applicant must receive a notice regarding this replacement.

Pre-1997 Insurance Contracts

Before the Health Insurance Portability and Accountability Act (HIPAA) was signed into law, long-term care insurance policies contained certain liberties in determining a person's ability to perform a certain number of ADLs before benefit triggers were honored. Policies varied greatly wherein some had five or seven ADLs listed. Insurers could stipulate how many ADLs would start the benefits. Some were as few as two or three, others had requirements for three or four. There was no standardized regulation for ADLs. So, if one policy had seven ADLs and the policyholder only needed to have the inability to perform two, this would be a better policy than one with five ADLs.

Material Modifications to Existing Policies

Tax Consequences and Rules of the Department of Treasury Interim Guidance.

The US Department of Treasury interim guidance provides the definition of what “material modification” rules are. This guidance states if the “grandfather” rule has been applied to a pre-January 1, 1997 policy to retain tax-qualified status, there must be no material modification made to the policy.

The rule stipulates that “material modification” means there can be no change in the terms of the policy that alters the amount or timing of any item payable by the policyholder (or certificate holder), the insured, or the insurance company. In other words, if the premium amount or the time period is altered, this constitutes a material modification and the policy is then considered a “new contract” and will not fall under the “grandfather rules” for tax-qualified status.

However, there is one exception and that is if there was a unilateral exercise of an option or right (for example, “non-forfeiture benefits”) granted to the policyholder that

was in effect as of December 31, 1996, this is not considered a material change. (*This simply means that the change becomes effective without the consent of the issuer of the contracts.*)

This is the only exception to the “grandfather” rule in order to retain favorable tax benefits. Any other change means the tax-qualified status is lost.

Material Modifications - Legal Consequences

State’s codes say in essence that, if a policy or certificate holder of an insurance contract, purchased before December 31, 1996, requests a material change in that policy the legal consequences must be explained by the agent or insurer.

Under “grandfather” rules, any policy issued before December 31, 1996 that meets the state and federal qualification is automatically considered a tax-qualified policy. However, federal regulations state, any “material change” as defined by IRS Notice 97-31 will cancel that tax-qualified privilege. Before approving such a request, the insurer must provide a written notice to the policy or certificate holder that the change might be a material modification that jeopardizes the contract’s federal tax status. The policyholder should be advised to counsel with a tax advisor.

Converting or retaining an existing policy: To convert or retain an existing policy depends on the client’s situation. Most insurers have developed procedures to determine whether to convert or retain an existing policy.

The following types of information should be reviewed with the client:

- Does he or she have the ability to pay for the proposed coverage?
- Other pertinent financial information related to purchase of coverage.
- The applicant’s goals and needs with respect to long-term care and the advantages and disadvantages of insurance to cover those needs.

- The value, benefits and costs of applicant's existing insurance when compared to the values, benefits and costs of the recommended purchase or replacement.

Considerations are the same for consumers deciding whether to buy a federal tax-qualified policy. Questions to be answered by the consumer are:

- Is income level high enough for the person to need a tax break?
- In the future, will the income be high enough?
- Will a state's have less restrictive benefit triggers be more important?

Permitted Exclusion and Limitations

State residents deserve and enjoy an added measure of quality and assurance of being covered without unwanted or unanticipated conditions when they buy a long-term policy in their state. This is, in part, due to the specific permitted exclusions and limitations that are clearly laid out in most insurance codes. Those individuals insured by long-term care policies in most states can feel confident that their policy cannot exclude them from coverage based on certain illnesses, specified treatments, certain medical conditions, or types of accidents after they purchase their policy.

Here's a section from one state's code from which we have included the wording section here:

No policy may be delivered or issued for delivery in this state as long-term care insurance if the policy limits or excludes coverage by type of illness, treatment, medical condition, or accident, except as to the following:

- Pre-existing conditions or diseases
- Alcoholism and drug addiction

Illness, treatment, or a medical condition arising out of any of the following:

- War or act of war, whether declared or undeclared.
- Participation in felony, riot, or insurrection.
- Service in the armed forces or units auxiliary.
- Suicide, whether sane or insane, attempted suicide or intentionally self-inflicted injury.
- Aviation in the capacity of a non-fare-paying passenger.

Treatment provided in a government facility, unless otherwise required by law, services for which benefits are available under Medicare or other government programs (except Medicaid), any state or federal workers' compensation, employer's liability or occupational disease law, or any other vehicle no fault law, services provided by a member of the covered person's immediate family, and services for which no charge is normally made in the absence of insurance.

Limitations and Exclusions

The NAIC Model Regulations deals with this issue as follows:

Limitations and Exclusions. A policy may not be delivered or issued for delivery in this state as long-term care insurance if the policy limits or excludes coverage by type of illness, treatment, medical condition or accident, except as follows:

- Pre-existing conditions or diseases;
- Mental or nervous disorders; however, this shall not permit exclusion or limitation of benefits on the basis of Alzheimer's Disease;
- Alcoholism and drug addiction;
- Illness, treatment or medical condition arising out of:
 - War or act of war (whether declared or undeclared);
 - Participation in a felony, riot or insurrection;
 - Service in the armed forces or units auxiliary thereto;
 - Suicide (sane or insane), attempted suicide or intentionally self-inflicted injury; or

- Aviation (this exclusion applies only to non-fare-paying passengers).
- Treatment provided in a government facility (unless otherwise required bylaw), services for which benefits are available under Medicare or other governmental program (except Medicaid), any state or federal workers' compensation, employer's liability or occupational disease law, or any motor vehicle no-fault law, services provided by a member of the covered person's immediate family and services for which no charge is normally made in the absence of insurance;
- Expenses for services or items available or paid under another long-term care insurance or health insurance policy;
- In the case of a qualified long-term care insurance contract, expenses for services or items to the extent that the expenses are reimbursable under Title XVIII of the Social Security Act or would be so reimbursable but for the application of a deductible or coinsurance amount.

This section is not intended to prohibit exclusions and limitations by type of provider. However, no long-term care issuer may deny a claim because services are provided in a state other than the state of policy issued under the following conditions:

- When the state other than the state of policy issue does not have the provider licensing, certification or registration required in the policy, but where the provider satisfies the policy requirements outlined for providers in lieu of licensure, certification or registration; or
- When the state other than the state of policy issue licenses, certifies or registers the provider under another name.

For purposes of this paragraph, "state of policy issue" means the state in which the individual policy or certificate was originally issued.

Extension of Benefits. Termination of long-term care insurance must be without prejudice to any benefits payable for institutionalization if the institutionalization began while the long-term care insurance was in force and continues without interruption after termination. The extension of benefits beyond the period the long-term care insurance was in force may be limited to the duration of the benefit period, if any, or to payment of the maximum benefits and may be subject to any policy waiting period, and all other applicable provisions of the policy.

Claim Denials

If a claim is denied and an insured is unaware of it, he or she will be unable to make other arrangements. The requirement for insurers to report the number of denials is so that the commissioners can verify if a denial is valid; not because an insurer is attempting to insure only those consumers that are the least likely to need insurance.

Every policyholder or certificate holder whose claim is denied generally in most states must receive a written notice within a certain number of days, example 30-40 days, of the denial from the insurer with the reasons for the denial. There can be no termination of policy coverage during a claim.

Denied LTC Claim- NAIC

According to the NAIC regulations, if a claim under a long-term care insurance contract is denied, the issuer must, within sixty (60) days of the date of a written request by the policyholder or certificate holder, or a representative thereof:

- Provide a written explanation of the reasons for the denial; and
- Make available all information directly related to the denial.

A report of the number of claims that have been denied and the reasons for the denials are to be mailed to the state departments of insurance by a certain date each year. Insurers' denial rates are public property and can be reviewed at will.

Renewability Provision

It is important for consumers to know if a long-term care policy cannot be canceled or is renewable. Consumers must know exactly why and how a policy can be canceled. Can the premium be changed? Are there limitations on eligibility? These are important questions for the insured to consider before purchasing a long-term care insurance policy.

In states, individual long-term care insurance policies generally contain a renewability provision. This provision is usually on the first page of the policy, captioned with initial term of coverage, renewal conditions and if the insurer may change the premium amount. If this right exists, the policy provisions briefly describe circumstances under which premiums may change.

If the insurer states a premium request in a policy and includes all riders or endorsements added to a policy after date of issue, reinstatement or renewal that reduces or eliminates benefits or coverage in the policy, they have the insured sign an agreement. Also, any rider or endorsement increasing benefits or coverage by an increase in premium after the date of policy must have the insured sign an agreement (unless the law requires coverage or benefits).

If there are limitations with respect to pre-existing conditions, those limitations generally appear as a separate paragraph of the policy or certificate and must also state this along with a description of the conditions and limitations. The paragraph must be labeled, "pre-existing condition limitations".

A long-term care insurance policy or certificate containing any limitations or conditions for eligibility is set forth in a separate paragraph with a description of those limitations or conditions, including any required number of days of confinement, and usually labels that paragraph, "Limitations or Conditions on Eligibility for Benefits."

Requirements for Application Forms and Replacement Coverage

The NAIC Model Regulations provide guidance for this area as follows:

- Application forms must include the following questions designed to elicit information as to whether, as of the date of the application, the applicant has another long-term care insurance policy or certificate in force or whether a long-term care policy or certificate is intended to replace any other accident and sickness or long-term care policy or certificate presently in force. A supplementary application or other form to be signed by the applicant and agent, except where the coverage is sold without an agent, containing the questions may be used. With regard to a replacement policy issued to a group defined by [insert reference to Section 4(E)(1) of the Model Act], the following questions may be modified only to the extent necessary to elicit information about health or long-term care insurance policies other than the group policy being replaced, provided that the certificate holder has been notified of the replacement.
 - Do you have another long-term care insurance policy or certificate in force (including health care service contract, health maintenance organization contract)?
 - Did you have another long-term care insurance policy or certificate in force during the last twelve (12) months?
 - If so, with which company?
 - If that policy lapsed, when did it lapse?
 - Are you covered by Medicaid?
 - Do you intend to replace any of your medical or health insurance coverage with this policy or certificate?

Agents must list any other health insurance policies they have sold to the applicant.

- List policies sold that are still in force.
- List policies sold in the past five (5) years that are no longer in force.

Solicitations Other than Direct Response. Upon determining that a sale will involve replacement, an insurer; other than an insurer using direct response solicitation methods, or its agent; must furnish the applicant, prior to issuance or delivery of the individual long-term care insurance policy, a notice regarding replacement of accident and sickness or long-term care coverage. One copy of the notice will be retained by the applicant and an additional copy signed by the applicant shall be retained by the insurer. The required notice must be provided in the following manner:

Direct Response Solicitations. Insurers using direct response solicitation methods must deliver a notice regarding replacement of accident and sickness or long-term care coverage to the applicant upon issuance of the policy. The required notice will be provided in the following manner:

“Where replacement is intended, the replacing insurer shall notify, in writing, the existing insurer of the proposed replacement. The existing policy shall be identified by the insurer, name of the insured and policy number or address including zip code. Notice shall be made within five (5) working days from the date the application is received by the insurer or the date the policy is issued, whichever is sooner.

Life Insurance policies that accelerate benefits for long-term care must comply with this section if the policy being replaced is a long-term care insurance policy. If the policy being replaced is a life insurance policy, the insurer will comply with the replacement requirements of the state's life insurance replacement regulation similar to the NAIC Life Insurance and Annuities Replacement Model Regulation. If a life insurance policy that accelerates benefits for long-term care is replaced by another such policy, the replacing insurer will comply with both the long-term care and the life insurance replacement requirements.”

Replacement of Existing Insurance Notice

In most states, this notice is supplied to the insured if he or she wishes to replace a current long-term care policy to ensure the replacement policy is truly desired. Often the insured does not know that the new policy may not offer as much protection as the old one. The notice generally must be given to the insured before a new policy is issued. Often, the agent will also have to give the insured a statement of "comparison to current coverage". Because insurance terminology is so complicated, the consumer often doesn't understand the precise meaning of each policy.

Long-term care insurance application forms usually include a question designed to elicit information as to whether the proposed insurance is intended to replace any other accident and sickness or long-term care insurance presently in force. A supplementary application or other form to be signed by the applicant containing such a question may be used.

Upon determining that a sale will involve replacement, an insurer, other than an insurer using direct response solicitation methods or its agent should furnish the applicant, prior to issuance or delivery of a policy or certificate, a notice regarding replacement of accident and sickness or long-term care coverage. One copy of this notice is retained by the applicant and the applicant retains an additional copy signed by the applicant and an additional copy signed by the applicant is to be retained by the insurer. The required form is often provided in approximately the following form:

Notice to Applicants Regarding Replacement of Accident and Sickness or Long-Term Care Insurance

[Insurance company's name and address]

SAVE THIS NOTICE! IT MAY BE IMPORTANT TO YOU IN THE FUTURE.

According to [your application] [information you have furnished], you intend to lapse or otherwise terminate existing accident and sickness or long-term care insurance and

replace it with the long-term care insurance policy delivered herewith issued by [company name] Insurance Company. Your new policy provides thirty (30) days within which you may decide, without cost, whether you desire to keep the policy. For your own information and protection, you should be aware of and seriously consider certain factors which may affect the insurance protection available to you under the new policy.

You should review this new coverage carefully, comparing it with all accident and sickness or long-term care insurance coverage you now have, and terminate your present policy only if, after due consideration, you find that purchase of this long-term care coverage is a wise decision.

1. Health conditions which you may presently have (pre-existing conditions), may not be immediately or fully covered under the new policy. This could result in denial or delay in payment of benefits under the new policy, whereas a similar claim might have been payable under your present policy.
2. State law provides that your replacement policy or certificate may not contain new pre-existing conditions or probationary periods. Your insurer will waive any time periods applicable to pre-existing conditions or probationary periods in the new policy (or coverage) for similar benefits to the extent such time was spent (depleted) under the original policy.
3. If you are replacing existing long-term care insurance coverage, you may wish to secure the advice of your present insurer or its agent regarding the proposed replacement of your present policy. This is not only your right, but it is also in your best interest to make sure you understand all the relevant factors involved in replacing your present coverage.
4. [To be included only if the application is attached to the policy.] If, after due consideration, you still wish to terminate your present policy and replace it with new coverage, read the copy of the application attached to your new policy and be sure that all questions are answered fully and correctly. Omissions or misstatements in the application could cause an otherwise valid claim to be denied. Carefully check the application and write to [company name and address]

within thirty (30) days if any information is not correct and complete, or if any past medical history has been left out of the application.

The above "Notice to Applicant" was delivered to me on:

(Date)

(Applicant's Signature)

The replacement notice shall include the following statement except when the replacement coverage is group insurance.

For group coverage not subject to the 30-day return provision, the notice shall be modified to reflect the appropriate time period in which the policy may be returned and premium refunded.

COMPARISON TO YOUR CURRENT COVERAGE: I have reviewed your current long-term care coverage. To the best of my knowledge, the replacement of insurance involved in this transaction materially improves your position for the following reasons:

_____ Additional or different benefits (please specify) _____

_____ No change in benefits but lower premiums.

_____ Fewer benefits and lower premiums.

_____ Other (please specify) _____

(Signature of Agent and Name of Insurer)

(Signature of Applicant)

(Date)

The above "Notice to Applicant" was delivered to me on:

(Date)

(Applicant's Signature)

"Inappropriate Replacement" in Consultation with Interested Parties

Many state's residents are entitled to replace their policies for others when it is appropriate and beneficial to them. Since not all replacements are conducted in a manner that benefits the policyholder, there are protections in most state regulations that make this issue especially important for them.

From time to time, the definition of "inappropriate replacement" needs to be revised as new replacement situations arise in the marketplace. When these surface, the insurance commissioner generally has the authority to redefine what is considered "inappropriate replacement" to incorporate any situations that are not currently included in that definition.

Shortened Benefit Period Non-forfeiture Benefit with Option to Purchase

The government has certain stipulations in the event that a consumer has owned a policy for many years and has not had to take advantage of the benefits. It was felt that it was not fair to spend money for years and get nothing out of it. It was not only the federal and state government that were concerned, but also the insurers. This is especially true in group policies where the policyholder is generally much younger. There is no regulation stating that the non-forfeiture value must be included in long-term care policies, but for both federal and state policies, the provision must generally be explained and offered to the client and the client may purchase the option if desired.

In some states, insurers may not generally deliver or issue for delivery a long-term care policy in most states unless the insurer offers at the time of the application an option to purchase a shortened benefit period non-forfeiture benefit with following features:

- Eligibility begins no later than after 10 years of premium payments.
- The lifetime maximum benefit is no less than the dollar equivalent of three months of care at a nursing facility per diem benefit contained in the policy or the amount of the premiums paid, whichever is greater.
- The same benefits covered in the policy and any riders at the time eligibility begins are payable for a qualifying claim.
- The lifetime maximum benefit may be reduced by the amount of any claims already paid or the amount of the premiums paid whichever is greater.
- Cash back, extended term, and reduced pay-up forms of non-forfeiture benefits shall not be allowed.
- The lifetime maximum benefit payment amount increases proportionally with the numbers of premium payment.

This does not always apply to life insurance policies that accelerate benefits for long-term care.

Nonforfeiture Benefits - General

A long-term care insurance policy may not be delivered or issued for delivery in this state unless the policyholder or certificate holder has been offered the option of purchasing a policy or certificate including a nonforfeiture benefit. The offer of a nonforfeiture benefit may be in the form of a rider that is attached to the policy. In the event the policyholder or certificate holder declines the nonforfeiture benefit, the insurer must provide a contingent benefit upon lapse that will be available for a specified period of time following a substantial increase in premium rates.

When a group long-term care insurance policy is issued, the offer must be made to the group policyholder. However, if the policy is issued as group long-term care insurance, other than to a continuing care retirement community or other similar entity, the offering must be made to each proposed certificate holder.

The commissioner will promulgate regulations specifying the type or types of nonforfeiture benefits to be offered as part of long-term care insurance policies and certificates, the standards for nonforfeiture benefits, and the rules regarding contingent benefit upon lapse, including a determination of the specified period of time during which a contingent benefit upon lapse will be available and the substantial premium rate increase that triggers a contingent benefit upon lapse.

(Note) A full version of the NAIC Model Regulations section on Nonforfeiture Provisions is in Appendix 16 section at the end of your course.

The chart following explains both state, as appropriate and federal government provisions.

Non-forfeiture Provisions – Fed & State

FEDERAL PROVISIONS	STATE PROVISIONS
A policy or certificate offered with non-forfeiture benefits shall have same coverage, eligibility, benefit length and triggers as coverage without non-forfeiture.	Provision qualifications begin after 10 years of premium payments. Lifetime maximum benefit is same as the dollar amount of 3 months of care at nursing facility per diem.
Offer shall be in writing if not described in outline or other materials given to client	Same benefits covered at time when eligibility are payable for qualifying claim

Non-forfeiture provision must have one of these conditions: longer term insurance, shortened benefit period or reduced fully paid insurance. The Secretary may also approve other conditions.	Lifetime benefit may be reduced by amount of claims already paid. Cash back, extended term, and reduced paid-up forms of non-forfeiture benefits not allowed. Lifetime maximum benefit increases proportionally with number of years of premium payment.
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Life insurance policies that have accelerated benefits are generally not included in the provision rules above.

Protection Against Unintended Lapse

This pertains to the designation of individuals to receive notice of lapse for nonpayment of premium. A consumer's protection that must be written into the long-term care insurance policy by the insurer is for forgetfulness. If there is a probability that a policy owner may develop a disease such as Alzheimer's or other brain impairment, the insurer must offer a provision to prevent the premiums from lapsing through no fault of the policy owner. He or she will designate another person to receive notice of the policy lapsing. This must be done 30 days before it does lapse. This designation can be reinstated every two years. A written designation listing the name, address and telephone number of at least one other individual who is to receive notice of lapse or termination of policy or certificate for nonpayment of premium. This is so important that if a policyholder does not wish to designate a person, he or she must generally sign a waiver to the effect.

In most states, notice must be given by United States first class mail, postage prepaid, not less than 30 days after a premium is due and unpaid. (See the sample copy of a waiver below.)

If a policy does lapse unintentionally, it can be reinstated if the reason is due to cognitive impairment or loss of functional capacity. However, it can generally only be reinstated if no more than 5 or 6 months have passed and all premiums must be paid. Policy owners who do not want this provision must sign a waiver like the following:

Protection against Unintended Lapse

I understand that I have the right to designate at least one person other than myself to receive notice of lapse or termination of this long-term care insurance policy for nonpayment of premium. I understand that notice will not be given until 30 days after a premium is due and unpaid. I elect not to designate any person to receive the notice.

_____ Signature of Applicant
_____ Date

The insurance company must also notify the policyholder that the designated person can be changed once every two years, but a change can be made within the two years as well.

Both the insured and the designated person must receive notification thirty days prior to date of lapse. If the policyholder has a payroll or pension deduction plan, the requirements of a designated person to receive notification of lapse does not need to be met generally until 60 days after a payroll or other deduction plan has stopped.

In every long-term care insurance policy there must be a provision that if there is a lapse due to cognitive impairment and proof is provided to insurer, the insured may request reinstatement if requested within five months. The cognitive impairment proof cannot be more severe than the benefit eligibility criteria in the policy.

Unintentional Lapse

NAIC Model Regulations address this as follows:

Each insurer offering long-term care insurance must, as a protection against unintentional lapse, comply with the following:

Notice before lapse or termination. No individual long-term care policy or certificate can be issued until the insurer has received from the applicant either a written designation of at least one person, in addition to the applicant, who is to receive notice of lapse or termination of the policy or certificate for nonpayment of premium, or a written waiver dated and signed by the applicant electing not to designate additional persons to receive notice. The applicant has the right to designate at least one person who is to receive the notice of termination, in addition to the insured. Designation will not constitute acceptance of any liability on the third party for services provided to the insured. The form used for the written designation must provide space clearly designated for listing at least one person. The designation must include each person's *full name* and *home address*. In the case of an applicant who elects not to designate an additional person, the waiver must state: "Protection against unintended lapse. I understand that I have the right to designate at least one person other than myself to receive notice of lapse or termination of this long-term care insurance policy for nonpayment of premium. I understand that notice will not be given until thirty (30) days after a premium is due and unpaid. I elect NOT to designate a person to receive this notice." The insurer must notify the insured of the right to change this written designation, no less often than once every two (2) years.

When the policyholder or certificate holder pays premium for a long-term care insurance policy or certificate through a payroll or pension deduction plan, the requirements need not be met until sixty (60) days after the policyholder or certificate holder is no longer on such a payment plan. The application or enrollment form for such policies or certificates must clearly indicate the payment plan selected by the applicant.

Lapse or termination for nonpayment of premium. No individual long-term care policy or certificate will lapse or be terminated for nonpayment of premium unless the insurer, at least thirty (30) days before the effective date of the lapse or termination, has given notice to the insured and to those persons designated pursuant to Subsection A(1), at the address provided by the insured for purposes of receiving notice of lapse or termination. Notice must be given by first class United States mail, postage prepaid; and notice may not be given until thirty (30) days after a premium is due and unpaid. Notice will be deemed to have been given as of five (5) days after the date of mailing.

Reinstatement after Unintentional Lapse

In addition to the requirement above, a long-term care insurance policy or certificate must include a provision that provides for reinstatement of coverage, in the event of lapse if the insurer is provided proof that the policyholder or certificate holder was cognitively impaired or had a loss of functional capacity before the grace period contained in the policy expired. This option will be available to the insured if requested within five (5) months after termination and must allow for the collection of past due premium, where appropriate. The standard of proof of cognitive impairment or loss of functional capacity cannot be more stringent than the benefit eligibility criteria on cognitive impairment or the loss of functional capacity contained in the policy and certificate.

Right to Reduce Coverage and Lower Premiums

Option to Lower Premium and Benefits (Downgrade)

People's situations change with regard to their coverage needs. So does their financial situation change with regard to their capacity to afford different levels of benefits at any given point in time while they are covered by a long-term care policy. Also, as premium increases are experienced in the future after the purchase of a policy, policy owners deserve and are protected by state's regulations in place that provide them with notification requirements about available alternatives to merely canceling their policies.

We have included a section of one state's code here to inform you more specifically about them:

Every policy or certificate should include a provision that gives the policyholder or certificate holder the following rights to reduce coverage and lower premiums.

A right, exercisable any time after the first year, to retain a policy or certificate while lowering premium in the following ways:

- Reducing the lifetime maximum benefit.
- Reducing the daily, weekly or monthly benefit amount.
- Reducing the nursing facility per diem and reducing the home and community-based service benefits of a home care only policy and of a comprehensive long-term care policy.
- Converting a "comprehensive long-term care" policy or certificate to a "Nursing Facility Only" or a "Home Care Only" policy or certificate if the insurer issues those policies for sale in the state.

The NAIC LTCI Model Regulations says this about downgrading:

The premium for the policy or certificate that is reduced in coverage will be based on the age of insured at issue age and the premium rate applicable to the amount of reduced coverage at the original issue date.

If the contract in force at the time a reduction in coverage is made provides for benefit adjustments for anticipated increases in the cost of long-term care services, then the reduced nursing facility per diem, lifetime maximum benefit, and daily, weekly, or monthly home care benefits will be adjusted in the same manner and in the same amount as the contract in force prior to the reduction in coverage.

In the event a policy or certificate is about to lapse, the insurer will provide written notice to the insured of the options in subdivision (a) to lower the premium by reducing coverage and of the premiums applicable to the reduced coverage options. The insurer may include in the notice additional options to those required in subdivision (a). The notice provides the insured at least 30 days in which to elect to reduce coverage and the policy shall be reinstated without underwriting if the insured elects the reduced coverage.

In the event of a premium increase, the insured should be offered the option to lower premiums and reduce coverage.

Duplication of Benefits

In most states, if a national non-Medicaid or state long-term care program is created through public funding with the same benefits as an insured's present policy or certificate, the policyholder will be entitled to elect either a reduction in future premiums or an increase in future benefits.

The method for determining the premium reductions and future benefit increases will be based on how much duplication of benefits there is. In addition, the amount of past premium payments the insured has paid and the extent of past claims will be generally considered.

Guaranteed Renewable or Non-Cancelable

The term **guaranteed renewable** generally means that the insured person can keep this policy as long as the premiums are paid on time. The insurer cannot change any terms of coverage or refuse to renew the policy. One exception is allowed, where the insurer can raise premiums in accordance with provisions in the policy and change the premium rates to all insured in the same class if they are raised to all insurers in the same class. The "class" is determined by the insurer for the purpose of setting rates at the time the policy is issued.

Non-cancellable means the insured can continue the policy if premiums are paid timely. During that time, insurers cannot change, decline to renew, or change the premium rate.

The “renewable” clause should be appropriately captioned on page one of the policy, which describes the initial term of coverage, conditions for renewal, classification of insured and if guaranteed renewable under what circumstance insurer may change premium amount.

Policy Practices and Provisions

The NAIC Model Regulations says this:

Renewability

The terms "**guaranteed renewable**" and "**noncancellable**" cannot be used in any individual long-term care insurance policy without further explanatory language in accordance with the disclosure requirements of this regulation.

- A policy issued to an individual shall not contain renewal provisions other than "guaranteed renewable" or "noncancellable."
- The term "guaranteed renewable" may be used only when the insured has the right to continue the long-term care insurance in force by the timely payment of premiums and when the insurer has no unilateral right to make any change in any provision of the policy or rider while the insurance is in force, and cannot decline to renew, except that rates may be revised by the insurer on a class basis.
- The term "noncancellable" may be used only when the insured has the right to continue the long-term care insurance in force by the timely payment of premiums during which period the insurer has no right to unilaterally make any change in any provision of the insurance or in the premium rate.

- The term "level premium" may only be used when the insurer does not have the right to change the premium.
- In addition to the other requirements of this subsection, a qualified long-term care insurance contract shall be guaranteed renewable, within the meaning of Section 7702B(b)(1)(C) of the Internal Revenue Code of 1986, as amended.

Continuation or Conversion of Group Coverage

In most states, if group insurance terminates, the individual can get another policy with these benefits:

- Age can be at time of issue of original group policy.
- Benefits are identical or equivalent to group policy.
- Premium is time of age when group policy was issued.

The certificate of group insurance protects individuals in the event the group terminates, in that the certificate holder can convert to another policy.

However, there are a few exceptions: A policyholder may have had to be a member of the group for 6 months before termination of the group and that all premiums were paid by the individual. Exception rules in this area might as well be similar to those below:

- The termination of group coverage was because the insured did not make required payment of premium or contribution when due.
- The terminating coverage is replaced within 31 days by new group coverage and is effective the day after termination. The replacement coverage must also meet both of the following:
 - The replacement coverage provides benefits identical to, or benefits determined by the commissioner to be equivalent to or more than those provided by the termination coverage.

- The premium for the replacement coverage is calculated on the policyholder's age at the time of issue of the group certificate for the coverage that is being replaced. If the coverage being replaced has itself replaced previous group coverage; the premium for the newest replacement coverage is calculated on the policyholder's age at the time the previous group certificate was issued.

Continuation coverage - If coverage would be or has been terminated, an existing group policy can be maintained through continued timely payment of the premium.

Any insured individual whose eligibility for group coverage is based on his or her relationship to another person, will be entitled to continuation coverage under the group policy if the qualifying relationship terminates by dissolution of marriage or death.

Conversion coverage - an individual policy of long-term care insurance can take the place of a terminated group policy if insurability and benefits are the same.

Typically, the premium for the converted policy is generally calculated on the insured's age at the time the group certificate was issued. If the terminating group coverage replaced previous group coverage, the premium for the converted policy is calculated on the insured's age at the time the previous group certificate was issued.

Generally, before issuing conversion coverage, the insurer may require, if adequate notice is provided to certificate holders in the certificate, that:

- The individual must have been continuously insured under a group policy, or any group policy that it replaced, for at least six months immediately prior to termination in order to be entitled to conversion coverage.

- In order for the conversion policy to be effective the day following group coverage termination, a timely written application must be submitted and the premium must be paid as directed by the insurer.
- The conversion policy contains a provision for a reduction of benefits if the insured has existing long-term care insurance, payable on an expense-incurred basis, which, together with the conversion policy, would result in payment of more than 100 percent of incurred expenses. This provision is not being included in the conversion policy unless the reduction in benefits is reflected in a premium decrease or refund.
- The conversion policy contains a provision limiting the payment for a single claim, spell of illness, or benefit period occurring at the time of conversion, to the amount that would have been payable had the group coverage remained in effect.

The NAIC Model Regulations provide for this in the following way:

Continuation or Conversion

Group long-term care insurance issued in the state will provide covered individuals with a basis for continuation or conversion of coverage.

- For the purposes of this section, "a basis for **continuation of coverage**" means a policy provision that maintains coverage under the existing group policy when the coverage would otherwise terminate and which is subject only to the continued timely payment of premium when due. Group policies that restrict provision of benefits and services to, or contain incentives to use certain providers or facilities may provide continuation benefits that are substantially equivalent to the benefits of the existing group policy. The commissioner will make a determination as to the substantial equivalency of benefits, and in doing so, will take into consideration the differences between managed care and non-managed care plans, including, but not limited to, provider system arrangements,

service availability, benefit levels and administrative complexity.

- For the purposes of this section, "a basis for **conversion of coverage**" means a policy provision that an individual whose coverage under the group policy would otherwise terminate or has been terminated for any reason, including discontinuance of the group policy in its entirety or with respect to an insured class, and who has been continuously insured under the group policy (and any group policy which it replaced), for at least six months immediately prior to termination, will be entitled to the issuance of a converted policy by the insurer under whose group policy he or she is covered, without evidence of insurability.
- For the purposes of this section, "**converted policy**" means an individual policy of long-term care insurance providing benefits identical to or benefits determined by the commissioner to be substantially equivalent to or in excess of those provided under the group policy from which conversion is made. Where the group policy from which conversion is made restricts provision of benefits and services to, or contains incentives to use certain providers or facilities, the commissioner, in making a determination as to the substantial equivalency of benefits, will take into consideration the differences between managed care and non-managed care plans, including, but not limited to, provider system arrangements, service availability, benefit levels and administrative complexity.
- Written application for the converted policy shall be made and the first premium due, if any, shall be paid as directed by the insurer not later than thirty-one (31) days after termination of coverage under the group policy. The converted policy shall be issued effective on the day following the termination of coverage under the group policy, and shall be renewable annually.
- Unless the group policy from which conversion is made replaced previous group coverage, the premium for the converted policy will be calculated on the basis of the insured's age at inception of coverage under the group policy from which conversion is made. Where the group policy from which

conversion is made replaced previous group coverage, the premium for the converted policy will be calculated on the basis of the insured's age at inception of coverage under the group policy replaced.

- Continuation of coverage or issuance of a converted policy will be mandatory, except where:
 - Termination of group coverage resulted from an individual's failure to make any required payment of premium or contribution when due; or
 - The terminating coverage is replaced not later than thirty-one days after termination, by group coverage effective on the day following the termination of coverage:
 - Providing benefits identical to or benefits determined by the commissioner to be substantially equivalent to or in excess of those provided by the terminating coverage; and
 - The premium for which is calculated in a manner consistent with the requirement.
- Notwithstanding any other provision of this section, a converted policy issued to an individual who at the time of conversion is covered by another long-term care insurance policy that provides benefits on the basis of incurred expenses, may contain a provision that results in a reduction of benefits payable if the benefits provided under the additional coverage, together with the full benefits provided by the converted policy, would result in payment of more than 100 percent of incurred expenses. The provision will only be included in the converted policy if the converted policy also provides for a premium decrease or refund which reflects the reduction in benefits payable.
- The converted policy may provide that the benefits payable under the converted policy, together with the benefits payable under the group policy from which conversion is made, may not exceed those that would have

been payable had the individual's coverage under the group policy remained in force and effect.

- Notwithstanding any other provision of this section, an insured individual whose eligibility for group long-term care coverage is based upon his or her relationship to another person will be entitled to continuation of coverage under the group policy upon termination of the qualifying relationship by death or dissolution of marriage.
- For the purposes of this section a "managed-care plan" is a health care or assisted living arrangement designed to coordinate patient care or control costs through utilization review, case management or use of specific provider networks.

Discontinuance and Replacement

If a group long-term care policy is replaced by another group long-term care policy issued to the same policyholder, the succeeding insurer must offer coverage to all persons covered under the previous group policy on its date of termination. Coverage provided or offered to individuals by the insurer and premiums charged to persons under the new group policy:

- Will not result in an exclusion for pre-existing conditions that would have been covered under the group policy being replaced; and
- Will not vary or otherwise depend on the individual's health or disability status, claim experience or use of long-term care services.

The premium charged to an insured cannot increase due to either:

- The increasing age of the insured at ages beyond sixty-five (65); or
- The duration the insured has been covered under the policy.

The purchase of additional coverage cannot be considered a premium rate increase, but for purposes of the calculation, the portion of the premium attributable to the additional coverage must be added to and considered part of the initial annual premium.

A reduction in benefits will not be considered a premium change, but for purpose of the calculation required, the initial annual premium shall be based on the reduced benefits.

Requirements for Replacing Insurers

In most states, requirements for replacement and renewability of group coverage have been identical with individual policies since the early 1990s.

Generally, when a group long-term care insurance policy is replaced by another policy with the same insurer, the following requirements will often apply:

- Benefits must be the same as the terminating coverage or benefits must be equivalent to the old policy. However the states' commissioners decide if lesser or greater benefits if they are more advantageous for group members.
- The premium must be calculated on the policyholder's age at time of issue of group certificate for coverage that is being replaced. The premium for newest replacement is calculated on age when previous group certificate was issued.
- All persons must be offered coverage covered under the replaced group policy at time of termination.
- Pre-existing conditions of an insured cannot prevent that person from the new group insurance if the old policy provided the benefits.
- New waiting periods, elimination periods, probationary periods or similar preconditions related to pre-existing conditions are prohibited if the same regulations were included in old policy.
- Benefits or premiums based on the policyholder's health, disability; claims experience or use of long-term care services cannot be varied.

The premium rate schedules for all individual and group long-term care insurance policies issued in most states must be filed with and receive the prior approval of their

commissioner before the policy may be offered, sold, issued, or delivered to a resident of their state.

Most states say that insurers may not increase the premium for an individual or group long-term care insurance policy or certificate approved for sale unless the insurer has received prior approval for the increase from their commissioners.

Licensing

A producer is not authorized to sell, solicit or negotiate with respect to long-term care insurance except as authorized by the state law equivalent to the NAIC Producer Licensing Model Act.

Reporting Requirements

Every insurer must maintain records for each agent of that agent's amount of replacement sales as a percent of the agent's total annual sales and the amount of lapses of long-term care insurance policies sold by the agent as a percent of the agent's total annual sales.

Every insurer must report annually by June 30 the ten percent (10%) of its agents with the greatest percentages of lapses and replacements.

Reported replacement and lapse rates do not alone constitute a violation of insurance laws or necessarily imply wrongdoing. The reports are for the purpose of reviewing more closely agent activities regarding the sale of long-term care insurance.

Every insurer must report annually by June 30 the number of lapsed policies as a percent of its total annual sales and as a percent of its total number of policies in force as of the end of the preceding calendar year. (See Appendix 14 Sample Form)

Every insurer must report annually by June 30 the number of replacement policies sold as a percent of its total annual sales and as a percent of its total number of policies in force as of the preceding calendar year. (See Appendix 14)

Every insurer must report annually by June 30, for qualified long-term care insurance contracts, the number of claims denied for each class of business, expressed as a percentage of claims denied. (See Appendix 12)

The definition of claim denied used in this reporting form is for HIPAA reporting purposes only, and is not intended to be applied to any other regulatory issues, such as market conduct examinations.

For purposes of this section:

- "Policy" means only long-term care insurance;
- Subject to Paragraph (3), "claim" means a request for payment of benefits under an in force policy regardless of whether the benefit claimed is covered under the policy or any terms or conditions of the policy have been met;
- "Denied" means the insurer refuses to pay a claim for any reason other than for claims not paid for failure to meet the waiting period or because of an applicable pre-existing condition; and
- "Report" means on a statewide basis.
- Reports required under this section must be filed with the commissioner.

Discretionary Powers of Commissioner

According to NAIC LTCI Model Regulations, the commissioner may, upon written request and after an administrative hearing, issue an order to modify or suspend a specific provision or provisions of this and other regulation with respect to a specific long-term care insurance policy or certificate upon a written finding that:

- The modification or suspension would be in the best interest of the insureds;
- The purposes to be achieved could not be effectively or efficiently achieved without the modification or suspension; and

- The modification or suspension is necessary to the development of an innovative and reasonable approach for insuring long-term care; or
- The policy or certificate is to be issued to residents of a life care or continuing care retirement community or some other residential community for the elderly & the modification or suspension is reasonably related to special needs or nature of such a community; or
- The modification or suspension is necessary to permit long-term care insurance to be sold as part of, or in conjunction with, another insurance product.

This provision is intended to provide the commissioner with limited discretion and flexibility to accommodate specific and innovative long-term care insurance products which are shown to be in the public's best interest. This provision is intended to be used sparingly for this purpose.

Inflation Escalator and Benefit Increase

Most, if not all, state insurance codes address the problem of inflation as it relates to long-term care. An estimated inflation factor of 5% per year in raising costs for nursing homes is sound to assume. A person in a nursing home in the early 1990s would pay 55% more today. The next ten years will add another 50% to the costs, if inflation stays at 5%. It is very important to include an inflation provision in long-term care insurance policies. All long-term care insurance policies have an option that the insurer must offer to the consumer that provides for benefit levels to increase to account for reasonably anticipated increases in the cost of long-term care services covered by the policy.

Option to Purchase Inflation Protection

State's insurance codes typically state that the inflation option would increase the benefit levels annually. It would guarantee the individual the right to increase benefit levels without the person having to provide evidence of insurability or health status as long as the option for the previous period has not been declined. It covers specified percentages of actual or reasonable charges.

Codes typically state that no insurer may deliver, or issue for delivery, a long-term care insurance policy or certificate in their states unless the insurer offers to each policyholder and certificate holder, in addition to any other inflation protection, the option to purchase a long-term care insurance policy or certificate that provides for benefit levels and benefit maximums to increase to account for reasonably anticipated increases in the costs of long-term care insurance covered by the policy.

Insurers offer to each policyholder and certificate holder, at the time of purchase, the option to purchase a long-term care insurance policy or certificate containing an inflation protection feature which is no less favorable than one that does one or more of the following:

- Increases benefit levels annually in a manner that the increases are compounded annually at a rate of not less than 5 percent.
- Guarantees the insured individual the right to periodically increase benefit levels without providing evidence of insurability or health status and without regard to claim status or history so long as the option for the previous period has not been declined.
- The amount of the additional benefit should be no less than the difference between the existing policy benefit and that benefit compounded annually at a rate of at least of 5% for the period beginning with the purchase of the existing benefit and extending until the year in which the offer is made.
- Covers a specified percentage of actual or reasonable charges and does not include a maximum specified limit amount.

Exemptions

There are a few exceptions to the increase offer, which are typically as follows:

- Life insurance policies or riders containing accelerated LTC benefits.
- Expense incurred long-term care insurance policies. The term “expense incurred” does not include policies paying a certain percentage of

reasonable and customary charges up to a specified, indemnity-type maximum amount.

Requirements for Offers of Inflation Protection

Inflation protection benefit increases without regard to an insured's age, claim status, history, or the length of time the person has been insured. Some states also added an offer of inflation protection that provided for automatic benefit increases so the premium would remain constant. There is also a provision that if this offer is made to the insured and it is turned down, the rejection must be included in the application.

An offer of inflation protection that provides for automatic benefit increases includes an offer of a premium that the insurer expects to remain constant. The offer discloses in a conspicuous manner that the premium may change in the future unless the premium is guaranteed to remain constant.

The inflation protection benefit increases under a policy or certificate that contains an inflation protection feature cannot be reduced due to payment of claims.

Rejections of Inflation Protection

Typically if the client does not want the inflation protection provision for the annual increase of not less than 5 percent, they must sign a rejection form stating this. The insurer must obtain this rejection of inflation protection signed by policyholder.

The rejection form that is included in the application or in a separate form often states something similar to the following:

"I have reviewed the outline of the coverage and the graphs that compare that compare the benefits and premiums of this policy with and without inflation protection. Specifically, I have reviewed the plan and I reject 5 percent annual compound inflation protection.

Signature of Applicant

Date

The insurer or his producer should discuss a rejection of inflation protection very carefully with the client. If the long-term care costs go up at 5% per year, the policy will have protection of 5% per year. If the client does not wish this protection, it should be brought to his or her attention of who will pay the difference if there is no protection.

Inflation in Outline of Coverage

In order to illustrate the dangers of rejecting the inflation protection clause, the insurer in many states must include a chart or other illustration that shows clearly the upward path of inflation.

They require an insurer should include the following information in or with the outline of coverage:

- A graphic comparison of the benefit level of a policy that increases benefits at a compounded annual rate of not less than 5 percent with a policy that does not increase benefits. The graphic comparison shows benefit levels over at least a 20-year period.

Policy Benefits with 0% vs. 5% Inflation Protection Clause

Year	Policy with 0% inflation protection	Policy with 5% inflation protection
1	\$ 58,400	\$ 58,400
5	\$ 58,400	\$ 74,534
10	\$ 58,400	\$ 95,127
15	\$ 58,400	\$ 121,409
20	\$ 58,400	\$ 154,952
25	\$ 58,400	\$ 197,763

Requirement to Offer Inflation Protection

The NAIC Model Regulations deals with this inflation issue more formally here:

No insurer may offer a long-term care insurance policy unless the insurer also offers to the policyholder in addition to any other inflation protection the option to purchase a policy that provides for benefit levels to increase with benefit maximums or reasonable durations which are meaningful to account for reasonably anticipated increases in the costs of long-term care services covered by the policy. Insurers must offer to each policyholder, at the time of purchase, the option to purchase a policy with an inflation protection feature no less favorable than one of the following:

- Increases benefit levels annually in a manner so that the increases are compounded annually at a rate not less than five percent (5%);
- Guarantees the insured individual the right to periodically increase benefit levels without providing evidence of insurability or health status so long as the option for the previous period has not been declined. The amount of the additional benefit will be no less than the difference between the existing policy benefit and that benefit compounded annually at a rate of at least five percent (5%) for the period beginning with the purchase of the existing benefit and extending until the year in which the offer is made; or
- Covers a specified percentage of actual or reasonable charges and does not include a maximum specified indemnity amount or limit.

Where the policy is issued to a group, the required will be made to the group policyholder; except, if the policy is issued to a group defined in the Act other than to a continuing care retirement community, the offering must be made to each proposed certificate holder.

The offer in the section above will not be required of life insurance policies **or** riders containing accelerated long-term care benefits.

States typically say insurers must include the following information in or with the outline of coverage:

- A graphic comparison of the benefit levels of a policy that increases benefits over the policy period with a policy that does not increase benefits. The graphic comparison should show benefit levels over at least a twenty (20) year period.
- Any expected premium increases or additional premiums to pay for automatic or optional benefit increases.

An insurer may use a reasonable hypothetical, or a graphic demonstration, for the purposes of this disclosure.

Also, it is intended that meaningful inflation protection be provided. Meaningful benefit minimums or durations could include providing increases to attained age, or for a period such as at least 20 years, or for some multiple of the policy's maximum benefit, or throughout the period of coverage.

Inflation protection benefit increases under a policy which contains these benefits must continue without regard to an insured's age, claim status or claim history, or the length of time the person has been insured under the policy.

An offer of inflation protection that provides for automatic benefit increases must include an offer of a premium which the insurer expects to remain constant. The offer must disclose in a conspicuous manner that the premium may change in the future unless the premium is guaranteed to remain constant.

Inflation protection must be included in a long-term care insurance policy unless an insurer obtains a rejection of inflation protection signed by the policyholder. The rejection may be either in the application or on a separate form. The rejection will be considered a part of the application and must state:

I have reviewed the outline of coverage and the graphs that compare the benefits and premiums of this policy with and without inflation protection. Specifically, I have reviewed (Plans), and I reject inflation protection.

Right to Appeal

In most states, residents who buy long-term policies have rights and are entitled to be allowed to formally question and appeal decisions made by the coverage providers of their policies when the situation warrants. Having this issue spoken directly to in the regulations gives an added measure of confidence to the policyholder as well as fosters integrity in the policy provider's decision-making principals and process.

To make sure policyholders are aware of those rights and the process to follow in appealing decisions, the various state's codes speak to this area formally when it says that every policy or certificate will include a provision giving the policyholder or certificate holder the right to appeal decisions regarding benefit eligibility, care plans, services and providers, and reimbursement payments.

Statutory Requirements

In most states applicants must be offered 5% annual compounded inflation in benefits. And, applicants must sign a statement refusing 5% annual compounding. This is important as the effects of inflation on future costs and benefits available for those escalated costs are enormous.

Long-Term Care Costs for Nursing Homes

Here's an example of what we are talking about by using one state's example. This chart shows how much long-term costs have risen.

YEAR	AVERAGE ANNUAL NURSING HOME COSTS
1980	\$15,500
1988	\$28,000
1990	\$32,000
1992	\$36,000
1994	\$38,000
1996	\$40,500
1998	\$44,000
2000	\$48,500
2002	\$51,500
2003	\$55,000
2011	\$77,745

Cost of Nursing Home Care Today

Overall, the cost of care among facility-based providers has steadily increased. For example, in 2005 the median annual rate for a private nursing home room was \$60,225, compared with the 2011 median annual rate of \$77,745 (Median rate for semi-private \$70,445). This means that Americans can expect to pay approximately \$17,520 more per year today for a nursing home than they had to pay in 2005. This increase represents a 4.35 percent compound annual growth rate over that period. In addition to a private or semi-private room and the care options you choose, where you live also affects how much you'll spend for Nursing Home care. Alaska reported the highest rates for a private room in a Skilled Nursing Facility, at \$204,765 a year or \$561 a day on average. The lowest rates were found in Shreveport, Louisiana, at \$36,135 a year or \$99 a day.

In 2005, according to a study by Met Life:

- The average daily rate for a private room in a nursing home was \$203, or \$74,095 annually.
- The average daily rate for a semiprivate room in a nursing home was \$176, or \$64,240 annually.
- The average hourly rate for Home Health Aides was \$19.
- The average hourly rate for homemakers/companions was \$17.

Estimate Life Expectancy for Applicants at Different Ages

Age Now	Years of Life Left	Years of Life Left	Years of Life Left	Years of Life Left
	White	White	Black	Black
	Men	Women	Men	Women
50	27.1	31.9	23.0	28.5
55	22.9	27.5	19.5	24.5
60	19.1	23.2	16.3	20.8
70	12.4	15.6	11.0	14.3
75	9.6	12.2	8.9	11.4
80	7.2	9.2	6.8	8.6

Projected Future Nursing Home Costs (for daily benefits and 5% stay of 2.25 years)

The cost of semi-private nursing home care in 2011 was approximately \$193.00 a day (depending on area). Total cost for a month was \$5870 (30 days) or approximately \$71,455 a year. The average stay is approx 3 years, so the total cost after the third year is \$214,365 three years. The benefit of having the protection plan is illustrated in the table below that shows annual increases without inflation protection.

Future Nursing Home Cost calculated using 5% Annual Increases @ \$193 Per Day

LENGTH OF CARE	TODAY'S Costs	FUTURE COSTS*	
		IN 10 YEARS	IN 20 YEARS
1 year	\$71,455	\$116,393	\$189,591
2 years	\$142,910	\$232,786	\$379,182
3 years	\$214,365	\$349,179	\$568,773
4 years	\$285,820	\$465,572	\$758,364
5 years	\$357,275	\$581,965	\$947,955
10 years	\$714,550	\$1,163,930	\$1,895,910

*Based on projected increases of 5% per year. All numbers rounded to the nearest thousand for clarity.

Please return to the Course Player in order to take Lesson 12 Quiz.

Lesson Thirteen

State & Federal Implications for Taxation of LTC Insurance

History of the Dilemma

Let's take a peek at this issue in brief from a historical perspective to lay the framework for an expanded discussion of the tax consequences and tax qualifications of Long-Term Care policies and expenses later in this lesson.

We live in a country where taxation and tax considerations are paramount in maximizing the efficiency of our clients' buying power and their net dollars spent in providing things they need in all facets of their lives, including Long-Term Care.

Responsibility of Insurers and Producers

Understanding these regulations and explaining them to consumers is the responsibility of brokers and producers selling long-term care insurance. There are both penalties and fines for failing to give the correct information to clients.

Prospective customers must be made aware of the many long-term care policies available and the differences between each policy. Some important issues to consider are:

- The number of long-term care insurance policies sold in a state.
- The differences between tax-qualified policies and non-tax-qualified policies and how they affect consumers.
- How benefit triggers affect the variations between qualified and non-qualified policies.

- The difference between Federal and State requirements for Activities of Daily Living.

Another stipulation that insurers and agents must understand is that if a company sells policies that are tax-qualified, they must also sell policies that are not tax-qualified (If only non-tax-qualified policies are sold, tax-qualified policies do not have to be offered).

Tax-qualified long-term care insurance policies allow the consumer to deduct all expenses related to these policies. The Internal Revenue Service treats them the same way as typical health and accident policies. Non-tax-qualified long-term care insurance policies cannot deduct any expenses associated with the policy.

Understanding

It is critical that consumers understand not only what long-term care policies have to offer but also how they might differ. Clients must understand the terms and benefits of each policy so they can weigh the advantages and disadvantages of each plan. Likewise, producers must determine if a tax-qualified policy is more beneficial to his client than a non-tax-qualified policy. In certain situations a tax-qualified policy may not benefit a consumer.

The Health Insurance Portability and Accountability Act of 1996 (HIPAA) contained a change in tax law for long-term care insurance contracts that meet certain federal standards. HIPAA treats certain “qualified” long-term care contracts the same as health insurance for tax purposes. The premiums for these contracts are deductible, the benefit payments are excluded from personal income, and the unreimbursed cost of qualified long-term care services are deductible as a medical expense.

The federal standards also specify the level of disability required before benefits can be paid under a qualified contract to qualify for tax advantages. These standards conflicted with those that had been required in many states. The differences between federal and state standards are as illustrated in the table below.

Federal and Some State Benefit Triggers

Differences Between Federal and Some State Benefit Triggers with Regard to Tax Issues are shown here.

Federal benefit triggers are comprised of six ADLs compared to states's seven. Yet, federal benefit triggers are more restrictive than state ADL requirements.

Qualified Tax Benefit Policies	Non-qualified Tax Benefit Policies
(A) ELIGIBILITY FOR BENEFITS	(A) ELIGIBILITY FOR BENEFITS
You will not be paid for any long-term care benefits until:	You will not be paid for any long-term care benefits until:
You are unable to do <u>2 out of 6 ADLs</u> which include:	You are unable to do <u>2 out of 7 ADLs</u> which include:
bathing dressing continence transferring eating toileting AND a certificate that services will be needed for 90 days. OR Severe Cognitive Impairment	bathing dressing continence transferring eating toileting <u>ambulating</u> (this added ADL may make it easier to qualify for home care benefits. AND no certificate needed. Cognitive Impairment not required to be as severe as federal regulations
In general, no policy benefits can be paid for services covered or be applied for Medicare deductibles or copayments.	In general there are no limitations regarding the use of policy benefits for Medicare-related services.

Questions about potential tax implications for each of the two policies should be directed to a **Tax Adviser** before deciding which type of policy to purchase.

Long-Term Care Insurance Potential Tax Saving

When Deducting Long-Term Care Insurance Premiums - Potential Tax Savings & Tax Brackets

Age	Amount of Premium Deductible	15%	28%	31%	36%
40 or Less	\$340	\$51	\$95	\$105	\$122
41-50	\$640	96	179	198	230
51-60	\$1,270	190	356	394	457
61-70	\$3,390	508	949	1050	1220
70+	\$4,420	663	1237	1370	1591

This table illustrates the 2011 tax possibilities for policyholders who itemize deductions on their tax return. If a standard deduction is taken, this deduction cannot be taken. In addition, in order to be deductible, the cost of qualified long-term care premiums plus other medical expenses must exceed 7.5% of adjusted gross income. (AGI) Only that part of medical expenses that exceed 7.5% can be deducted.

Federal Tax Provisions Individual Benefit

Following are three examples to illustrate both tax advantages and limitations of premium deductibility. The first one is a single person (age 55) with an AGI of \$60,000 who buys a qualified long-term care policy costing \$1060 a year. The deduction is limited to \$1270. He has enough deductible non-medical income to file an itemized return. He also has \$7,157 for qualifying medical expenses.

SINGLE TAXPAYER	55	NO DEPENDENTS	ITEMIZES
	BEFORE	PREMIUM	AFTER
ADJ. GROSS INCOME	\$60,000		\$60,000

ITEMIZED DEDUCTIONS			
Medical	\$7,157	1060	\$8,117
Less 7.5% AGI	(4,500)		(4,500)
Taxes	4,236		4,236
Interest	6,824		6,824
Contributions	1,862		1,862
TOTAL	15,579	1060	16,639
	44,421		43,601
EXEMPTIONS	2,800		2,800
TAXABLE INCOME	\$41,621		\$40,801
INCOME TAX	\$8,308		\$8,084
		TAX SAVING	\$464

SINGLE TAXPAYER	AGE 55	NO DEPENDENTS	ITEMIZES ITEMIZED DEDUCTIONS
	BEFORE	PREMIUM	AFTER
ADJ. GROSS INCOME	\$60,000		\$60,000
Itemized Deductions			
Medical	\$ 3,680		\$ 4,500
Less 7.5% AGI	(4,500)		(4,500)
Taxes	4,236		4,236
Interest	6,824		6,824
Contributions	1,862		1,862
TOTAL	12,922		12,922
	47,078		47,078
EXEMPTIONS	2,800		2,800
TAXABLE INCOME	\$44,278		\$44,278
INCOME TAX	\$9,050		\$9,050
		TAX SAVING	0

This taxpayer has only \$3,680 of medical expenses instead of the \$7,157 before buying LTC insurance. All other factors are identical, but there is no tax saving here.

This following illustration shows that it is always wise to consult a tax consultant before buying Long-Term Care Insurance merely with the idea of saving income taxes.

MARRIED TAXPAYER	AGE 61	FILING JOINTLY	
	BEFORE	PREMIUM	AFTER
Adjusted Gross Income	\$75,000		\$75,000
Itemized Deductions			
Medical	4,725	2,200	6,925
Less 7.5% AGI	(5,625)		(5,625)
Taxes	4,236		4,236
Interest	6,824		6,824
Contributions	1,862		1,862
TOTAL	12,922	2200	14,222
	62,078		60,778
PERSONAL EXEMPTIONS	5,600		5,600
TAXABLE INCOME	\$56,478		\$55,178
INCOME TAX	\$10,217		\$9,853
		SAVINGS	\$364

As the illustrations above indicate, the amount of tax savings can be nothing or very modest. Second, note that as the AGI increases, it is more difficult to qualify for the 7.5% threshold. Third, the taxpayer must have sufficient non-medical deductible expenses to make it worthwhile to give up the standard deduction and itemize expenses. Finally the taxpayer must have enough other qualifying medical expenses in addition to the qualifying LTC to receive the maximum tax savings possible. Had these taxpayers been renters without mortgage interest and real estate tax deductions, their deductions would not have exceeded the standard deduction and they would have had no tax benefits. Non-qualifying LTC insurance is not deductible and therefore has no tax benefits arising from premiums. However, many taxpayers do not need deductions. Once again, these are points an agent must make clear to a potential customer because the agent is ultimately responsible for decisions made by his or her client.

IRS and LTC Benefits

How does the IRS regard benefits received as result of Long-Term Care insurance when the taxpayer files a claim for a benefit?

For a **Federal Tax-qualified** long-term care insurance policy, the tax treatment of payments received under a qualified long-term care policy is **not** generally considered to be taxable income. The only exception is a certain per diem type reimbursement; and then taxable only if and to the extent that the per diem rate is over certain amount (\$300 per day in 2011) and adjusted each year to reflect the effect of inflation).

IRS pub 525, p.17

What about the tax status of benefits received under long-term care policies that are considered **non-qualified** contracts? As an example, how would a policyholder report \$35,000 received from the insurer for reimbursement of nursing home cost paid by the policyholder? The Federal Government is silent on how benefit payments made under a non-qualified contract are to be taxed. The insurance carrier is required to report to the IRS on a form 1099-LTC these benefit amounts. Congress has been asked to address this issue for non-qualified insurance policies, but as of this date has done nothing, thereby leaving the tax status of these payments a matter of debate and an issue for a client's tax professional to determine how these benefits would be taxed for them specifically.

At this time the only resolution is for agents and insurers to advise the potential purchaser of a non-qualified policy to consult his or her tax advisor to determine what the tax treatment would be for benefit payments. Agents should be cautious about discussing tax implications of either type of policy until there is a resolution from the IRS.

Replacement Issues

Agents need to do a thorough comparison of a grandfathered contract (one that was sold before 1/1/97) in the event it is being replaced by another contract. There may be

more favorable benefits, benefit triggers, or other features that are not in a proposed contract. This is another area when a policyholder should be advised to consult a tax advisor in the event a premium increase should result in loss of qualified status for a grandfathered contract, whether or not the contract is being replaced. The Treasury Department has recently ruled that a premium increase does result in a material change to a contract and the loss of favorable tax treatment. That decision could change in the future, but until then the ruling stands.

Tax Implication on Non-Tax-Qualified Premiums & Benefits

The Internal Revenue Code has not resolved how benefits received under a non-qualified policy should be handled for tax purposes. The insurance carrier reporting the benefits sends a form 1099 to the IRS, but Congress has made no further decisions to date. It would appear that these benefits do not need to be reported. However, some individuals believe they should be reported as taxable income. If a taxpayer does report these benefits they can be offset by deductible medical expenses. That means the taxpayer must itemize on his or her tax return. This may complicate the process because money received from Social Security can be affected.

If a taxpayer wishes to buy a long-term care insurance policy, the agent should direct him or her to an accountant or tax adviser to determine the best course of action. New legislation allowed tax-qualified policies to be sold in many states. This gave consumers the option to deduct insurance premiums with a federal policy or to waive tax benefits for easier ADL requirements through a state policy.

As noted above, long-term care benefits paid under policies that meet state's standards, but are not intended to qualify for tax benefits, may or may not be taxable as income. If the U.S. Congress or the IRS resolves this issue, consumers will be provided a one-time opportunity to **EXCHANGE POLICIES**. Consumers would be notified by their insurer regardless of whether the policy was a qualified or non-qualified policy. The exchanged policy will be issued without any additional evidence of insurability and the new policy premium (which may be lower or higher) will be based on the person's age at

the time they were issued the original policy. However, consumers will not be allowed to exchange policies if they are receiving long-term care benefits under an existing policy at the time of notice; or if the exchange would provide immediate eligibility to receive benefits.

Carrier Implications and Problems

The Internal Revenue Service requires companies to complete a 1099 form for any benefits paid on a policy or contract sold as long-term care insurance. However, companies do not have to disclose whether a policy was qualified or non-qualified. As a result, both taxpayers and insurance companies are unable to discern whether a qualified or a non-qualified policy is more advantageous.

State Tax-Qualified Implications

Some state's personal income tax laws allow a deduction for expenses paid during a taxable year not compensated by insurance. Their regulations modified provisions to include paid-for qualified long-term medical care services and any qualified long-term care insurance contract. These regulations largely clarified the following definitions:

- "Medical care" to include amounts paid for qualified long-term care services (as defined in Section 7702B c, of the IRS Code which was added by the Health Insurance Portability and Accountability Act of 1996).
- The term "insurance covering medical care" now covers any qualified long-term care.
- Only "eligible long-term care premiums" are included in this section. This term is defined as the amount paid during a taxable year for any qualified long-term care.

Qualified long-term care is not care received by a relative or spouse of the individual, unless the service was by a licensed professional with respect to that particular service (for a detailed explanation refer to IRS Code, Section 152 a).

- For purposes of this section and Section 213 of IRS Code, the term “eligible long-term care premiums” means the amount paid during a taxable year for qualified long-term care insurance contract services (as defined in Section 7702B(b) of the IRS Code as added by the Health Insurance Portability and Accountability Act of 1996) covering an individual to the extent that amount does not exceed the limitation determined the following table.

Limitations Of Qualified Long-Term Care Per Taxable Year - 2008	
Age before close of taxable year	Deductible Premiums for taxable year
40 or less	\$340
40 but not more than 50	\$640
More than 50 but not more than 60	\$1,270
More than 60 but not more than 70	\$3,390
More than 70	\$4,240

(Source: IRS Publication 502 – Tax Year 2011)

Carriers Must Provide 1099 for LTC Insurance.

IRS requires companies to complete form 1099 for benefits paid for LTC insurance.

Tax Implications for Self-Employed Individuals

The present deduction for self-employed health insurance expenses is 100%.

Regulations treat LTC premiums in the same manner as medical insurance premiums and this deduction applies to LTC premiums on policies to self-employed individuals.

Employer Ramifications

- Employers who pay LTC premiums on behalf of employees will be entitled to deduct the premiums as a business expense as they do for medical expenses.

- LTC premiums paid by an employer in behalf of an employee will not be treated as income to that employee.

Individual

- LTC insurance premiums are considered a medical expense for itemized deductions; subject to limitations of 7.5% of the adjusted gross income.
- LTC insurance benefits received by a claimant will be tax-free to the recipient, subject to limitation.
- LTC expenses not covered by insurance can be deducted on Schedule A, subject to limitations.

Please return to the Course Player in order to take Lesson 13 Quiz.

Lesson Fourteen

Long-Term Care Partnership Policies

Introduction

Several states have implemented a unique long-term care program that addresses two issues a person might encounter if they need long-term care. The first issue is having long-term care available when it is needed. The second is protecting a person's assets if policy benefits are depleted and Medicaid assistance is required.

Long-Term Care Partnership Policies are an alliance between certain state governments and insurance companies. Together they offer policies that meet certain standards and provide lifetime asset protection.

Concept of Partnership Policies

Concept of Partnership Policy asset protection, generally include the following:

- Generally, anyone who buys a Partnership policy may keep part or all assets equal to the amount of insurance purchased and still have eligibility for Medicaid benefits.
- Automatic inflation protection must be included to ensure that benefits keep pace with rising cost of care.
- Deductible must be met only once throughout lifetime.
- Waivers of premiums while receiving care in nursing home or residential care facility.
- Interchangeable policy benefits so care can be customized to meet individual needs; meaning insurance designs are available with benefits available from one year to lifetime coverage.
- Lapse protection.

- Partnership policies are transportable throughout the USA, but if insured needs to apply for Medicaid benefits for LTC, they will have to return to the partnership state for the special Medicaid asset protection.

Coverage Offered

There are two types of Partnership policies available:

The first covers care in a residential care facility and nursing home care only.

The second provides care at home, in the community, as well as a residential or nursing care facility.

Inflation Protection — Why It's So Important?

As previously mentioned, the Partnership Policy includes an inflation clause. So why is inflation important for anyone buying a long-term care insurance policy?

At some point, everyone has experienced the effects of inflation. For example, when a person pays \$3 for a pound of butter at the grocery store that cost \$2 the week before. Long-term care is just as sensitive to inflation as groceries are. Long-term care costs have soared in the last ten years. If a person has a long-term care insurance policy without inflation protection the difference between what the policy pays and what current the costs are will be paid by the policyholder. This could be a significant amount based on the last ten years. The chart below shows the increase in nursing home costs per year.

Example of Average Annual Nursing Home Costs

YEAR	AVERAGE ANNUAL NURSING HOME COSTS WITH INFLATION
1980	\$15,500
1988	\$28,000
1990	\$32,000

1992	\$36,000
1994	\$38,000
1996	\$40,500
1998	\$44,000
2000	\$48,500
2002	\$51,500
2003	\$55,000
2004	\$58,400
2005	\$62,000
2006	\$65,600

The table above shows that nursing home costs increased from \$3000 to \$4000 every two years between 1980 and 2006. Therefore, buying insurance policies with inflation protection that increases benefits at a rate of 5 percent per year are highly recommended.

Statistics have shown that people are living longer, so the likelihood is that long-term care will be needed some time within a person's life span; especially during the last years of life. The chart below shows costs for long-term care in the last years of life.

When considering long-term care, it is extremely important to look closely at how inflation can affect a person's future. Without inflation protection an individual may find him or herself with benefits that pay only a small portion of the actual costs for long-term care. Therefore, it is important to ensure that an insurance policy will pay most of these expenses. If a policy is purchased without inflation protection and used 20 years from now, the policyholder will need to pay the difference between what the insurance pays (based on cost 20 years earlier) and what the actual cost of is. The reason is that benefits will not have increased with the rising cost of services. The consequences of failing to protect oneself against inflation can be enormous as shown in the following table.

Annual Cost of Care in Last Year of Life

It could be easily 20 or 30 more years into the future before one needs to use their benefits. Without some inflation protection built into their long-term care policy, how would one pay these costs?

AGE TODAY*	LIFE EXPECTANCY	ANNUAL COST OF CARE IN LAST YEAR
50	36 yrs. to age 86	\$318,500
55	31 yrs. to age 86	\$250,000
60	27 yrs. to age 87	\$205,000
65	23 yrs. to age 88	\$169,000
70	19 yrs. to age 89	\$139,000
75	15 yrs. to age 90	\$114,000
80	12 yrs. to age 92	\$ 99,000

*Source: Life Expectancy Data: Dept of Health Services, Center for Health Statistics; assuming 5% annual inflation.

As can be seen from the table above, a person who is currently 60 years old may not need long-term care for another 27 years. Even at age 75, it could easily be another 15 years before care is necessary.

All costs have increased substantially for all types of long-term care in most all states, such as nursing home facilities, assisting living programs, etc. The cost of long-term care increased on an average of 6.7% from 1980 to 1997. If long-term care continues to rise by 5 percent a year, the cost to stay in a nursing home will double from \$70,000 in 2011 to \$145,000 a year by the year 2024. *Aging America Magazine* writes, the typical nursing-home stay is an average 2½-3 years. Consequently, a nursing home stay in one state could cost more than \$110,000 today and nearly \$235,000 by 2014.

A simple illustration to show how inflation protection works can be seen by the following:

Tom and Barry are both 65 and healthy. They purchase a long-term care policy on the same day. James buys a 2-year Partnership policy that provides \$90,000 in long-term care benefits. If he needed long-term care today, the policy would pay \$90,000 of his LTC expenses. His Partnership policy includes inflation protection that increases benefits by 5% a year, compounded. Tom will probably not need long-term care for 20 years. At that time, he will be 85 and costs will have more than doubled. With inflation protection, Tom's policy will pay for more than \$240,000 in costs for his long-term care needs.

Barry will also not need long-term care for another 20 years. But he purchased his policy without inflation protection. Hence, his policy will only pay for \$90,000 leaving an estimated balance of \$150,000 in unpaid expenses. Barry will have to pay the difference out of his own pocket.

Partnership Products Differ From Current LTC Policies

This program is unique because it offers protection for middle class persons if long-term care is needed. Elderly, middle class people usually have few assets such as a home and some savings. Many live on a pension or social security. This group of people will be impacted the most if they need long-term care and are forced to use available assets with Medicaid assistance. Under one state's Partnership Program, people can buy one dollar of asset protection for each dollar paid out by the insurance policy. This is uniquely different from other long-term care insurance policies. As little as one year of coverage can be purchased (\$173 per day or \$63,145 based on 2011 requirements), or for one's lifetime if they can afford the premium.

Medicaid Asset Protection is generally available only in Partnership policies. This important feature guarantees that one may apply to Medicaid and be entitled to keep the assets Medicaid normally allows, plus assets equal to the amount their Partnership policy paid out in benefits for their care, should they use up their long-term care

insurance benefits and still need long-term care. The state will also disregard these protected assets when it recovers from their estate the amount paid by Medicaid on their behalf.

Partnership Policy Minimum Daily Benefits: One state had an Average Daily Private Pay Rate (ADPPR) for nursing facility care that increased to \$250. Remember that while the year ADPPR for nursing facility care is \$250, the minimum Daily Benefit, for Partnership policies that can be sold in that state for a calendar year is \$173 with a \$77 (70%) Residential Care and Assisted Living benefit. Additionally, the daily benefit for nursing home must be rounded off to the nearest \$10.00 increment. For example, one cannot buy a \$169 or \$173 policy, instead, they will buy \$170.00 or \$180.00 policies respectively.

Selecting the Maximum Lifetime Benefit: No one can predict how many days or years of long-term care a person will need, or the reason they will require care. Some people can afford lifetime coverage; others have so little money they would quickly qualify for Medicaid once they begin paying for long-term care. Choosing the right amount of benefit depends on the premium they can afford, and the assets they would otherwise have to spend.

Maximum Lifetime Benefit: When one buys a long-term care policy they choose the maximum lifetime dollar amount for approximately the number of years they want the policy to pay benefits. The premiums they pay will be based in part on the number of years the policy will pay. While everyone would like to have lifetime coverage or unlimited benefits, not everyone can afford to do so. A policy that pays for a few years can provide valuable coverage, and will be all many people will need. One should not pass up long-term care insurance just because they cannot afford lifetime coverage.

Partnership Requirements: Generally, partnership-certified policies cannot be purchased for less than an equivalent of the cost of one-year's stay in a nursing home. Since the premium for lifetime coverage is not affordable for many people, Partnership

policies offer another method of selecting the Maximum Lifetime Benefit. One should choose the period that is roughly proportional to their current non-housing assets that they might otherwise have to use to pay for their care. (Remember that the value of their house is not counted when applying for Medicaid, but can be collected against if it is part of their estate). The following section will provide a better explanation of how the Partnership's unique Asset Protection benefit can impact their decision on their policy's Maximum Lifetime Benefit.

If a person buys a Partnership policy, the Partnership insurance will pay for the benefits that are needed first. When the policy has paid all benefits purchased and more is needed for continuing costs, a person can apply for Medicaid assistance. (The need to "spend-down" assets until poverty level is reached.) The Partnership policy will protect a person's assets purchased with the insurance policy. So if a long-term care policy is purchased for \$63,150, the policy will pay \$63,150 for long-term care expenses. When this amount is paid out fully, Medicaid will pay for continuing long-term care costs, while allowing the policyholder to keep his or her \$63,150 in assets.

Alternatively, for example, if a person's assets decrease to \$40,000, he or she may apply to Medicaid when the policy has paid out \$40,000. The Partnership policy will continue to pay benefits up to the \$63,150 maximum, but Medicaid will cover other health-related problems.

If one spouse enters a nursing home, Medicaid will exempt \$109,560 (as of 2011 regulations) if the other spouse is living in the community. This is in addition to the home and some personal possessions. If a person is receiving Medicaid aid, he or she will be required to pay a monthly share from their income, as well. A community spouse's monthly income allowance is between \$1,820 and \$2,739 for 2011, as long as the income is actually made available to her/him.

Partnership Requirements for Elimination Periods: The Partnership may not allow the Elimination Period to be longer than 30 days for a policy that will pay less than the

equivalent of two years in a nursing facility. Similarly, the Partnership may not allow more than a 90-day elimination period for policies that will pay more than the equivalent of two years in a nursing facility. Other elimination periods can be offered in addition to the 90-day option, but not more than 90 days. Partnership-certified policies may require the insurance company to also count days Medicare pays for long-term care to count toward meeting the Elimination Period. The Partnership may also require that the Elimination Period be met only once in a lifetime.

Eligibility Criteria

All long-term care policies require that one's physical or mental conditions meet certain standards before benefits will be paid. These standards are often called Benefit Triggers. To receive benefits, the two Benefit Triggers allowed in Partnership-certified long-term care insurance policies in Partnership states generally are:

Impairment in Activities of Daily Living (ADLs) "Activities of Daily Living" (ADLs) are used to measure one's physical abilities to determine if they qualify for benefits. The law requires tax-qualified policies to pay benefits if they are impaired in two out of the following six ADLs: bathing, dressing, transferring, eating, toileting and continence. Some states have seven activities of daily living for non-qualified policies, whereas federal law contains six for tax-qualified policies. Ambulating is added to the six federal triggers for non-qualified policies. Only two ADLs can be required before benefits will be paid for nursing home care, RCFE care, or home care. "Impairment" means that they need human assistance or continual supervision to perform an Activity of Daily Living.

Impairment in Cognitive Ability (or Cognitive Impairment) "Impairment in Cognitive Ability" means that one needs supervision or assistance to protect themselves or others because of mental deterioration caused by a mental disease such as Alzheimer's disease or a mental illness. A diagnosis of cognitive impairment is based on clinical evidence and by the use of standardized tests.

LTC Agents Training Requirements

Agents and brokers selling long-term care Partnership insurance in some states must in addition to being properly licensed and have completed basic LTC training also must satisfactorily complete the following training which can be required as a part of, or in addition to, the overall training requirements of eight hours of specialized LTC training prior to each license renewal.

The training required consists of topics related to long-term care services and long-term care insurance, including, but not limited to, state regulations and requirements, available long-term care services and facilities, changes or improvements in services or facilities, and alternatives to the purchase of private long-term care insurance.

Partnership LTC Policies Require Additional Training

Agents selling or marketing Long-Term Care insurance policies certified by a state's Partnership area are generally required to be so certified by completing eight hours of Long-Term Care training and eight (or some other amount) hours of training specifically on the state's Partnership for Long-Term Care PRIOR to marketing or selling of any Partnership approved policies or certificates. After the initial certification, renewal of certification is generally required every two years. In accordance with regulations, agents who fail to comply with this training requirement cannot sell Partnership policies and companies are required to enforce this requirement or jeopardize their relationship with the state Partnership for LTC.

How Products May Differ from Existing Long-Term Care Policies

The various state's Partnership Programs are the only long-term care policies that offer asset protection. The premium is generally lower than other policies because it is a not-for-profit organization. This is a federally tax-qualified policy with tax advantages. The premiums are tax deductible and benefits payments are tax-free. There is a return-of-premium death benefit and the insured may live anywhere in the USA.

Please return to the Course Player in order to take Lesson 14 Quiz.

Lesson Fifteen

Alternatives to Long-Term Care Insurance

Not all people should buy long-term care insurance. Wealthy people who can pay for expenses out-of-pocket and people at or below poverty level who are eligible for Medicaid do not need long-term care insurance. All others who do not already have adequate long-term insurance should consider this type of coverage for peace of mind.

Once more, agents should collect adequate information from the client so they can discern whether or not long-term care insurance is necessary. It is the responsibility of insurers and agents to thoroughly understand a prospect's situation to know whether or not this type of insurance is needed.

Additionally, agents should encourage prospects to seek additional advice from other independent sources for more information on long-term care insurance.

Insurers and producers must also explain all the possibilities that may force a person to spend down their assets and go on Medicaid, and what to do to avoid that possibility.

Informal Care by Family or Friends

Long before Social Security or Medicare, family members have taken care of elders because no other option existed. Even with Social Security and Medicare, however, the many elderly are still being taken care of by family and friends rather than living in a nursing home. In the past, traditional caregivers were women in the home. However, as women continue to enter the work force they have less time to devote to elderly family members. Still, family or friends primarily provide informal care.

For homemaker and personal care benefits to be paid out to the policyholder must be a formal plan of care created, submitted, and approved.

Medicare - Summary

Summary View: (2011 Figures Shown) Medicare is a national health insurance program for people 65 years and older, younger disabled people and people with permanent kidney failure. It is divided into two parts, Part A: hospital insurance and Part B: medical insurance. Part A pays for 90 days of hospital in a benefit period. For the first 60 days, Medicare pays all covered expenses except for the deductible of \$1,132. After 60 days, Medicare pays all costs except for coinsurance of \$283 per day. The individual is required to pay the coinsurance. After 90 days, a person may use lifetime "reserve days" to help pay the remaining balance. If using "reserve days," a person must still pay a coinsurance of \$566 per day. For Part B, medical insurance, \$96.40 is deducted each month from Social Security and a \$162 deductible for each year.

In 2011, Most Medicare beneficiaries will continue to pay the same \$96.40 or \$110.50 Part B premium amount in 2011. Beneficiaries who currently have the Social Security Administration (SSA) withhold their Part B premium and have incomes of \$85,000 or less (\$170,000 or less for joint filers) will not have an increase in their Part B premium for 2011.

For all others, the standard Medicare Part B monthly premium will be \$115.40 in 2011, which is a 4.4% increase over the 2010 premium. The Medicare Part B premium is increasing in 2011 due to possible increases in Part B costs. If your income is above \$85,000 (single) or \$170,000 (married couple), then your Medicare Part B premium may be higher than \$115.40 per month.

Social Security will use the income reported two years ago on your IRS income tax return to determine your premium (if unavailable, SSA will use income from three years ago). For example, the income reported on your 2009 tax return will be used to determine your monthly Part B premium in 2011. If your income has decreased since 2009, you can ask that the income from a more recent tax year be used to determine your premium, but you must meet certain criteria.

The chart below shows the Part B monthly premium amounts based on income. These amounts change each year. There may be a late-enrollment penalty.

You Pay	If Your Yearly Income is	
	<i>Single</i>	<i>Married Couple</i>
\$96.40 if beneficiary had SSA withhold in 2009 \$110.50 if beneficiary was new in 2010 and had SSA withhold \$115.40 for all others	\$85,000 or less	\$170,000 or less
\$161.50 (increased by \$46.10 due to IRMAA)	\$85,001-\$107,000	\$170,001-\$214,000
\$230.70 (increased by \$115.30 due to IRMAA)	\$107,001-\$160,000	\$214,001-\$320,000
\$299.90 (increased by \$184.50 due to IRMAA)	\$160,001-\$214,000	\$320,001-\$428,000
\$369.10 (increased by \$253.70 due to IRMAA)	Above \$214,000	Above \$428,000

These limits will reduce Medicare costs by an estimated \$8+ billion over the next five years and \$20+ billion over the next 10 years, improving Medicare's sustainability to provide effective coverage for all eligible persons in the future.

Premiums and Deductibles for 2011

- Most people do not pay a monthly Part A premium because they or a spouse has 40 or more quarters of Medicare-covered employment.
- The Part A premium is \$248.00 per month for people having 30-39 quarters of Medicare-covered employment.
- The Part A premium is \$450.00 per month for people who are not otherwise eligible for premium-free hospital insurance and have less than 30 quarters of Medicare-covered employment. (paid by about 1 percent of beneficiaries)
- Part A deductible: \$1,132
- Part B standard premium: Most beneficiaries pay \$96.40 or \$110.50 premium amount in 2011. Beneficiaries who currently have the Social Security Administration (SSA) withhold their Part B premium and have incomes of \$85,000 or less (or \$170,000 or less for joint filers) did not have an increase in their Part B premium in 2011.
- Part B deductible: \$162

Skilled Nursing Facility Coinsurance

- \$141.50 per day for days 21 through 100 each benefit period.

Background

The Centers for Medicare & Medicaid Services (CMS) updates the premiums, deductibles, and co-payments made by Medicare beneficiaries each year. These adjustments are made according to formulas set by statute. The law requires that the standard monthly premium for Medicare Part B must be sufficient to cover 25 percent of the program's costs for aged beneficiaries, including the costs of maintaining a reserve against unexpected spending increases. The federal government pays the remaining 75 percent. Statutory formulas are also used to determine the Medicare Part B deductible, the Part A deductible for hospital stays, and other enrollee contributions.

As required in the Medicare Modernization Act, in 2007, single beneficiaries with annual incomes over \$85,000 and married couples with incomes over \$170,000 will pay a

higher percentage of the cost of Medicare Part B coverage, reducing Medicare's share. These higher-income beneficiaries will pay a monthly premium equal to 35, 50, 65, or 80 percent of the total cost, depending on their income level, by the end of the 3-year transition period.

For 2011, the higher-income beneficiaries will be responsible for one-third of the income-related monthly adjustment amount.

Medicare Premiums and Beneficiary Out-of-Pocket Costs in 2011

The Medicare Part B premiums and deductible apply to all Part B beneficiaries, but other factors provide additional financial support for beneficiaries. One important factor in reducing beneficiary total out-of-pocket medical costs is Medicare's new prescription drug benefit, which provides an average of around \$1,200 in savings.

Beneficiaries with incomes below 135 percent of poverty and limited resources are eligible for subsidies that pay for some or all of their Medicare premiums. As a result, about one in four Medicare beneficiaries can get extra assistance that enables them to pay little or no premium for Part B, and even more beneficiaries are eligible for assistance with their Part D premiums. Beneficiaries with limited incomes may also receive assistance with their Part B and Part D copayments. Most beneficiaries living on incomes that come only from their monthly Social Security check are eligible for this extra help in reducing their out-of-pocket costs. Consequently, beneficiaries living only on a Social Security check usually pay a significantly smaller portion of their income toward Medicare coverage than Medicare premiums and average spending would suggest. In addition, for the vast majority of beneficiaries, the amount of the Social Security cost of living increase will be much greater than the additional premium they will pay. Based on the Trustees Report baseline, the average Social Security benefit check has been projected to increase by an estimated \$34.

In addition, more beneficiaries than ever have Medicare Advantage plans available where they live. These plans generally offer additional coverage at a lower cost to

beneficiaries than the standard Medicare drug benefit in 2011, as well as additional benefits and reduced copayments for Medicare-covered services. On average, these plans save beneficiaries around \$82 a month compared to enrollment in fee-for-service Medicare for Part A and B benefits. For 2011, the average Part D premium for Medicare Advantage plans will be around \$11 less than for stand-alone Part D plans. Many MA plans also use a portion of the savings from Part A and B coverage to further reduce their Part D premiums.

Information on these and other programs that can help beneficiaries lower their out-of-pocket costs is available at 1-800-MEDICARE (1-800-633-4227), and for hearing and speech impaired at TTY/TTD: 1-877-486-2048.

Reasons for Increase in Part B Premium

The new Part B premium in 2011 of \$96.40 is the same amount projected in the 2010. The most recent data indicate that actual incurred historical Part B spending was lower than indicated by preliminary data and historical trends, resulting in a lower than projected Part B premium for 2011. In particular, physician claims are being paid more quickly than in the past, which could be attributable to more timely submission of electronic claims and steps to promote quality performance through competition among the Part B carriers.

Additional Details on Other Announced Premiums and Deductibles

Part B Deductible: The Part B deductible was increased to \$110 from \$100 in 2005 and was subsequently indexed to the increase in the average cost of Part B services for aged beneficiaries, as part of the Medicare Modernization Act. In 2011, the Part B deductible will be \$162, compared to \$155 in 2010, a 4.0 percent increase.

Part A Deductible: Medicare Part A pays for inpatient hospital, skilled nursing facility, hospice, and certain home health care. The \$1132 deductible (2011), paid by the beneficiary when admitted as a hospital inpatient, is an increase of \$32 from \$1,100 in

2011. The Part A deductible is the beneficiary's only cost for up to 60 days of Medicare-covered inpatient hospital care in a benefit period. Beneficiaries must pay an additional \$283 per day for days 61 through 90 in 2011, and \$566 per day for hospital stays beyond the 90th day for lifetime reserve days. This compares with \$275 and \$550 in 2011. The daily coinsurance for the 21st through 100th day in a skilled nursing facility will be \$141.50 in 2011, up from \$137.50 in 2011.

Part A Premium: About 99 percent of Medicare beneficiaries do not pay a premium for Part A services, since they have at least 40 quarters for Medicare-covered employment. However, seniors who have fewer than 30 quarters of covered employment, and certain people under age 65 with disabilities may obtain Part A coverage by paying a monthly premium set according to a statutory formula. This premium will be \$450 per month for 2011, vs. \$461, an decrease of \$22 from 2010. In addition, seniors with 30 to 39 quarters of covered employment, and certain disabled persons with 30 or more quarters of covered employment, will pay a premium of \$248 in 2011, compared to \$254 in 2010.

Long-Term Care Benefits of Medicare

Medicare has minimal benefits for long-term care. Regulations are that after a stay in a hospital for three consecutive days (does not include the day of discharge), you may qualify for an approved Medicare skilled nursing facility (SNF) with these stipulations:

- You must require daily skilled care that can only be given in a skilled nursing facility on an inpatient basis.
- Be admitted to the skilled nursing facility for the same condition you were treated for in the hospital.
- Be admitted to the facility within 30 days of discharge from hospital.
- Be certified by a medical professional as needing skilled nursing or skilled rehabilitation services on a daily basis.

Medicare Part A pays for up to 100 days in a skilled nursing facility. However, only the first 20 days are paid in full. Medicare pays the next 80 days, except for a daily coinsurance of \$141.50 (2011 figures). After these latter 80 days, the patient is responsible for all charges.

Medicare does not pay for custodial care such as assistance in walking, getting in and out of bed, eating, dressing, bathing and taking medication. Medicare will pay the full cost of medically necessary home health visits by an approved Medicare health agency. Medicare home health will not cover full-time nursing, meals delivered to the home, homemaker services to assist in meeting personal care or house keeping needs.

Medicare-Supplement

A Medicare Supplement is specifically designed to supplement Medicare's benefits and is regulated by federal and state law. Each state (with the exception of Minnesota, Massachusetts and Wisconsin) has limited the number of different Medicare supplements to no more than 10 standard plans. Each plan has a basic benefit package with each having a different combination of additional benefits. The plans are A to J, with Plan A being the basic plan and Plans B through J having different benefits. There are no long-term care benefits in any plan, however Plans C through J provide for skilled nursing with a coinsurance amount of \$141.50 per day for days 21 through 100. Plans D, G, I and J provide for home coverage benefits up to \$1,600 per year. This includes activities of daily living.

Medicaid in Relation to Long-Term Care

Medicaid is a state-based program administered by the state's departments of health services and derived from the federal health insurance program Medicaid.

Medicaid insures low-income families and individuals against the cost associated with acute conditions and chronic long-term care. Acute care expenditures are made for younger families and children, but the fastest growing portion of the Medicaid budget is the costs of long-term care for the elderly and disabled. The program is restricted to

individuals who are impoverished. Many times these individuals had savings and long-term care insurance, but because the costs of health care have risen so dramatically in the last few years, both savings and insurance were exhausted. If care is still needed, the only recourse for a person is to apply for Medicaid.

Elderly people who have Medicare might believe it pays for long-term care expenses, but it does not.

Eligibility for Medicaid is established by the state's departments of health services. The items important for eligibility are:

- Income of each person in family
- Own home
- Income from real estate (rentals, etc.)
- Miscellaneous assets such as savings, stock and bonds or other investments.

The Departments of Health Services determine if a person is eligible for long-term care. If he or she is eligible, an allowance of generally \$60 a month is distributed for the individual. If there is more income, it must be applied towards the monthly costs of care.

Different rules apply to married couples with one spouse needing long-term care. The person living at home may keep \$109,560 (2011) worth of assets and have an income of up to \$2,739 (2011) per month (if the income is only from the spouse in long-term care). If the person not receiving long-term care has his or her own income, this is not counted towards assets. One car may be kept and rental properties providing for the healthy spouse can also be kept. There are other complex regulations that change annually.

These rules apply to 2011 eligibility requirements only. Each year legislators may change eligibility rules and regulations.

Before 1996, there were individuals who attempted to transfer assets to their children or relatives in order to “spend-down” so they could qualify for Medicaid. There were also attorneys and counselors who claimed they could show individuals how to do this legally. However, the Health Insurance Portability and Accountability Act (HIPAA) enacted legislation with criminal penalties for anyone attempting to transfer assets to qualify for Medicaid. This regulation also addressed counselors who advertised this service.

Medicaid regulations provide for the recovery of any costs by Medicaid when the long-term care recipient dies, unless the well spouse is still living in the home. The recovery of the costs will not take place until both spouses have died. If the patient is single, the recovery will be paid upon death of the patient.

Alternative Financing Options

“Reverse” Annuity Mortgages

Reverse Annuity Mortgages are a fairly new way of helping seniors who do not have the finances to pay for long-term care. These are only available to people whom have a home that has been paid off and there is an adequate equity available. With this option, a bank or other financial institution agrees to a reverse annuity mortgage with an individual. The equity in the home is established with an appraiser, the bank buys the equity and pays the owner a predetermined payment each month. After the sale, the bank buys an annuity from an insurance company. The owner can live there as long as his or her spouse is still living. After both spouses have passed away, the bank owns the house.

A reverse mortgage is a type of home equity loan that allows one to convert some of the equity in a home into cash while continuing to own the home. Reverse mortgages operate like traditional mortgages, only in reverse. Rather than paying a lender each month, the lender pays an individual. Reverse mortgages differ from home equity loans in that most reverse mortgages do not require any repayment of principal, interest, or

servicing fees as long as he or she lives in the home. The reverse mortgage's benefit is that it allows homeowners who are age 62 and over to keep living in their homes and to use their equity for whatever purpose they choose. A reverse mortgage might be used to cover the cost of home health care, to pay off an existing mortgage to stop a foreclosure, or to support children or grandchildren.

“Forward” Mortgage

With a “forward” mortgage, the kind of loan most people have on homes, payments are made on a loan balance that compounds interest. This balance is reduced by our payments (usually in monthly installments over a period of years), until it is finally paid off. A reverse mortgage, true to its name, reverses that process. It is a loan against a home's equity (net worth—market value minus what is owed on it) in which there are no payments required, so the principal balance actually increases. (Interest is added to the principal loan each month.) The balance, however, is guaranteed never to exceed the net value of the home. This enables the “house rich, cash poor” older homeowner to utilize the home's equity, turning it into spendable cash with no monthly payments, while owning and living in the home. The borrower retains ownership of the home, as with any other mortgage, and remains responsible for taxes, repairs and maintenance. When the borrower no longer occupies the home—whether because he moves, sells, or dies—the loan must be paid off as would any other mortgage. The loan is then due with interest and loan fees. It is paid off by the proceeds of the sale, or through refinancing as a regular (forward) mortgage by heirs. The cash generated by the reverse mortgage is not taxable income. It does not affect Social Security, Medicare benefits, or Medicaid benefits. Interest on a reverse mortgage is not deductible for income tax purposes until the loan is paid off. The money may be used for any purpose the borrower wants, including long-term care or the spouse of an individual who is on Medicaid.

Loan fees reduce the amount of cash available; fees are not due until the loan is paid back (they are financed—added to the total loan amount). The funds can be drawn out in lump sum, monthly advances, or a line of credit. Reverse mortgages are usually adjustable rate mortgages, or ARMs. Interest accrues according to an interest rate that

adjusts annually, within a limited margin. These changes are tied to an independent index not controlled by the lender (usually based on a national measure like the prime rate). To be eligible for a reverse mortgage, the younger borrower must be 62 years of age or older, the home must be owner-occupied, and there must be accessible equity in the property. The borrower does not need to have any income to qualify, but must own the home free and clear, or close to it. Specific loans may have other requirements. With few exceptions, the borrowers also must undergo a consumer education session with an approved agency not affiliated with the lender.

Viaticals

Viatical settlements may not be a comprehensive financial solution to normal long-term care needs for most people since they apply only to terminal illness and chronically ill issues. Where accelerated life insurance benefits (discussed earlier) provide for partial payout of death benefits under certain circumstances as part of the policy benefits, viaticals relate to third party companies advancing people portions of their death benefit proceeds on their life insurance policies under certain circumstances such as terminal illness and/or chronic illnesses.

Most state regulations define a "Viatical settlement" as any person that enters into an agreement with a viator under the terms of which the viatical settlement provider pays compensation or anything of value, in return for the assignment, transfer, sale, devise, or bequest of the death benefit or ownership of the insurance policy or certificate of insurance to the viatical settlement provider.

It is significant to note that the tax treatment of accelerated death benefits has changed as a result of HIPPA (Health Insurance Portability and Accountability Act). Signed by President Clinton in 1996, this new law provides for tax-free treatment of accelerated death benefits for terminal and chronically ill people paid directly by insurance companies. This should serve as another reason for the seriously ill to make use of accelerated provisions in their life policies for current "living benefits," including long-term care where permitted. Caution must be advised, however, in how one defines terminal or chronic illness to the satisfaction of the Internal Revenue Service. A real

boost to viatical settlements should come as a result of HIPPA of 1996 which allows people diagnosed with a terminal illness to sell their life insurance policies to viatical settlement companies for a tax free lump sum payment. This tax-free provision will apply ONLY to people whose life expectancy is less than 24 months and the purchasing company must be licensed by the state in which the viator (seller) resides.

Life Insurance Options

Another option is to obtain a rider on a life insurance policy for a pre-death accelerated benefit. This may or may not be tax-free. A tax advisor should be consulted. Each insurance company has different limits for benefits, whereas others have a ceiling on the amount to be paid out as a living benefit. These benefits are later taken from death benefits when the person dies. This type of financial decision should be thoroughly researched prior to any decision.

Riders or supplementary benefits are additions to an insurance policy which expand or limit the benefits payable and which become a part of the insurance contract. Riders are usually attached to insurance policies in order to provide additional benefits. Some life insurers package different types of benefits as separate riders so that they can have the flexibility of combining these riders to form contracts with different sets of benefits to suit different customers without having to go through the process of designing a new policy.

Only 4% of people over age 65 have LTC insurance. People generally apply for these policies at age 65 or 70. Only those in good health are accepted. Then these healthy 65- and 70-year-olds pay \$1,000, \$1,500, or \$2,000 every year for coverage. By age 85, they've probably paid \$20,000 or more in premiums. Unless their health deteriorates markedly and they need long-term home care, they'll never collect any benefits. From one point of view, that money is the cost of a 20-year asset protection; from another, it's a waste of \$20,000. If an individual takes the latter perspective, he or she may prefer to buy life insurance with an "LTC" rider" or to add such a rider to a life insurance policy that they already own. LTC riders can be added to permanent life policies. That includes whole life, universal life, and variable life policies.

Loans on home equity

H.R. 1776 also allows homeowners to waive counseling for a refinance under certain circumstances; this would also save time and costs. In addition, H.R. 1776 authorizes HUD to waive the FHA mortgage insurance premium entirely for loans used to purchase long-term care insurance or long-term home health care services, as long as the funds provided by the HECM are paid out over time as the expenses are incurred. This would encourage seniors to purchase long-term care insurance or home health care, which could result in substantial savings on health care costs later in life as well as savings to the federal government in reduced Medicaid expenditures. H.R. 1776 authorizes a study of the feasibility of a single national FHA loan limit for Home Equity Conversion Mortgage (HECMs). It is believed a single limit would be more equitable for seniors whose day-to-day living and healthcare expenses do not vary across the country as widely as housing prices, and would enable lenders to engage in more effective nationwide marketing of the program.

Savings

Individuals with substantial savings can pay for long-term care. However, with health care costs increasing each year, it is important to evaluate whether enough savings are in place.

Apathy (or doing nothing)

Apathy poses the greatest risk. There are yearly nursing- home rises in costs. In 1980, nursing home cost only around \$43.00 daily. By 1996 it had risen to \$116.00 a day. Today the average cost is generally anywhere from \$150.00 to \$270.00 a day depending on the area and care needed. Even if long-term care is never needed, it is wise to prepare for the unknown, otherwise the patient or the family will be burdened by the costs of long-term care.

Purchasing a LTC policy does not ensure that Medicaid won't be needed.

Agents should be prepared to explain the pros and cons of various situations to help people acknowledge the potential need to apply for Medicaid.

Whether it is to their advantage or not depends upon the particular circumstances. People who are likely not able to afford premiums or unable to absorb even a moderate increase should not purchase a long-term care policy. The safety net of Medicaid may be their only option.

Please return to the Course Player in order to take Lesson 15 Quiz.

Appendix One

Outline of Coverage

[COMPANY NAME]

[ADDRESS - CITY & STATE]

[TELEPHONE NUMBER]

LONG-TERM CARE INSURANCE

OUTLINE OF COVERAGE

[Policy Number or Group Master Policy and Certificate Number]

[Except for policies or certificates which are guaranteed issue, the following caution statement, or language substantially similar, must appear as follows in the outline of coverage.]

Caution: The issuance of this long-term care insurance [policy] [certificate] is based upon your responses to the questions on your application. A copy of your [application] [enrollment form] [is enclosed] [was retained by you when you applied]. If your answers are incorrect or untrue, the company has the right to deny benefits or rescind your policy. The best time to clear up any questions is now, before a claim arises! If, for any reason, any of your answers are incorrect, contact the company at this address: [insert address]

3. This policy is [an individual policy of insurance]([a group policy] which was issued in the [indicate jurisdiction in which group policy was issued]).
4. PURPOSE OF OUTLINE OF COVERAGE. This outline of coverage provides a very brief description of the important features of the policy. You should compare this outline of coverage to outlines of coverage for other policies available to you. This is not an insurance contract, but only a summary of coverage. Only the individual or group policy contains governing contractual provisions. This means that the policy or group policy sets forth in detail the rights and obligations of both you and the insurance company. Therefore, if you purchase this coverage, or any other coverage, it is important that you READ YOUR POLICY (OR CERTIFICATE) CAREFULLY!
5. FEDERAL TAX CONSEQUENCES.

This [POLICY] [CERTIFICATE] is intended to be a federally tax-qualified long-term care insurance contract under Section 7702B(b) of the Internal Revenue Code of 1986, as amended.

OR

Federal Tax Implications of this [POLICY] [CERTIFICATE]. This [POLICY] [CERTIFICATE] is not

intended to be a federally tax-qualified long-term care insurance contract under Section 7702B(b) of the Internal Revenue Code of 1986 as amended. Benefits received under the [POLICY] [CERTIFICATE] may be taxable as income.

6. TERMS UNDER WHICH THE POLICY OR CERTIFICATE MAY BE CONTINUED IN FORCE OR DISCONTINUED.

(a) [For long-term care health insurance policies or certificates describe one of the following permissible policy renewability provisions:

(1) Policies and certificates that are guaranteed renewable shall contain the following statement:] RENEWABILITY: THIS POLICY [CERTIFICATE] IS GUARANTEED RENEWABLE. This means you have the right, subject to the terms of your policy, [certificate] to continue this policy as long as you pay your premiums on time. [Company Name] cannot change any of the terms of your policy on its own, except that, in the future, IT MAY INCREASE THE PREMIUM YOU PAY.

(2) [Policies and certificates that are non-cancellable shall contain the following statement:] RENEWABILITY: THIS POLICY [CERTIFICATE] IS NON-CANCELLABLE. This means that you have the right, subject to the terms of your policy, to continue this policy as long as you pay your premiums on time. [Company Name] cannot change any of the terms of your policy on its own and cannot change the premium you currently pay. However, if your policy contains an inflation protection feature where you choose to increase your benefits, [Company Name] may increase your premium at that time for those additional benefits.

(d) [For group coverage, specifically describe continuation/conversion provisions applicable to the certificate and group policy;]

(e) [Describe waiver of premium provisions or state that there are not such provisions.]

5. TERMS UNDER WHICH THE COMPANY MAY CHANGE PREMIUMS.

[In bold type larger than the maximum type required to be used for the other provisions of the outline of coverage, state whether or not the company has a right to change the premium, and if a right exists, describe clearly and concisely each circumstance under which the premium may change.]

6. TERMS UNDER WHICH THE POLICY OR CERTIFICATE MAY BE RETURNED AND PREMIUM REFUNDED.

- (c)** [Provide a brief description of the right to return—"free look" provision of the policy.]
- (d)** [Include a statement that the policy either does or does not contain provisions providing for a refund or partial refund of premium upon the death of an insured or surrender of the policy or certificate. If the policy contains such provisions, include a description of them.]

7. THIS IS NOT MEDICARE SUPPLEMENT COVERAGE. If you are eligible for Medicare, review the Medicare Supplement Buyer's Guide available from the insurance company.

- (c)** [For agents] Neither [insert company name] nor its agents represent Medicare, the federal government or any state government.
- (d)** [For direct response] [insert company name] is not representing Medicare, the federal government or any state government.

8. LONG-TERM CARE COVERAGE. Policies of this category are designed to provide coverage for one or more necessary or medically necessary diagnostic, preventive, therapeutic, rehabilitative, maintenance, or personal care services, provided in a setting other than an acute care unit of a hospital, such as in a nursing home, in the community or in the home.

This policy provides coverage in the form of a fixed dollar indemnity benefit for covered long-term care expenses, subject to policy [limitations] [waiting periods] and [coinsurance] requirements. [Modify this paragraph if the policy is not an indemnity policy.]

9. BENEFITS PROVIDED BY THIS POLICY.

- (e)** [Covered services, related deductibles, waiting periods, elimination periods and benefit maximums.]
- (f)** [Institutional benefits, by skill level.]
- (g)** [Non-institutional benefits, by skill level.]
- (h)** Eligibility for Payment of Benefits

[Activities of daily living and cognitive impairment shall be used to measure an insured's need for long-term care and must be defined and described as part of the outline of coverage.]

[Any additional benefit triggers must also be explained. If these triggers differ for different benefits, explanation of the triggers should accompany each benefit description. If an attending physician or other specified person must certify a certain level of functional dependency in order to be eligible for benefits, this too must be specified.]

10. LIMITATIONS AND EXCLUSIONS.

[Describe:

- (f)** Pre-existing conditions;
- (g)** Non-eligible facilities and provider;
- (h)** Non-eligible levels of care (e.g., unlicensed providers, care or treatment provided by a family member, etc.);
- (i)** Exclusions and exceptions;
- (j)** Limitations.]

[This section should provide a brief specific description of any policy provisions which limit, exclude, restrict, reduce, delay, or in any other manner operate to qualify payment of the benefits described in Number 6 above.]

THIS POLICY MAY NOT COVER ALL THE EXPENSES ASSOCIATED WITH YOUR LONG-TERM CARE NEEDS.

11. RELATIONSHIP OF COST OF CARE AND BENEFITS. Because the costs of long-term care services will likely increase over time, you should consider whether and how the benefits of this plan may be adjusted. [As applicable, indicate the following:

- (f)** That the benefit level will not increase over time;
- (g)** Any automatic benefit adjustment provisions;
- (h)** Whether the insured will be guaranteed the option to buy additional benefits and the basis upon which benefits will be increased over time if not by a specified amount or percentage;
- (i)** If there is such a guarantee, include whether additional underwriting or health screening will be required, the frequency and amounts of the upgrade options, and any significant

restrictions or limitations;

- (j) And finally, describe whether there will be any additional premium charge imposed, and how that is to be calculated.]

12. ALZHEIMER'S DISEASE AND OTHER ORGANIC BRAIN DISORDERS.

[State that the policy provides coverage for insureds clinically diagnosed as having Alzheimer's disease or related degenerative and dementing illnesses. Specifically describe each benefit screen or other policy provision which provides preconditions to the availability of policy benefits for such an insured.]

16. PREMIUM.

- [(a) State the total annual premium for the policy;
- (b) If the premium varies with an applicant's choice among benefit options, indicate the portion of annual premium which corresponds to each benefit option.]

17. ADDITIONAL FEATURES.

- [(a) Indicate if medical underwriting is used;
- (b) Describe other important features.]

18. CONTACT THE STATE SENIOR HEALTH INSURANCE ASSISTANCE PROGRAM IF YOU HAVE GENERAL QUESTIONS REGARDING LONG-TERM CARE INSURANCE. CONTACT THE INSURANCE COMPANY IF YOU HAVE SPECIFIC QUESTIONS REGARDING YOUR LONG-TERM CARE INSURANCE POLICY OR CERTIFICATE.

Appendix Two

Long-Term Care Insurance Model Act

Notice to Applicant Regarding Replacement

OF INDIVIDUAL ACCIDENT AND SICKNESS OR LONG-TERM CARE INSURANCE

[Insurance company's name and address]

SAVE THIS NOTICE - IT MAY BE IMPORTANT TO YOU IN THE FUTURE.

According to [your application] [information you have furnished], you intend to lapse or otherwise terminate existing accident and sickness or long-term care insurance and replace it with an individual long-term care insurance policy to be issued by [company name] Insurance Company. Your new policy provides thirty (30) days within which you may decide, without cost, whether you desire to keep the policy. For your own information and protection, you should be aware of and seriously consider certain factors which may affect the insurance protection available to you under the new policy.

You should review this new coverage carefully, comparing it with all accident and sickness or long-term care insurance coverage you now have, and terminate your present policy only if, after due consideration, you find that purchase of this long-term care coverage is a wise decision.

STATEMENT TO APPLICANT BY AGENT [BROKER OR OTHER REPRESENTATIVE]: (Use additional sheets, as necessary.)

I have reviewed your current medical or health insurance coverage. I believe the replacement of insurance involved in this transaction materially improves your position. My conclusion has taken into account the following considerations, which I call to your attention:

1. Health conditions that you may presently have (pre-existing conditions), may not be immediately or fully covered under the new policy. This could result in denial or delay in payment of benefits under the new policy, whereas a similar claim might have been payable under your present policy.
2. State law provides that your replacement policy or certificate may not contain new pre-existing conditions or probationary periods. The insurer will waive any time periods applicable to pre-existing conditions or probationary periods in the new policy (or coverage) for similar benefits to the extent such time was spent (depleted) under the original policy.
3. If you are replacing existing long-term care insurance coverage, you may wish to secure the advice your present insurer or its agent regarding the proposed replacement of your present policy. This is not only your right, but it is also in your best interest to make sure you understand all the relevant

factors involved in replacing your present coverage.

4. If, after due consideration, you still wish to terminate your present policy and replace it with new coverage, be certain to truthfully and completely answer all questions on the application concerning your medical health history. Failure to include all material medical information on an application may provide a basis for the company to deny any future claims and to refund your premium as though your policy had never been in force. After the application has been completed and before you sign it, reread it carefully to be certain that all information has been properly recorded.

(Signature of Agent, Broker or Other Representative)

[Typed Name and Address of Agent or Broker]

The above "Notice to Applicant" was delivered to me on:

(Applicant's Signature)

(Date)

**NOTICE TO APPLICANT REGARDING REPLACEMENT
OF ACCIDENT AND SICKNESS OR LONG-TERM CARE INSURANCE**

[Insurance company's name and address]

SAVE THIS NOTICE IT MAY BE IMPORTANT TO YOU IN THE FUTURE.

According to [your application] [information you have furnished], you intend to lapse or otherwise terminate existing accident and sickness or long-term care insurance and replace it with the long-term care insurance policy delivered herewith issued by [company name] Insurance Company. Your new policy provides thirty (30) days within which you may decide, without cost, whether you desire to keep the policy. For your own information and protection, you should be aware of and seriously consider certain factors which may affect the insurance protection available to you under the new policy.

You should review this new coverage carefully, comparing it with all accident and sickness or long-term care insurance coverage you now have, and terminate your present policy only if, after due consideration, you find that purchase of this long-term care coverage is a wise decision.

1. Health conditions which you may presently have (pre-existing conditions), may not be immediately or fully covered under the new policy. This could result in denial or delay in payment of benefits under the new policy, whereas a similar claim might have been payable under your

present policy.

2. State law provides that your replacement policy or certificate may not contain new pre-existing conditions or probationary periods. Your insurer will waive any time periods applicable to pre-existing conditions or probationary periods in the new policy (or coverage) for similar benefits to the extent such time was spent (depleted) under the original policy.
3. If you are replacing existing long-term care insurance coverage, you may wish to secure the advice of your present insurer or its agent regarding the proposed replacement of your present policy. This is not only your right, but it is also in your best interest to make sure you understand all the relevant factors involved in replacing your present coverage.
4. [To be included only if the application is attached to the policy.] If, after due consideration, you still wish to terminate your present policy and replace it with new coverage, read the copy of the application attached to your new policy and be sure that all questions are answered fully and correctly. Omissions or misstatements in the application could cause an otherwise valid claim to be denied. Carefully check the application and write to [company name and address] within thirty (30) days if any information is not correct and complete, or if any past medical history has been left out of the application.

Appendix Three

Things You Should Know Before You Buy LTC

Things You Should Know Before You Buy Long-Term Care Insurance

Long-Term Care Insurance

- A long-term care insurance policy may pay most of the costs for your care in a nursing home. Many policies also pay for care at home or other community settings. Since policies can vary in coverage, you should read this policy and make sure you understand what it covers before you buy it.
- [You should not buy this insurance policy unless you can afford to pay the premiums every year.] [Remember that the company can increase premiums in the future.]

Drafting Note: For single premium policies, delete this bullet; for non-cancellable policies, delete the second sentence only.

- The personal worksheet includes questions designed to help you and the company determine whether this policy is suitable for your needs.

Medicare

- Medicare does not pay for most long-term care.

Medicaid

- Medicaid will generally pay for long-term care if you have very little income and few assets. You probably should not buy this

policy if you are now eligible for Medicaid.

- Many people become eligible for Medicaid after they have used up their own financial resources by paying for long-term care services.
- When Medicaid pays your spouse's nursing home bills, you are allowed to keep your house and furniture, a living allowance, and some of your joint assets.
- Your choice of long-term care services may be limited if you are receiving Medicaid. To learn more about Medicaid, contact your local or state Medicaid agency.

Shopper's Guide

- Make sure the insurance company or agent gives you a copy of a book called the National Association of Insurance Commissioners' "Shopper's Guide to Long-Term Care Insurance." Read it carefully. If you have decided to apply for long-term care insurance, you have the right to return the policy within 30 days and get back any premium you have paid if you are dissatisfied for any reason or choose not to purchase the policy.

Counseling

- Free counseling and additional information about long-term care insurance are available through your state's insurance counseling program. Contact your state insurance department or department on aging for more information about the senior health insurance counseling program in your state.

Facilities

- Some long-term care insurance contracts provide for benefit payments in certain facilities only if they are licensed or certified, such as in assisted living centers. However, not all states regulate these facilities in the same way. Also, many people move into a different state from where they purchased their long-term care insurance policy. Read the policy carefully to determine what types of facilities qualify for benefit payments, and to determine that payment for a covered service will be made if you move to a state that has a different licensing scheme for facilities than the one in which you purchased the policy.

Appendix Four

Notice of Replacement Policy

Sample

A Notice of Replacement Policy is Required to be Signed by Agents and Applicants

**NOTICE TO APPLICANT REGARDING REPLACEMENT
OF INDIVIDUAL ACCIDENT AND SICKNESS OR LONG-TERM CARE INSURANCE**

[Insurance company's name and address]

SAVE THIS NOTICE! IT MAY BE IMPORTANT TO YOU IN THE FUTURE.

According to [your application] [information you have furnished], you intend to lapse or otherwise terminate existing accident and sickness or long-term care insurance and replace it with an individual long-term care insurance policy to be issued by [company name] Insurance Company. Your new policy provides thirty (30) days within which you may decide, without cost, whether you desire to keep the policy. For your own information and protection, you should be aware of and seriously consider certain factors which may affect the insurance protection available to you under the new policy.

You should review this new coverage carefully, comparing it with all accident and sickness or long-term care insurance coverage you now have, and terminate your present policy only if, after due consideration, you find that purchase of this long-term care coverage is a wise decision.

**STATEMENT TO APPLICANT BY AGENT [BROKER OR OTHER
REPRESENTATIVE]:** (Use additional sheets, as necessary.)

I have reviewed your current medical or health insurance coverage. I believe the replacement of insurance involved in this transaction materially improves your position. My conclusion has taken into account the following considerations, which I call to your attention:

1. Health conditions that you may presently have (pre-existing conditions), may not be immediately or fully covered under the new policy. This could result in denial or delay in payment of benefits under the new policy, whereas a similar claim might have been payable under your present policy.
2. State law provides that your replacement policy or certificate may not contain new pre-existing conditions or probationary periods. The insurer will waive any time periods applicable to pre-existing conditions or probationary periods in the new policy (or coverage) for similar benefits to

the extent such time was spent (depleted) under the original policy.

3. If you are replacing existing long-term care insurance coverage, you may wish to secure the advice of your present insurer or its agent regarding the proposed replacement of your present policy. This is not only your right, but it is also in your best interest to make sure you understand all the relevant factors involved in replacing your present coverage.
4. If, after due consideration, you still wish to terminate your present policy and replace it with new coverage, be certain to truthfully and completely answer all questions on the application concerning your medical health history. Failure to include all material medical information on an application may provide a basis for the company to deny any future claims and to refund your premium as though your policy had never been in force. After the application has been completed and before you sign it, reread it carefully to be certain that all information has been properly recorded.

(Signature of Agent, Broker or Other Representative)

[Typed Name and Address of Agent or Broker]

The above "Notice to Applicant" was delivered to me on:

(Applicant's Signature)

(Date)

**NOTICE TO APPLICANT REGARDING REPLACEMENT
OF ACCIDENT AND SICKNESS OR LONG-TERM CARE INSURANCE**

[Insurance company's name and address]

SAVE THIS NOTICE! IT MAY BE IMPORTANT TO YOU IN THE FUTURE.

According to [your application] [information you have furnished], you intend to lapse or otherwise terminate existing accident and sickness or long-term care insurance and replace it with the long-term care insurance policy delivered herewith issued by [company name] Insurance Company. Your new policy provides thirty (30) days within which you may decide, without cost, whether you desire to keep the policy. For your own information and protection, you should be aware of and seriously consider certain factors which may affect the insurance protection available to you under the new policy.

You should review this new coverage carefully, comparing it with all accident and sickness or long-term care insurance coverage you now have, and terminate your present policy only if, after due consideration,

you find that purchase of this long-term care coverage is a wise decision.

5. Health conditions which you may presently have (pre-existing conditions), may not be immediately or fully covered under the new policy. This could result in denial or delay in payment of benefits under the new policy, whereas a similar claim might have been payable under your present policy.
6. State law provides that your replacement policy or certificate may not contain new pre-existing conditions or probationary periods. Your insurer will waive any time periods applicable to pre-existing conditions or probationary periods in the new policy (or coverage) for similar benefits to the extent such time was spent (depleted) under the original policy.
7. If you are replacing existing long-term care insurance coverage, you may wish to secure the advice of your present insurer or its agent regarding the proposed replacement of your present policy. This is not only your right, but it is also in your best interest to make sure you understand all the relevant factors involved in replacing your present coverage.
8. [To be included only if the application is attached to the policy.] If, after due consideration, you still wish to terminate your present policy and replace it with new coverage, read the copy of the application attached to your new policy and be sure that all questions are answered fully and correctly. Omissions or misstatements in the application could cause an otherwise valid claim to be denied. Carefully check the application and write to [company name and address] within thirty (30) days if any information is not correct and complete, or if any past medical history has been left out of the application.

(Company name)

COMPARISON TO YOUR CURRENT COVERAGE: I have reviewed your current long-term care coverage. To the best of my knowledge, the replacement of insurance involved in this transaction materially improves your position for the following reasons:

- _____ Additional or different benefits (please specify)_____
- _____ No change in benefits but lower premiums.
- _____ Fewer benefits and lower premiums.
- _____ Other (please specify)_____

(Signature of Agent and Name of Insurer)

(Signature of Applicant)

(Date)

The replacement notice shall include the following statement except when the replacement coverage is group insurance.

For group coverage not subject to the 30-day return provision, the notice shall be modified to reflect the appropriate time period in which the policy may be returned and premium refunded.

COMPARISON TO YOUR CURRENT COVERAGE: I have reviewed your current long-term care coverage. To the best of my knowledge, the replacement of insurance involved in this transaction materially improves your position for the following reasons:

_____ Additional or different benefits (please specify)_____

_____ No change in benefits but lower premiums.

_____ Fewer benefits and lower premiums.

_____ Other (please specify)_____

(Signature of Agent and Name of Insurer)

Appendix Five

Long-Term Care Suitability Letter

Sample

FORMS REQUIRED BY THE NATIONAL ASSOCIATION OF INSURANCE COMMISSIONERS (NAIC) AND STATE LAW

Most State laws and NAIC require that a Long-Term Care Insurance Worksheet be included so as to help you and the insurance company to see if you should buy a long-term care insurance policy.

A Long-Term Care Insurance Suitability Letter is also required along with the Long-Term Care Insurance Worksheet which is included below.

(Source: 1997 National Association of Insurance Commissioners)

Long-Term-Care Suitability Letter

Dear (Applicant):

Your recent applicant for long-term care insurance included a “personal worksheet”, which asked questions about your finances and your reasons for buying long-term care insurance. For your protection, state law requires us to consider this information when we review your application, to avoid selling a policy to those who do not need coverage.

(Your answers indicate that long-term care insurance may not meet your financial needs. We suggest you review the information provided along with the application, including the booklet “Shoppers’s guide to long-term Care Insurance”. Your state insurance department also has information about long-term care insurance and may be

able to refer you to a counselor free of charge who can help you decide whether to buy this policy.)

(You chose not to provide any financial information for us to review)

Note: Insurer choose the paragraph that applies.

We have suspended our final review of your application. If, after careful consideration, you still believe this policy is what you want, check the appropriate box below and return this letter to us within the next 60 days. We will then continue reviewing your application and issue a policy if you meet our medical standards.

If we do not hear from you within the next 60 days, we will close your file and not issue you a policy. You should understand that you will not have any coverage until we hear back from you, approve your application and issue you a policy.

Please check one box and return in enclosed envelope.

- **Yes**, (although my worksheet indicate that long-term care insurance may not be a suitable purchase,) I wish to purchase this coverage. Please resume review of my application.

Drafting Note: Delete the phrase in brackets if the applicant did not answer the questions about income.

- **No**, I have decided not to buy a policy at this time.

APPLICANT'S SIGNATURE

DATE

Please return to (issuer) at (address) by (date)

Appendix Six

Long-Term Care Personal Worksheet

Sample

LONG-TERM CARE INSURANCE PERSONAL WORKSHEET

(Source: National Association of Insurance Commissioners, Suitability, Section 21, 1997)

This worksheet is for the purpose of establishing suitability for a prospect to buy long-term insurance because not everyone needs this type of insurance. The State of (Name State) requires all insurance companies to have applicants fill this out before buying long-term care insurance. Must be in 12-point type at time of application.

People buy long-term care insurance for many reasons. Some don't want to use their own assets to pay for long-term care. Some buy insurance to make sure they can choose the type of care they get. Others don't want their family to have to pay for care or don't want to go on Medicaid. But long-term care insurance may be expensive, and may not be right for everyone.

By state law, the insurance company must fill out part of the information on this worksheet and **ask** you to fill out the rest to help you and the company decide if you should buy this policy.

Premium Information

Policy Form Numbers _____

The premium for the coverage you are considering will be \$_____per month, or \$_____per year. (A one-time single premium of \$_____.)

Type of Policy (non-cancellable/guaranteed renewable): _____

The Company's Right to Increase Premiums:

(The company cannot raise your rates on this policy.) (The company has a right to increase premiums in the future provided it raises rates for all policies in the same class.) The company has sold long-term care insurance since (year) and has sold this policy since (year). (The last rate increase for this policy in this state was in (year), when premiums went up by an average of (%). (The company has not raised its rates for this policy.)

***Drafting Note:** The issuer shall use the bracketed sentence or sentences applicable to the product offered. If a company includes a statement regarding not having raised rates, it must disclose the company's rate increases under prior policies providing essentially similar coverage.*

Rate Increase History

The company has sold long-term care insurance since [year] and has sold this policy since [year]. [The company has never raised its rates for any long-term care policy it has sold in this state or any other state.] [The company has not raised its rates for this policy form or similar policy forms in this state or any other state in the last 10 years.] [The company has raised its premium rates on this policy form or similar policy forms in the last 10 years. Following is a summary of the rate increases.]

***Drafting Note:** A company may use the first bracketed sentence above only if it has never increased rates under any prior policy forms in this state or any other state. The issuer shall list each premium increase it has instituted on this or similar policy forms in this state or any other state during the last 10 years. The list shall provide the policy form, the calendar years the form was available for sale, and the calendar year and the amount (percentage) of each increase. The insurer shall provide minimum and maximum percentages if the rate increase is variable by rating characteristics. The insurer may provide, in a fair manner, additional explanatory information as appropriate.*

Drafting Note: The issuer shall use the bracketed sentence unless the policy is fully paid up or is a non-cancellable policy.

Questions Related to Your Income

How will you get the money to pay each year's premium?

☐ From My Income ☐ From My Savings/Investments ☐ My Family Will Pay

(Have you considered whether you could afford to keep this policy if the premiums are raised, for example by 20%?)

What is your annual income? (check one)

☐ Under \$10,000 ☐ (\$10-20,000) ☐ (\$20-30,000) ☐ (\$30,50,000) ☐ Over \$50,000

Note: The insurer may choose the number to put in brackets to fit its suitability standards.

How do you expect your income to change over the next 10 years? (check one)

☐ No change ☐ Increase ☐ Decrease

If you will be paying premiums with money received only from your own income, a rule of thumb is that you may not be able to afford this policy if the premiums will be more than 7% of your income.

Will you buy inflation protection? (check one) ☐ Yes ☐ No

If not, have you considered how you will pay for the difference between future costs and your daily benefit amount?

☐ From my Income ☐ From my Savings/Investments ☐ My Family will Pay

The national average annual cost of care in [insert year] was [insert \$ amount], but this figure varies across the country. In ten years the national average annual cost would be about [insert \$ amount] if costs increase 5% annually.

Drafting Note: The projected cost can be based on federal estimates in a current year. In the above statement, the second figure equals 163% of the first figure.

What elimination period are you considering?

- ☐ Number of days ☐ Approximate cost \$for that period of care

How are you planning to pay for your care during the elimination period? (check one)

- ☐ From my Income ☐ From my Savings/Investments ☐ My Family will Pay

Questions Related to Savings and Investments

Not counting your home, what is the approximate value of all your assets (your savings and investments)? (check one)

- ☐ Under \$20,000 ☐ \$20,000-\$30,000 ☐ \$30,000-\$50,000 ☐ Over \$50,000

How do you expect your assets to change over the next ten year? (check one)

- ☐ Stay about the same ☐ Increase ☐ Decrease

If you are buying this policy to protect your assets and your assets are less than \$30,000, you may wish to consider other options for financing your long-term care.

Disclosure Statement

The information provided above accurately describes my financial situation. I choose not to complete this Information.

I acknowledge that the carrier and/or its agent (below) has reviewed this form with me including the premium, premium rate increase history and potential for premium increases in the future. [For direct mail situations, use the following: I acknowledge that I have reviewed this form including the premium, premium rate increase history and potential for premium increases in the future.] I understand the above disclosures.

I understand that the rates for this policy may increase in the future. (This box must be checked).

Signed _____
(Applicant) (Date)

(I explained to the applicant the importance of completing this information.

Signed: _____
(Agent) (Date)

Agent's Printed Name _____)

(Note: In order for us to process your application, please return this signed statement to (name of company) along with your application.)

My agent has advised me that this policy does not appear to be suitable for me. However, I still want the company to consider my application.

Signed _____
(Applicant) (Date)

The company may contact you to verify your answers.

Drafting Note: When the Long-Term Care Insurance Personal Worksheet is furnished to employees and their spouses under employer group policies, the text from the heading "Disclosure Statement" to the end of the page may be removed.

Even though it is the carrier responsibility to develop and use suitability standards and train its agents on its use, agents should have a solid understanding of the basic NAIC model requirements and the reasoning behind them.

Appendix Seven

Inflation

TABLES

Future Nursing Home Cost calculated using 5% Annual Increases @ \$160 Per Day

LENGTH OF CARE	TODAY'S Costs	FUTURE COSTS*	
		IN 14 YEARS	IN 28 YEARS
1 year	\$58,400	\$115,540	\$231,080
2 years	\$116,800	\$231,800	\$463,600
3 years	\$175,200	\$346,620	\$693,240
4 years	\$233,600	\$461,630	\$923,820
5 years	\$292,000	\$577,710	\$1,155,420
10 years	\$584,000	\$1,166,000	\$2,332,000

*Based on projected increases of 5% per year. All numbers rounded to the nearest thousand for clarity.

Inflation Calculator Assuming 5% Compounded Interest

Rise in Nursing Home Cost calculated using 5% Annual Increases

\$160 Per Day for Years 1 Through 30

Year	Daily Cost	Monthly Cost	Annual Cost
2011	\$ 160.00	\$ 4,866.67	\$ 58,400.00
2012	\$ 168.00	\$ 5,110.00	\$ 61,320.00
2013	\$ 176.40	\$ 5,365.50	\$ 64,386.00
2014	\$ 185.22	\$ 5,633.78	\$ 67,605.30
2015	\$ 194.48	\$ 5,915.46	\$ 70,985.57
2016	\$ 204.21	\$ 6,211.24	\$ 74,534.84
2017	\$ 214.42	\$ 6,521.80	\$ 78,261.59

2018	\$ 225.14	\$ 6,847.89	\$ 82,174.66
2019	\$ 236.39	\$ 7,190.28	\$ 86,283.40
2020	\$ 248.21	\$ 7,549.80	\$ 90,597.57
2021	\$ 260.62	\$ 7,927.29	\$ 95,127.45
2022	\$ 273.65	\$ 8,323.65	\$ 99,883.82
2023	\$ 287.34	\$ 8,739.83	\$ 104,878.01
2024	\$ 301.70	\$ 9,176.83	\$ 110,121.91
2025	\$ 316.79	\$ 9,635.67	\$ 115,628.01
2026	\$ 332.63	\$ 10,117.45	\$ 121,409.41
2027	\$ 349.26	\$ 10,623.32	\$ 127,479.88
2028	\$ 366.72	\$ 11,154.49	\$ 133,853.87
2029	\$ 385.06	\$ 11,712.21	\$ 140,546.56
2030	\$ 404.31	\$ 12,297.82	\$ 147,573.89
2031	\$ 424.53	\$ 12,912.72	\$ 154,952.59
2032	\$ 445.75	\$ 13,558.35	\$ 162,700.22
2033	\$ 468.04	\$ 14,236.27	\$ 170,835.23
2034	\$ 491.44	\$ 14,948.08	\$ 179,376.99
2035	\$ 516.02	\$ 15,695.49	\$ 188,345.84
2036	\$ 541.82	\$ 16,480.26	\$ 197,763.13
2037	\$ 568.91	\$ 17,304.27	\$ 207,651.28

Policy Benefits \$160 per day with 0% vs. 5% Compound Inflation Protection Clause

Year	Policy with 0% inflation protection	Policy with 5% inflation protection
1	\$ 58,400	\$ 58,400
5	\$ 58,400	\$ 74,534
10	\$ 58,400	\$ 95,127
15	\$ 58,400	\$ 121,409
20	\$ 58,400	\$ 154,952
25	\$ 58,400	\$ 197,763

Appendix Eight

Sample

Federal and State ADL Differences

DEFINITIONS OF THE DIFFERENCES OF THE ACTIVITIES OF DAILY LIVING (ADLs) BETWEEN FEDERAL AND STATE

The principle differences between the federal and state regulations concern the activities of daily living or ADLs. The chart below points out the similarities and differences:

Federal ADLs	State ADLs
<u>Bathing</u> means washing oneself by sponge bath; or in shower or tub including getting in and out of tub/shower.	<u>Bathing</u> which shall mean cleaning the body using a tub, shower, or sponge bath, including getting a basin or water, managing faucets, getting in and out of tub or shower, and reaching head and body parts for soaping, rinsing, and drying.
<u>Dressing</u> means putting on and taking off clothing and any necessary braces, fasteners or artificial limbs.	<u>Dressing</u> which shall mean putting on and taking off fastening, and unfastening garments and undergarments and special devices such as back or leg braces, corsets, elastic stocking or garments, and artificial limbs or splints.
<u>Eating</u> means feeding; oneself from a plate, cup or table or by feeding tube or intravenously.	<u>Eating</u> , which shall mean reaching for, picking up, and grasping a utensil and cup; getting food on a utensil, and bringing food, utensil, and cup to mouth; manipulating food on plate; and cleaning face and hands as necessary following meal.

<u>Continence</u> means control of bowl/bladder function; or when unable to maintain control of bowl or bladder functions perform associated personal hygiene (including caring for a catheter or colostomy).	<u>Continence</u> , which shall mean the ability to control bowel and bladder as well as use colostomy or catheter receptacles, and apply diapers and disposable barrier pads.
<u>Toileting</u> means getting/off the toilet and performing necessary personal hygiene.	<u>Toileting</u> , which shall mean getting on and off a toilet or commode and emptying a commode, managing clothing and wiping and cleaning the body after toileting, and using and emptying a bedpan and urinal.
<u>Transferring</u> means moving into or out of bed, a chair or wheelchair.	<u>Transferring</u> , which shall mean moving from one sitting or lying position to another sitting or lying position; for example, from bed to or from a wheelchair or sofa, coming to a standing position, or repositioning to promote circulation and prevent skin breakdown.
	<u>Ambulating</u> , which shall mean walking or moving around inside or outside the home.
Source: National Association of Insurance Commissioners Model Act Report 641, Sec.4, 1997 pp. 2.	Source: Sample State.

Appendix Nine

Health Insurance Portability & Accountability Act

Health Insurance Portability & Accountability Act

Tax Treatment of Long-Term Care Expenses & Long-Term Care Insurance

Health Insurance Portability & Accountability Act of 1996

(Public Law 104-191, 110 Statutes 1936, 2054 & 2063)

Federal & state tax codes have a purpose beyond raising revenue. Public policy is often served by providing economic relief to taxpayers or motivation for particular behavior. The 1996 Health Insurance Portability & Accountability Act (HIPAA – Public Law 104-191, 110 Stat. 1936, 2054 & 2063) is one of the most far-reaching laws passed by Congress in the latter part of the 20th century. The effects of HIPAA are so complex that federal and state governments continue to grapple with its legislative intent.

HIPAA's impact on the treatment of long-term care expenses and long-term care insurance is the focus of this section. Congress attempted to fulfill a number of different public policy objectives in taking on long-term care as a topic: (1) classifying long-term care costs as a medical expense thus providing taxpayers with some economic relief; (2) categorizing long-term care insurance as accident & health insurance thereby providing clarity as to the tax treatment of premiums and benefits; and (3) providing the general public an incentive to purchase long-term care insurance.

The general categories in this section include:

- Tax treatment of long-term care expenses
- Definition of a “chronically ill” individual

- General tax treatment of TQ & NTQ long-term care insurance
- Tax-qualified long-term care insurance deductibility
- Current efforts to expand tax incentives for long-term care expenses and long-term care insurance

Tax Treatment of Long-Term Care Expenses

The Internal Revenue Code allows deductions for medical and dental expenses under certain circumstances (IRC Sec. 213d). Prior to the passage of HIPAA, a broad range of long-term care expenses were generally not deductible. Part of Congress' intent in enacting HIPAA was to provide tax relief to individuals and families that were incurring long-term care costs. However, part of the challenge facing legislators was determining which expenses would qualify.

The broad & expanding nature of long-term care expenses made it difficult to stipulate a "laundry list" of qualified services. The IRS defines "qualified long-term care services" as:

Necessary diagnostic, preventative, therapeutic, curing, treating, mitigating, rehabilitative services and maintenance and personal care services required by a chronically ill individual pursuant to a plan of care prescribed by a licensed health care practitioner.

This overly broad universe of services could potentially be used by anyone at any time for services normally covered under healthcare insurance. To control when the cost of long-term care services could receive favorable tax treatment, Congress established a trigger basis for initiating benefits by tying services to a state of disability defined as a *chronically ill individual*.

- A chronically ill individual must be certified by a licensed health care practitioner within the previous 12 months as one of the following:

1. The insured is unable, for at least 90 days, to perform at least two activities of daily living (ADLs) without substantial assistance from another individual, due to loss of functional capacity. Activities of daily living are eating, toileting, transferring, bathing, dressing and continence. (See IRS Notice 97-31, issued May 6, 1997 or CIC 10232.8(e1 – 6) for the definitions of the ADLs.)

The insured requires substantial supervision to be protected from threats to health and safety due to severe cognitive impairment.

This standardized definition of a chronically ill person cannot be altered in any way by state law, and it is the only definition allowed to receive the favorable tax treatment for the cost of long-term care services.

Licensed Health Care Practitioner

The Internal Revenue Service defines the licensed health care practitioner (LHP) in very general terms. They can include doctors, nurses, social workers, chiropractors, Christian Science practitioners, mental health professionals, and other licensed therapists. IRS Publication 502 includes an extensive list of licensed health care practitioners. Some States narrow the list by specifying the role of the LHP in the certification, assessment, and plan of care of the insured for the purposes of the claims process. The LHP must be independent of the insurance company and “should not be compensated in any manner that is linked to the outcome of the certification. Federal and State law requires the certification of the insured’s assessment be renewed annually.

90-Day Certification for Activities of Daily Living

This component of the long-term care qualification may be the most misunderstood. A review of its impact as it applies to long-term care insurance is addressed later in this section. Its relevance to the deductibility of long-term care expenses is clear. Congress

intended to limit long-term care costs to those associated with chronic illness. A clinical definition of chronic illness is one that is expected to last 90 days or more. Some expenses for acute or short-term illnesses were already deductible as a medical expense. If policy makers had ignored the distinction between acute and chronic, it would have had the unintended consequence of allowing taxpayers to deduct all their expenses associated with short-term disabilities, due to the vague nature of the definition of a qualified *long-term care service*.

Therefore, a taxpayer who wishes to deduct qualified long-term care expenses using the ADL definition, must have a licensed health care practitioner certify that the insured is likely to need substantial assistance for at least 90 days. Keep in mind, the requirement concerns the likelihood of needing care, not the actual receipt of care. In fact, there is no requirement that the person actually receives the full 90 days of care. The insured must be recertified at least annually.

Note: IRS Publication 502 stipulates that the 90 day certification period is not a deductible period for people who have long-term care insurance. Long-term care insurance can still pay benefits following the deductible period of the policy, if any, as long as the certification stipulates that the person is likely to need qualified long-term care services for at least 90 days. The certification may also be done retroactively in the event a claim is not filed until after the deductible period in the policy has been met.

Substantial Assistance

For the purposes of the activities of daily living, IRS Notice 97-31 (1997) allows substantial assistance to be defined to mean both *hands-on assistance* and *standby assistance*.

- **Hands-On Assistance:** means the physical assistance of another person without which the individual would be unable to perform the ADL.

- **Stand-By Assistance:** means the presence of another person within arm's reach of the individual that is necessary to prevent, by physical intervention, injury to the individual while the individual is performing the ADL.

Severe Cognitive Impairment & Substantial Supervision

Notice 97-31 defines a *severe cognitive impairment* "as a loss or deterioration in intellectual capacity that is similar to Alzheimer's disease and like forms of irreversible dementia and is measured by clinical evidence and standardized tests that reliably measure impairment in short-term and long-term memory, orientation to people, places or time and deductive or abstract reasoning." The 90-day certification by a LHP is not a requirement for qualification under the cognitive impairment trigger. Similar to the ADL qualification however, the insured must be recertified every 12 months to ensure that they still qualify for benefits.

Note: Taxpayers and tax preparers must document an ADL or cognitive impairment consistent with HIPAA rules in order to deduct long-term care expenses as a medical expense. Many tax preparers miss this point and it could be a critical matter during a tax audit.

Tax-Qualified Long-Term Care Insurance

The Federal and state governments have recognized the impact of long-term care expenses on their state Medicaid budgets. Over the last 40 years the Medicaid program has become the primary source for long-term care expenses in the United States for the middle class population. Congress is attempting to shift the Medicaid burden to the private sector by providing general tax incentives to purchase long-term care insurance in anticipation of the huge number of baby boomers who may need care in the future.

Prior to HIPAA, neither long-term care insurance premiums or benefits were addressed in the Federal tax code. There was uncertainty as to whether LTC insurance would be

classified as (accident & health or disability) for the purposes of both the deductibility of premiums and the taxation of the benefits. However, the common belief was that as long as premiums were paid with after-tax dollars, benefits would be tax-free. This was pre-HIPAA a rule of thumb for all insurance.

HIPAA stipulates that generally, long-term care insurance policies that use the definition of a chronically ill individual will be “qualified long-term care insurance” and that long-term care expenses incurred by a taxpayer who qualifies as a chronically ill individual will be deductible as a medical expense (HIPAA also requires certain consumer protection provisions that will be discussed later).

Tax-qualified long-term care insurance is treated the same as an accident & health insurance policy.

- Benefits pass tax-free

1. Per diem and cash method policy benefits received are subject to an annually adjusted amount -- \$220/day in 2003 (indexed upwards annually by approximately 5%.

- Premiums are generally deductible

1. Limits apply to individuals, sole proprietors, owners of S-corporations, & LLP's
2. Premiums paid by an employer for an employee are 100% deductible
3. Not counted as income to an employee
4. Cannot currently include qualified long-term care insurance in a Section 125 Cafeteria plan or flexible spending arrangement

Various deductibility scenarios will be explored later in this section.

Congress created a generalized structure to which qualified products must adhere. For purposes of HIPAA, a qualified long-term care insurance product must pay benefits

using no less than 5 or no more than 6 of the following activities of daily living:

- Eating
- Toileting
- Transferring
- Bathing
- Dressing
- Continence

Note: Qualified long-term care insurance policies may not use “medical necessity” as a benefit trigger and must coordinate benefit payment with Medicare.

This 5–6 ADL structure created concern in some states because policies provided benefits for home care services, were required to use a benefit trigger of 7 ADLs; the six listed above plus ambulating. Generally, all qualified long-term care insurance policies issued nationwide utilize a 6 ADL structure requiring a loss of 2 or 3 ADLs to qualify for benefits (subject to certification by a LHP that the impairment is likely to last for at least 90 days).

Qualified long-term care insurance policies are required to meet specific consumer protection guidelines of the 1993 National Association of Insurance Commissioners Model Act and Regulations for Long-term Care Insurance. Many of the consumer protections in the NAIC Models had already been adopted in other states, including:

- Guaranteed renewability or non-cancellability
- Prohibitions on exclusions and limitations
- Provisions relating to extension of benefits & conversions
- Replacement
- Unintentional lapse
- Post-claim underwriting

- Requirement to offer inflation protection & rejection by consumer
- Restrictions on pre-existing conditions and probationary periods
- Disclosure
- Non-forfeiture provisions

HIPAA requires that long-term care insurance policies comply with its guidelines to be considered “qualified” long-term care insurance. Policies that do not meet these requirements are considered to be non-qualified long-term care insurance policies. Premiums paid for a non-qualified policy are not presumed to be deductible as accident and health insurance. However, HIPAA was silent as to the tax treatment of benefits received from non-qualified policies issued after January 1, 1997 causing confusion. To date, the Department of the Treasury has not issued an opinion on this conflict and Congress has not taken the matter up again leading to continued speculation about the tax implications of these benefits.

HIPAA also establishes a reporting mechanism for benefits received under *all* long-term care insurance policies. Similar to disability insurance, if a policyholder receives benefits from a long-term care insurance policy, they will receive an IRS 1099 LTC Form issued by the carrier. Benefits reported on the 1099 must also be reported on IRS Form 8853. The 1099 form must identify the method of benefit payment (reimbursement or per diem) but does not need to determine the tax-qualified status of the actual long-term care insurance policy from which the benefits were paid. Form 8853, which contains the medical savings and the IRS 1099 information, adds additional mystery to the taxation of non-qualified benefits conundrum because it provides a vehicle for these benefits to be taxed. Despite continuing confusion neither the Department of the Treasury nor Congress seems anxious to clarify this matter. Long-term care insurance benefits that are part of a life insurance or annuity contract may not receive the same tax favored status as benefits received from a tax-qualified long-term care insurance policy. If the benefits constitute an advance payment of death benefit, then it is likely that they will not be taxed as income. If, however, the benefits received are part of the accumulation value of the contract, taxes may be payable. In no case are the premiums paid for life

insurance or annuity contracts, which include long-term care insurance benefits, deductible as tax-qualified long-term care insurance premiums.

Grandfathered Long-Term Care Insurance Policies

Congress realized that there were many long-term care insurance policies issued prior to January 1, 1997, that would not comply with HIPAA. Either their benefit structures or payment mechanisms were inferior to its guidelines or, in the case of some states, the benefit triggers were considered too generous. Legislators left it to the Department of the Treasury to establish guidelines for “grandfathered” policies. In its interim directive on tax-qualified long-term care insurance (Notice 97-31, May 1997), the Department of the Treasury indicated that long-term care insurance policies issued prior to January 1, 1997, meeting “long-term care insurance requirements of the State in which the contract was ... issued” would be grandfathered for the purposes of tax qualification unless the policyholder made a “material change” to the policy. However, they did not define material change.

Final regulations issued in December 1998 identified criteria for which a material modification that would result in a policy losing its tax-qualified status. Action that could be taken by the policyholder that is not material and *would not* jeopardize the policy’s grandfathered status includes the following:

- A change in the mode of premium payment
- A class wide increase or decrease in premiums for contracts that have been issued on a guaranteed renewable basis
- A reduction in premiums due to the purchase of a long-term care insurance policy by a member of the policyholder’s family
- A reduction in coverage (with correspondingly lower premium) made at the request of a policyholder
- A reduction in premiums that occurs because the policyholder becomes entitled to a discount under the issuer’s pre-1997 premium rate structure

(such as a group or association discount or change from smoker to non-smoker status)

- The addition without an increase in premiums of alternative forms of benefits that may be selected by the policyholder
- The addition of a rider to increase benefits under a pre-1997 contract if the rider would constitute a qualified long-term care insurance contract if it were a separate contract
- The deletion of a rider or provision of a contract (called an HHS – Health & Human Services – rider) that prohibited coordination of benefits with Medicare
- The effectuation of a continuation or conversion of coverage right under a group contract following an individual's ineligibility for continued coverage under the group contract
- The substitution of one insurer for another in an assumption reinsurance transaction
- Expansion of coverage under a group contract caused by corporate merger or acquisition
- Extension of coverage to collectively bargained employees
- The addition of former employees

The Final Regulations suggest that the following practices will be treated as issuance of a new contract:

- A change in terms of a contract that alters the amount or timing of an item payable by either the policyholder, the insured or insurance company
- A substitution of the insured under an individual contract
- A change (other than an immaterial change) in the contractual terms or in the plan under which the contract was issued relating to eligibility for membership in the group covered under a group contract

Note: The important message that should be grasped from this review is that anytime a

consumer considers replacing a policy issued prior to January 1, 1997, great caution must be exercised. A pre-HIPAA policy may contain provisions that might make it easier to qualify for benefits: for example, 2 out of 7 activities of daily living instead of the 2 out of 6 required by HIPAA; a medical necessity benefit trigger that is prohibited in HIPAA; no HIPAA 90 day certification requirement; the benefits of a pre-HIPAA policy do not require coordination with Medicare, which increases the amount available to pay for care.

Tax-Qualified Long-Term Care Insurance Premium Deductibility

The Health Insurance Portability & Accountability Act of 1996 and subsequent Department of the Treasury rulings have created a number of different premium deduction scenarios that benefit consumers. The tax incentives that allow for premium deductibility help the self-employed and employees of companies that provide employer-paid long-term care insurance. To a lesser extent, some individual taxpayers, who are not self-employed, will benefit from the premium deductibility allowed by HIPAA.

There are four primary deductibility scenarios for tax-qualified long-term care insurance. They are:

- Medical Savings Accounts
- Individual deductibility
- Deductibility for the self-employed, owners of S-corporations, limited liability partnerships (LLP) & limited liability corporations (LLC)
- Deductibility for employee/owners of C-corporations

The intent of this next section is to provide students with broad-brush guidance pertaining to the tax deductibility of TQ long-term care insurance premiums. Most agents are not Certified Public Accountants (CPA's) or tax preparers. They should always refer clients to insured's tax advisor for the final analysis as to whether premium

deductibility makes sense for them.

Medical Savings Accounts

Medical Savings Accounts (MSA) were established under HIPAA. Their primary appeal is to consumers under age 65, who are willing to take on the responsibility of a relatively large medical insurance deductible in favor of lower premiums. Simply stated, the consumer purchases a medical insurance plan with a high deductible that generally exceeds \$1,790. They are then allowed to take an above-the-line deduction on their taxes equal to a percentage of the deductible (65% for an individual, 75% for a couple or family). The amount of money deducted must be placed in an MSA account. The money placed in the MSA grows tax deferred, similar to an IRA or other qualified retirement plan. The funds accumulated can be used to pay for un-reimbursed medical expense (allowed by IRC Sec. 213(d)) deductibles and co-insurance. *The money in the MSA can also be used to pay the premiums on a tax-qualified long-term care insurance policy.*

From a practical standpoint, this is the only way an individual, self-employed or otherwise, can garner the equivalent of an above-the-line deduction for a qualified long-term care insurance policy.

MSAs have achieved inconsistent acceptance since their introduction in 1997. Their applicability depends on the regional make-up of the medical care delivery system, the availability of medical insurance plans in an area, and the pricing disparity between conventional “low-deductible” plans and the “high-deductible” plans that qualify for the MSA program. MSAs represent an opportunity for some consumers to tailor their medical insurance and long-term care insurance priorities in a cost and tax-efficient manner.

Individual Deductibility

This deductibility scenario for tax-qualified long-term care insurance is one of the most misunderstood applications. It is true that only taxpayers who itemize their deductions can benefit from the deductibility of qualified long-term care insurance premiums. It is also a fact that, based on the taxpayer's age, only a portion of the long-term care insurance premium may be deducted. With this in mind, taxpayers and their advisors may be wise to step back and take a broader view of the opportunities.

Taxpayers over age 60 with above average income and assets are typically interested in long-term care insurance. Often these individuals do itemize their deductions because they own property and the standard deduction is not in their best interest. In this situation, expenses for medical care and insurance premiums are deductible to the extent that they exceed 7.5% of adjusted gross income. Prior to HIPAA, most taxpayers in this circumstance would not exceed 7.5% of their adjusted gross income in un-reimbursed medical expenses. However, with the inclusion of qualified long-term care insurance as an accident and health insurance policy, some taxpayers may benefit.

HIPAA states that premiums for tax-qualified long-term care insurance are deductible as an accident and health insurance policy. *However, unlike other accident and health insurance premiums, the amount of qualified long-term care insurance premiums is limited by a stipulated age to the amount that can be deducted.* In 2003, the age that "banded" amounts that may be applied towards the taxpayer's un-reimbursed medical expenses are:

- Under Age 40 \$250
- Ages 41 - 50 \$470
- Ages 51 - 60 \$940
- Ages 61 - 70 \$2,510
- Ages 71 + \$3,130

Note: These amounts allowable towards deductions are indexed upward annually by a factor of approximately 5%.

Individual taxpayers under age 61 who itemize their deductions, may not get much of a tax relief by including the allowable long-term care insurance premium amount in their un-reimbursed medical expenses. However, someone age 61+ may do better. To reiterate, individual taxpayers who itemize their deductions, may include the cost of tax-qualified long-term care insurance as an accident and health insurance premium. The amount allowed is limited by the above-referenced, age-related amounts. The following is a *thumbnail* example of how this may work for a hypothetical husband and wife, both age 65, who are considering purchasing a qualified long-term care insurance policy with a joint annual premium of \$7,000. Assume, for the purposes of this example, that this couple has an adjusted gross income of \$100,000 therefore they must exceed \$7,500 of un-reimbursed medical expenses before they receive any type of tax relief from these types of deductions.

- Amount Allowed For TQ-LTCi: \$5,020
- Medicare Supplement Premiums 3,600
- Medicare Part B Premiums 1,400
- Other Allowable Medical Expenses 2,000 (Rx, eyeglasses, dental)

Total \$12,020

In this example, the taxpayers would be allowed to deduct \$4,520 (\$12,020 minus their \$7,500 threshold) of un-reimbursed medical expenses. If they are in a combined federal *and* state income tax bracket of 35%, their tax savings would equal \$1,582 (\$4,520 x 35%). This would amount to a 22% premiums savings (\$1,520 ÷ \$7,000). *Clearly, anyone can create an example that works!* But consider the facts in this case. The deductible amount allowed for long-term care insurance premiums in and of itself is not enough to trigger a deduction for these taxpayers, nor are the stand-alone deductions for the other un-reimbursed medical expenses. *However, the combination of all of them does provide this hypothetical couple with a meaningful savings.* Most agents are not

qualified tax advisors and as such need to be cautious and circumspect in their recommendations. However, an agent may spot an opportunity for a taxpayer that might go unnoticed by the client's tax preparer. Clearly, if the agent inquires as to the unreimbursed expenses illustrated above they can spot a potential tax savings for the consumer and refer them to their tax advisor.

Deductibility for the Self-Employed

For the purposes of this discussion, self-employed individuals include sole proprietors, partners and owners of S-corporations, limited liability partnerships ("LLP") and limited liability corporations ("LLC"). An owner is defined as any individual who owns 2% or more of the business entity. While these types of business entities can have a separate tax identification number for the reporting of income, the tax return that is filed is informational in nature only. The profit or loss from the business entity is passed through to the owners pursuant to their share of ownership. Typically, in sole proprietorships and partnerships, spouses are not considered owners. If they are on the payroll, they would be considered employees. Spouses of owners of S-corporations, LLP's and LLC's are considered owners regardless of their direct or indirect participation in the business' activities. *With respect to accident and health insurance coverage purchased by one of these entities for a non-owner/employee, premiums are fully deductible, there is no imputed income to employee of premiums and the benefits pass tax-free at time of claim.*

The good news for owners of these entities is that beginning in 2003 premiums for accident and health insurance are 100% deductible. It is not necessary for these taxpayers to exceed 7.5% of adjusted gross income to benefit from the tax code for these expenses. Tax-qualified long-term care insurance, being accident and health insurance, falls into this general rule. The bad news is that the amount allowable for deduction is limited by the previously discussed age-related schedule.

While this is not optimal, it can lead to savings. Consider a self-employed husband and

wife, both age 55 who are considering purchasing a tax-qualified long-term care insurance policy with a joint annual premium of \$3,600 per year. They would be allowed to deduct \$1,880 ($\940×2). If they are in the combined Federal & State tax bracket of 35% their tax savings would be \$658 or approximately 18% of premium. Additionally, they may save on their self-employment taxes because the premium amount paid by the business entity would be received not as income, but as an employee benefit. This may save this self-employed couple an additional 16% of the premium paid. Individually or combined, these tax savings provides incentives to owners of these entities to purchase qualified long-term care insurance through their businesses. *Agents should be very cautious and understand their limitations of advising consumers about their insured's specific tax situation and circumstances.*

Deductibility in Closely-Held C-Corporations

The fine difference between owners of business entities discussed in the previous section and employee owners of closely-held C-corporations is that for the purposes of paying taxes they are considered employees, not owners. Therefore, premiums paid by the C-corporation for tax-qualified long-term care insurance (a.k.a. accident and health insurance) for stockholder employees is deductible to the corporation: there is no imputed income to the employee stockholder for premiums paid; and the benefits will pass tax-free at time of claim. Some believe that this tax treatment of accident and health insurance premiums and benefits means that every employee in the company must receive “like” benefits. Others go to the other extreme and tell consumers that they can discriminate as to who receives such benefits. Both are incorrect.

The Internal Revenue Code Sec. 105 clearly indicates that accident and health insurance specifically provided to stockholder employees on a selective basis, without creating a distinguishable *class of employees* who are eligible for the benefit, is not allowed. The class must be based on employment status. It cannot be based on stock ownership. A class of employees such as “officer employees” can be created for the corporation who are eligible for a specific accident and health insurance benefit.

However, they must be employees, not just officers or stockholders.

Court decisions on this matter go back to 1968. If the closely-held corporation cannot validate a clear class of employees who are eligible for the benefit then the premiums could be treated as dividends to the stockholder-employee and the premiums are not deductible to the corporation. It is therefore incumbent upon agents and their tax advisors to be judicious in establishing classes eligible for coverage. It is also important for the corporation to establish the plan in their minutes and to clearly identify the classes of employees that are eligible for benefits.

Again, once a bona fide class of employees is established, tax-qualified long-term care insurance premiums are deductible to the corporation; there is no income imputed to the employee and the benefits pass tax free at time of claim. This tax scenario is the best of all worlds for employees of any corporation and owner-employees of closely-held C corporations.

Final Items On Tax Deductibility

Currently long-term care insurance *may not* be included in Section 125 Cafeteria Plans or Flexible Spending arrangements. However, for the past several sessions of Congress, legislation has been introduced to allow for this. Additionally, this legislation has attempted to expand individual deductibility and create tax credits for taxpayers who incur long-term care expenses. Over the years there has also been legislation in states designed to expand premium deductibility for State income tax purposes and to provide credits for long-term care expenses. While this paper will not speculate on the outcome of these efforts to provide additional incentives to purchase qualified long-term care insurance, legislators appear to see private insurance as an important tool of public policy.

Appendix Ten

RESCISSION REPORTING FORM

**RESCISSION REPORTING FORM
FOR LONG-TERM CARE POLICIES
FOR THE STATE OF _____
FOR THE REPORTING YEAR 19[]**

Company Name: _____
Address: _____
Phone Number: _____

Due: March 1 annually

Instructions:

The purpose of this form is to report all rescissions of long-term care insurance policies or certificates. Those rescissions voluntarily effectuated by an insured are not required to be included in this report. Please furnish one form per rescission.

Policy Form #	Policy and Certificate #	Name of Insured	Date of Policy Issuance	Date/s Claim/s Submitted	Date of Rescission

Detailed reason for rescission:

Signature

Name and Title (please type)

Date

Appendix Eleven

Claims Denial Reporting Form

Long-Term Care Insurance

For the State of _____

For the Reporting Year of _____

Due: June 30 annually

Company Name: _____

Company Address: _____

Company NAIC Number: _____

Contact Person: _____ Phone Number _____

Line of Business: Individual Group

Instructions

The purpose of this form is to report all long-term care claim denials under in force long-term care insurance policies. "Denied" means a claim that is not paid for any reason other than for claims not paid for failure to meet the waiting period or because of an applicable pre-existing condition.

		State Data	Nationwide Data
1	Total Number of Long-Term Care Claims Reported		
2	Total Number of Long-Term Care Claims Denied/Not Paid		
3	Number of Claims Not Paid due to Pre-existing Condition Exclusion		
4	Number of Claims Not Paid due to Waiting (Elimination) Period Not Met		
5	Net Number of Long-Term Care Claims Denied for Reporting Purposes (Line 2 Minus Line 3 Minus Line 4)		

6	Percentage of Long-Term Care Claims Denied of Those Reported (Line 5 Divided By Line 1)		
7	Number of Long-Term Care Claim Denied due to:		
8	▪ Long-Term Care Services Not Covered under the Policy ²		
9	▪ Provider/Facility Not Qualified under the Policy ³		
10	▪ Benefit Eligibility Criteria Not Met ⁴		
11	▪ Other _____		

1. The nationwide data may be viewed as a more representative and credible indicator where the data for claims reported and denied for your state are small in number.
2. Example—home health care claim filed under a nursing home only policy.
3. Example—a facility that does not meet the minimum level of care requirements or the licensing requirements as outlined in the policy.

Examples—a benefit trigger not met, certification by a licensed health care practitioner not provided, no plan of care.

Appendix Twelve

Potential Rate Increase Disclosure Form

Long-Term Care Insurance

Instructions:

This form provides information to the applicant regarding premium rate schedules, rate schedule adjustments, potential rate revisions, and policyholder options in the event of a rate increase.

Insurers shall provide all of the following information to the applicant:

Long-Term Care Insurance

Potential Rate Increase Disclosure Form

1. **[Premium Rate] [Premium Rate Schedules]:** [Premium rate] [Premium rate schedules] that [is] [are] applicable to you and that will be in effect until a request is made and [filed] [approved] for an increase [is] [are] [on the application] [\$ _____])

Drafting Note: Use "approved" in states requiring prior approval of rates.

2. **The [premium] [premium rate schedule] for this policy [will be shown on the schedule page of] [will be attached to] your policy.**
3. **Rate Schedule Adjustments:**

The company will provide a description of when premium rate or rate schedule adjustments will be effective (e.g., next anniversary date, next billing date, etc.) (fill in the blank)•

4. **Potential Rate Revisions:**

This policy is Guaranteed Renewable. This means that the rates for this product may be increased in the future. Your rates can NOT be increased due to your increasing age or declining health, but your rates may go up based on the experience of all policyholders with a policy similar to yours.

If you receive a premium rate or premium rate schedule increase in the future, you will be notified of the new premium amount and you will be able to exercise at least one of the following options:

- Pay the increased premium and continue your policy in force as is.
- Reduce your policy benefits to a level such that your premiums will not increase. (Subject to state law minimum standards.)
- Exercise your nonforfeiture option if purchased. (This option is available for purchase for an additional premium.)
- Exercise your contingent nonforfeiture rights.* (This option may be available if you do not purchase a separate nonforfeiture option.)

Contingent Nonforfeiture

Cumulative Premium Increase over Initial Premium That qualifies for Contingent Nonforfeiture

(Percentage increase is cumulative from date of original issue. It does NOT represent a one-time increase.)

Issue Age	Percent Increase Over Initial Premium
29 and under	200%
30-34	190%
35-39	170%
40-44	150%
45-49	130%
50-54	110%
55-59	90%
60	70%
61	66%
62	62%
63	58%
64	54%
65	50%
66	48%
67	46%
68	44%
69	42%
70	40%
71	38%
72	36%
73	34%
74	32%
75	30%
76	28%
77	26%
78	24%
79	22%
80	20%
81	19%
82	18%
83	17%
84	16%

85	15%
86	14%
87	13%
88	12%
89	11%
90 and over	10%

[The following contingent nonforfeiture disclosure need only be included for those limited pay policies to which Sections 28D(4) and 28D(6) of the regulation are applicable].

In addition to the contingent nonforfeiture benefits described above, the following reduced "paid-up" contingent nonforfeiture benefit is an option in all policies that have a fixed or limited premium payment period, even if you selected a nonforfeiture benefit when you bought your policy. If both the reduced "paid-up" benefit AND the contingent benefit described above are triggered by the same rate increase, you can chose either of the two benefits.

You are eligible for the reduced "paid-up" contingent nonforfeiture benefit when all three conditions shown below are met:

1. The premium you are required to pay after the increase exceeds your original premium by the same percentage or more shown in the chart below:

<u>Triggers for a Substantial Premium Increase</u>	
<u>Issue Age</u>	<u>Percent Increase Over Initial Premium</u>
Under 65	50%
65-80	30%
Over 80	10%

2. You stop paying your premiums within 120 days of when the premium increase took effect;
AND
3. The ratio of the number of months you already paid premiums is 40% or more than the number of months you originally agreed to pay.

If you exercise this option your coverage will be converted to reduced "paid-up" status. That means there will be no additional premiums required. Your benefits will change in the following ways:

- a. The total lifetime amount of benefits your reduced paid up policy will provide can be determined by multiplying 90% of the lifetime benefit amount at the time the policy becomes paid up by the ratio of the number of months you already paid premiums to the number of months you agreed to pay them.
- b. The daily benefit amounts you purchased will also be adjusted by the same ratio.

If you purchased lifetime benefits, only the daily benefit amounts you purchased will be adjusted by the applicable ratio.

Example:

- You bought the policy at age 65 with an annual premium payable for 10 years.
- In the sixth year, you receive a rate increase of 35% and you decide to stop paying premiums.
- Because you have already paid 50% of your total premium payments and that is more than the 40% ratio, your "paid-up" policy benefits are .45 (.90 times .50) times the total benefit amount that was in effect when you stopped paying your premiums. If you purchased inflation protection, it will not continue to apply to the benefits in the reduced "paid-up" policy.

Appendix Thirteen

Replacement and Lapse Reporting Form

Long-Term Care Insurance

Replacement and Lapse Reporting Form

For the State of _____

For the Reporting Year of _____

Company Name: _____

Due: June 30 annually

Company _____

Company NAIC Number: _____

Address: Contact _____

Phone Number: () _____

Person: _____

Instructions

The purpose of this form is to report on a statewide basis information regarding long-term care insurance policy replacements and lapses. Specifically, every insurer shall maintain records for each agent on that agent's amount of long-term care insurance replacement sales as a percent of the agent's total annual sales and the amount of lapses of long-term care insurance policies sold by the agent as a percent of the agent's total annual sales. The tables below should be used to report the ten percent (10%) of the insurer's agents with the greatest percentages of replacements and lapses.

Listing of the 10% of Agents with the Greatest Percentage of Replacements

Agent's Name	Number of Policies Sold By This Agent	Number of Policies Replaced By This Agent	Number of Replacements As % of Number Sold By This Agent

Listing of the 10% of Agents with the Greatest Percentage of Lapses

Agent's Name	Number of Policies Sold By This Agent	Number of Policies Lapsed By This Agent	Number of Lapses As % of Number Sold By This Agent

Company Totals

Percentage of Replacement Policies Sold to Total Annual Sales %

Percentage of Replacement Policies Sold to Policies In Force (as of the end of the preceding calendar year)

Percentage of Lapsed Policies to Total Annual Sales %

Percentage of Lapsed Policies to Policies In Force (as of the end of the preceding calendar year) %

Appendix Fourteen

Premium Rate Schedule Increases

Section 20. Premium Rate Schedule Increases

A. This section shall apply as follows:

- (1) Except as provided in Paragraph (2), this section applies to any long-term care policy or certificate issued in this state on or after [insert date that is 6 months after adoption of the amended regulation].
- (2) For certificates issued on or after the effective date of this amended regulation under a group long-term care insurance policy as defined in Section [insert reference to Section 4E(1) of the NAIC Long-Term Care Insurance Model Act], which policy was in force at the time this amended regulation became effective, the provisions of this section shall apply on the policy anniversary following [insert date that is 12 months after adoption of the amended regulation].

B. An insurer shall provide notice of a pending premium rate schedule increase, including an exceptional increase, to the commissioner at least [30] days prior to the notice to the policyholders and shall include:

Drafting Note: *In states where the commissioner is required to approve premium rate schedule increases, "shall provide notice" may be changed to "shall request approval." States should consider whether a time period other than 30 days is desirable. An alternate time period would be the time period required for policy form approval in the applicable state regulation or law.*

- (1) Information required by Section 9;
- (2) Certification by a qualified actuary that:
 - (a) If the requested premium rate schedule increase is implemented and the underlying assumptions, which reflect moderately adverse conditions, are realized, no further premium rate schedule increases are anticipated;
 - (b) The premium rate filing is in compliance with the provisions of this section;
- (3) An actuarial memorandum justifying the rate schedule change request that includes:
 - (a) Lifetime projections of earned premiums and incurred claims based on the filed premium rate schedule increase; and the method and assumptions

used in determining the projected values, including reflection of any assumptions that deviate from those used for pricing other forms currently available for sale;

- i. Annual values for the five (5) years preceding and the three (3) years following the valuation date shall be provided separately;
 - ii. The projections shall include the development of the lifetime loss ratio, unless the rate increase is an exceptional increase;
 - iii. The projections shall demonstrate compliance with Subsection C; and
 - iv. For exceptional increases,
 - (I) The projected experience should be limited to the increases in claims expenses attributable to the approved reasons for the exceptional increase; and
 - (II) In the event the commissioner determines as provided in Section 4A(4) that offsets may exist, the insurer shall use appropriate net projected experience;
- (b) Disclosure of how reserves have been incorporated in this rate increase whenever the rate increase will trigger contingent benefit upon lapse;
 - (c) Disclosure of the analysis performed to determine why a rate adjustment is necessary, which pricing assumptions were not realized and why, and what other actions taken by the company have been relied on by the actuary;
 - (d) A statement that policy design, underwriting and claims adjudication practices have been taken into consideration; and
 - (e) In the event that it is necessary to maintain consistent premium rates for new certificates and certificates receiving a rate increase, the insurer will need to file composite rates reflecting projections of new certificates;
- (4) A statement that renewal premium rate schedules are not greater than new business premium rate schedules except for differences attributable to benefits, unless sufficient justification is provided to the commissioner; and
 - (5) Sufficient information for review [and approval] of the premium rate schedule increase by the commissioner.

C. All premium rate schedule increases shall be determined in accordance with the following requirements:

- (1) Exceptional increases shall provide that seventy percent (70%) of the present value of projected additional premiums from the exceptional increase will be returned to policyholders in benefits;
- (2) Premium rate schedule increases shall be calculated such that the sum of the accumulated value of incurred claims, without the inclusion of active life reserves, and the present value of future projected incurred claims, without the inclusion of active life reserves, will not be less than the sum of the following:
 - (a) The accumulated value of the initial earned premium times fifty-eight percent (58%);
 - (b) Eighty-five percent (85%) of the accumulated value of prior premium rate schedule increases on an earned basis;
 - (c) The present value of future projected initial earned premiums times fifty-eight percent (58%); and
 - (d) Eighty-five percent (85%) of the present value of future projected premiums not in Subparagraph © on an earned basis;
- (3) In the event that a policy form has both exceptional and other increases, the values in Paragraph (2)(b) and (d) will also include seventy percent (70%) for exceptional rate increase amounts; and
- (4) All present and accumulated values used to determine rate increases shall use the maximum valuation interest rate for contract reserves as specified in the [insert reference to state equivalent to the Health Reserves Model Regulation Appendix A, Section IIA]. The actuary shall disclose as part of the actuarial memorandum the use of any appropriate averages.

D. For each rate increase that is implemented, the insurer shall file for review [approval] by the commissioner updated projections, as defined in Subsection B(3)(a), annually for the next three (3) years and include a comparison of actual results to projected values. The commissioner may extend the period to greater than three (3) years if actual results are not consistent with projected values from prior projections. For group insurance policies that meet the conditions in Subsection K, the projections required by this subsection shall be provided to the policyholder in lieu of filing with the commissioner.

- E. If any premium rate in the revised premium rate schedule is greater than 200 percent of the comparable rate in the initial premium schedule, lifetime projections, as defined in Subsection B(3)(a), shall be filed for review [approval] by the commissioner every five (5) years following the end of the required period in Subsection D. For group insurance policies that meet the conditions in Subsection K, the projections required by this subsection shall be provided to the policyholder in lieu of filing with the commissioner.
- F. (1) If the commissioner has determined that the actual experience following a rate increase does not adequately match the projected experience and that the current projections under moderately adverse conditions demonstrate that incurred claims will not exceed proportions of premiums specified in Subsection C, the commissioner may require the insurer to implement any of the following:
- (a) Premium rate schedule adjustments; or
 - (b) Other measures to reduce the difference between the projected and actual experience.

Drafting Note: The terms "adequately match the projected experience" include more than a comparison between actual and projected incurred claims. Other assumptions should also be taken into consideration, including lapse rates (including mortality), interest rates, margins for moderately adverse conditions, or any other assumptions used in the pricing of the product.

It is to be expected that the actual experience will not exactly match the insurer's projections. During the period that projections are monitored as described in Subsections D and E, the commissioner should determine that there is not an adequate match if the differences in earned premiums and incurred claims are not in the same direction (both actual values higher or lower than projections) or the difference as a percentage of the projected is not of the same order.

- (2) In determining whether the actual experience adequately matches the projected experience, consideration should be given to Subsection B(3)(e), if applicable.

- G. If the majority of the policies or certificates to which the increase is applicable are eligible for the contingent benefit upon lapse, the insurer shall file:
- (1) A plan, subject to commissioner approval, for improved administration or claims processing designed to eliminate the potential for further deterioration of the policy form requiring further premium rate schedule increases, or both, or to demonstrate that appropriate administration and claims processing have been

implemented or are in effect; otherwise the commissioner may impose the condition in Subsection H of this section; and

- (2) The original anticipated lifetime loss ratio, and the premium rate schedule increase that would have been calculated according to Subsection C had the greater of the original anticipated lifetime loss ratio or fifty-eight percent (58%) been used in the calculations described in Subsection C(2)(a) and (c).

H. (1) For a rate increase filing that meets the following criteria, the commissioner shall review, for all policies included in the filing, the projected lapse rates and past lapse rates during the twelve (12) months following each increase to determine if significant adverse lapsation has occurred or is anticipated:

- (a) The rate increase is not the first rate increase requested for the specific policy form or forms;
- (b) The rate increase is not an exceptional increase; and
- (c) The majority of the policies or certificates to which the increase is applicable are eligible for the contingent benefit upon lapse.

(2) In the event significant adverse lapsation has occurred, is anticipated in the filing or is evidenced in the actual results as presented in the updated projections provided by the insurer following the requested rate increase, the commissioner may determine that a rate spiral exists. Following the determination that a rate spiral exists, the commissioner may require the insurer to offer, without underwriting, to all in force insureds subject to the rate increase the option to replace existing coverage with one or more reasonably comparable products being offered by the insurer or its affiliates.

(a) The offer shall:

- (i) Be subject to the approval of the commissioner;
- (ii) Be based on actuarially sound principles, but not be based on attained age; and
- (iii) Provide that maximum benefits under any new policy accepted by an insured shall be reduced by comparable benefits already paid under the existing policy.

(b) The insurer shall maintain the experience of all the replacement insureds separate from the experience of insureds originally issued the policy forms. In the event of a request for a rate increase on the policy form, the rate increase shall be limited to the lesser of:

- (i) The maximum rate increase determined based on the combined

experience; and

- (ii) The maximum rate increase determined based only on the experience of the insureds originally issued the form plus ten percent (10%).

- I. If the commissioner determines that the insurer has exhibited a persistent practice of filing inadequate initial premium rates for long-term care insurance, the commissioner may, in addition to the provisions of Subsection H of this section, prohibit the insurer from either of the following:

Drafting Note: States may want to consider examining their statutes to determine whether a persistent practice of filing inadequate initial premium rates would be considered a violation of the state's unfair trade practice act and subject to the penalties under that act.

- (1) Filing and marketing comparable coverage for a period of up to five (5) years;

Or

- (2) Offering all other similar coverages and limiting marketing of new applications to the products subject to recent premium rate schedule increases.

- J. Subsections A through I shall not apply to policies for which the long-term care benefits provided by the policy are incidental, as defined in Section 4B, if the policy complies with all of the following provisions:

- (1) The interest credited internally to determine cash value accumulations, including long-term care, if any, are guaranteed not to be less than the minimum guaranteed interest rate for cash value accumulations without long-term care set forth in the policy;

- (2) The portion of the policy that provides insurance benefits other than long-term care coverage meets the nonforfeiture requirements as applicable in any of the following:

- (a) [Cite state's standard nonforfeiture law similar to the NAIC's Standard Nonforfeiture Law for Life Insurance];
- (b) [Cite state's standard nonforfeiture law similar to the NAIC's Standard Nonforfeiture Law for Individual Deferred Annuities], and
- (c) [Cite state's section of the variable annuity regulation similar to Section 7 of the NAIC's Model Variable Annuity Regulation];

- (3) The policy meets the disclosure requirements of [cite appropriate sections in the state's long-term care insurance law similar to Section 61, 6J, and 6K of the NAIC's

Long-Term Care Insurance Model Act];

- (4) The portion of the policy that provides insurance benefits other than long-term care coverage meets the requirements as applicable in the following:
 - (a) Policy illustrations as required by [cite state's life insurance illustrations regulation similar to the NAIC's Life Insurance Illustrations Model Regulation];
 - (b) Disclosure requirements in [cite state's annuity disclosure regulation similar to the NAIC's Annuity Disclosure Model Regulation]; and
 - (c) Disclosure requirements in [cite state's variable annuity regulation similar to the NAIC's Model Variable Annuity Regulation].
- (5) An actuarial memorandum is filed with the insurance department that includes:
 - (a) A description of the basis on which the long-term care rates were determined;
 - (b) A description of the basis for the reserves;
 - (c) A summary of the type of policy, benefits, renewability, general marketing method, and limits on ages of issuance;
 - (d) A description and a table of each actuarial assumption used. For expenses, an insurer must include percent of premium dollars per policy and dollars per unit of benefits, if any;
 - (e) A description and a table of the anticipated policy reserves and additional reserves to be held in each future year for active lives;
 - (f) The estimated average annual premium per policy and the average issue age;
 - (g) A statement as to whether underwriting is performed at the time of application. The statement shall indicate whether underwriting is used and, if used, the statement shall include a description of the type or types of underwriting used, such as medical underwriting or functional assessment underwriting. Concerning a group policy, the statement shall indicate whether the enrollee or any dependent will be underwritten and when underwriting occurs; and
 - (h) A description of the effect of the long-term care policy provision on the required premiums, nonforfeiture values and reserves on the underlying insurance policy, both for active lives and those in long-term care claim status.

K. Subsections F and H shall not apply to group insurance policies as defined in Section [insert reference to Section 4E(1) of the NAIC Long-Term Care Insurance Model Act] where:

- (1) The policies insure 250 or more persons and the policyholder has 5,000 or more eligible employees of a single employer; or
- (2) The policyholder, and not the certificate holders, pays a material portion of the premium, which shall not be less than twenty percent (20%) of the total premium for the group in the calendar year prior to the year a rate increase is filed.

Appendix Fifteen

Nonforfeiture Benefit Requirement

Section 28. Nonforfeiture Benefit Requirement

- A. This section does not apply to life insurance policies or riders containing accelerated long-term care benefits.
- B. To comply with the requirement to offer a nonforfeiture benefit pursuant to the provisions of [insert reference to Section 8 of the NAIC Long-Term Care Insurance Model Act]:
 - (1) A policy or certificate offered with nonforfeiture benefits shall have coverage elements, eligibility, benefit triggers and benefit length that are the same as coverage to be issued without nonforfeiture benefits. The nonforfeiture benefit included in the offer shall be the benefit described in subsection E; and
 - (2) The offer shall be in writing if the nonforfeiture benefit is not otherwise described in the Outline of Coverage or other materials given to the prospective policyholder.
- C. If the offer required to be made under [insert reference to Section 8 of the NAIC Long-Term Care Insurance Model Act] is rejected, the insurer shall provide the contingent benefit upon lapse described in this Section. Even if this offer is accepted for a policy with a fixed or limited premium paying period, the contingent benefit on lapse in Subsection D(4) shall still apply.
- D. (1) After rejection of the offer required under [insert reference to Section 8 of the NAIC Long-Term Care Insurance Model Act], for individual and group policies without nonforfeiture benefits issued after the effective date of this section, the insurer shall provide a contingent benefit upon lapse.
 - (2) In the event a group policyholder elects to make the nonforfeiture benefit an option to the certificate holder, a certificate shall provide either the nonforfeiture benefit or the contingent benefit upon lapse.
 - (3) A contingent benefit on lapse shall be triggered every time an insurer

increases the premium rates to a level which results in a cumulative increase of the annual premium equal to or exceeding the percentage of the insured's initial annual premium set forth below based on the insured's issue age, and the policy or certificate lapses within 120 days of the due date of the premium so increased. Unless otherwise required, policyholders shall be notified at least thirty (30) days prior to the due date of the premium reflecting the rate increase.

Triggers for a Substantial Premium Increase

Issue Age	Percent Increase Over Initial Premium
29 and under	200%
30-34	190%
35-39	170%
40-44	150%
45-49	130%
50-54	110%
55-59	90%
60	70%
61	66%
62	62%
63	58%
64	54%
65	50%
66	48%
67	46%
68	44%

Triggers for a Substantial Premium Increase

Issue Age	Percent Increase Over Initial Premium
69	42%
70	40%
71	38%
72	36%
73	34%
74	32%
75	30%
76	28%
77	26%
78	24%

79	22%
80	20%
81	19%
82	18%
83	17%
84	16%
85	15%
86	14%
87	13%
88	12%
89	11%
90 and over	10%

- (4) A contingent benefit on lapse shall also be triggered for policies with a fixed or limited premium paying period every time an insurer increases the premium rates to a level that results in a cumulative increase of the annual premium equal to or exceeding the percentage of the insured's initial annual premium set forth below based on the insured's issue age, the policy or certificate lapses within 120 days of the due date of the premium so increased, and the ratio in Paragraph (6)(b) is forty percent (40%) or more. Unless otherwise required, policyholders shall be notified at least thirty (30) days prior to the due date of the premium reflecting the rate increase.

<u>Triggers for a Substantial Premium Increase</u>	
<u>Issue Age</u>	<u>Percent Increase Over Initial Premium</u>
Under 65	50%
65-80	30%
Over 80	10%

This provision shall be in addition to the contingent benefit provided by Paragraph (3) above and where both are triggered, the benefit provided shall be at the option of the insured.

- (5) On or before the effective date of a substantial premium increase as defined in

Paragraph (3) above, the insurer shall:

- (a) Offer to reduce policy benefits provided by the current coverage without the requirement of additional underwriting so that required premium payments are not increased;

Drafting Note: The insured's right to reduce policy benefits in the event of the premium increase does not affect any other right to elect a reduction in benefits provided under the policy.

- (b) Offer to convert the coverage to a paid-up status with a shortened benefit period in accordance with the terms of Subsection E. This option may be elected at any time during the 120-day period referenced in Subsection D(3); and
- (c) Notify the policyholder or certificate holder that a default or lapse at any time during the 120-day period referenced in Subsection D(3) shall be deemed to be the election of the offer to convert in Subparagraph (b) above unless the automatic option in Paragraph (6)(c) applies.

(6) On or before the effective date of a substantial premium increase as defined in Paragraph (4) above, the insurer shall:

- (a) Offer to reduce policy benefits provided by the current coverage without the requirement of additional underwriting so that required premium payments are not increased;

Drafting Note: The insured's right to reduce policy benefits in the event of the premium increase does not affect any other right to elect a reduction in benefits provided under the policy.

- (b) Offer to convert the coverage to a paid-up status where the amount payable for each benefit is ninety percent (90%) of the amount payable in effect immediately prior to lapse times the ratio of the number of completed months of paid premiums divided by the number of months in the premium paying period. This option may be elected at any time during the 120-day period referenced in Subsection D(4); and
- (c) Notify the policyholder or certificate holder that a default or lapse at any time during the 120-day period referenced in Subsection D(4) shall be deemed to be the election of the offer to convert in Subparagraph (b) above

if the ratio is forth percent (40%) or more.

E. Benefits continued as nonforfeiture benefits, including contingent benefits upon lapse in accordance with Subsection D(3) but not Subsection D(4), are described in this subsection:

- (1) For purposes of this subsection, attained age rating is defined as a schedule of premiums starting from the issue date which increases age at least one percent per year prior to age fifty (50), and at least three percent (3%) per year beyond age fifty (50).
- (2) For purposes of this subsection, the nonforfeiture benefit shall be of a shortened benefit period providing paid-up long-term care insurance coverage after lapse. The same benefits (amounts and frequency in effect at the time of lapse but not increased thereafter) will be payable for a qualifying claim, but the lifetime maximum dollars or days of benefits shall be determined as specified in Paragraph (3).
- (3) The standard nonforfeiture credit will be equal to 100% of the sum of all premiums paid, including the premiums paid prior to any changes in benefits. The insurer may offer additional shortened benefit period options, as long as the benefits for each duration equal or exceed the standard nonforfeiture credit for that duration. However, the minimum nonforfeiture credit shall not be less than thirty (30) times the daily nursing home benefit at the time of lapse. In either event, the calculation of the nonforfeiture credit is subject to the limitation of Subsection F.
- (4)
 - (a) The nonforfeiture benefit shall begin not later than the end of the third year following the policy or certificate issue date. The contingent benefit upon lapse shall be effective during the first three (3) years as well as thereafter.
 - (b) Notwithstanding Subparagraph (a), for a policy or certificate with attained age rating, the nonforfeiture benefit shall begin on the earlier of:
 - (i) The end of the tenth year following the policy or certificate

issue date; or

- (ii) The end of the second year following the date the policy or certificate is no longer subject to attained age rating.

- (5) Nonforfeiture credits may be used for all care and services qualifying for benefits under the terms of the policy or certificate, up to the limits specified in the policy or certificate.

- F. All benefits paid by the insurer while the policy or certificate is in premium paying status and in the paid up status will not exceed the maximum benefits which would be payable if the policy or certificate had remained in premium paying status.
- G. There shall be no difference in the minimum nonforfeiture benefits as required under this section for group and individual policies.
- H. The requirements set forth in this section shall become effective twelve (12) months after adoption of this provision and shall apply as follows:
 - (1) Except as provided in Paragraph (2) and (3) below, the provisions of this section apply to any long-term care policy issued in this state on or after the effective date of this amended regulation.
 - (2) For certificates issued on or after the effective date of this section, under a group long-term care insurance policy as defined in Section [insert reference to Section 4E(1) of the NAIC Long-Term Care Insurance Model Act], which policy was in force at the time this amended regulation became effective, the provisions of this section shall not apply.
 - (3) The last sentence in Subsection C and Subsections D(4) and D(6) shall apply to any long-term care insurance policy or certificate issued in this state after six (6) months after their adoption, except new certificates on a group policy as defined in Subsection 4E(1) one year after adoption.
- I. Premiums charged for a policy or certificate containing nonforfeiture benefits or a contingent benefit on lapse shall be subject to the loss ratio requirements of Section 19 or Section 20, whichever is applicable, treating the policy as a whole.

- J. To determine whether contingent nonforfeiture upon lapse provisions are triggered under Subsection D(3) or D(4), a replacing insurer that purchased or otherwise assumed a block or blocks of long-term care insurance policies from another insurer shall calculate the percentage increase based on the initial annual premium paid by the insured when the policy was first purchased from the original insurer.
- K. A nonforfeiture benefit for qualified long-term care insurance contracts that are level premium contracts shall be offered that meets the following requirements:
- (1) The nonforfeiture provision shall be appropriately captioned;
 - (2) The nonforfeiture provision shall provide a benefit available in the event of a default in the payment of any premiums and shall state that the amount of the benefit may be adjusted subsequent to being initially granted only as necessary to reflect changes in claims, persistency and interest as reflected in changes in rates for premium paying contracts approved by the commissioner for the same contract form; and
 - (3) The nonforfeiture provision shall provide at least one of the following:
 - a. Reduced paid-up insurance;
 - b. Extended term insurance;
 - c. Shortened benefit period; or

Other similar offerings approved by the commissioner.

Appendix Sixteen

Standards for Benefit Triggers

Section 29. Standards for Benefit Triggers

- A. A long-term care insurance policy shall condition the payment of benefits on a determination of the insured's ability to perform activities of daily living and on cognitive impairment. Eligibility for the payment of benefits shall not be more restrictive than requiring either a deficiency in the ability to perform not more than three (3) of the activities of daily living or the presence of cognitive impairment.
- B. (1) Activities of daily living shall include at least the following as defined in Section 5 and in the policy:
- (a) Bathing;
 - (b) Continence;
 - (c) Dressing;
 - (d) Eating;
 - (e) Toileting; and
 - (f) Transferring.
- (2) Insurers may use activities of daily living to trigger covered benefits in addition to those contained in Paragraph (1) as long as they are defined in the policy.
- C. An insurer may use additional provisions for the determination of when benefits are payable under a policy or certificate; however the provisions shall not restrict, and are not in lieu of, the requirements contained in Subsections A and B.
- D. For purposes of this section the determination of a deficiency shall not be more restrictive than:
- 1) Requiring the hands-on assistance of another person to perform the prescribed activities of daily living; or
 - 2) If the deficiency is due to the presence of a cognitive impairment, supervision or verbal cueing by another person is needed in order to protect the insured

or others.

- E. Assessments of activities of daily living and cognitive impairment shall be performed by licensed or certified professionals, such as physicians, nurses or social workers.
- F. Long-term care insurance policies shall include a clear description of the process for appealing and resolving benefit determinations.
- G. The requirements set forth in this section shall be effective [insert date 12 months after adoption of this provision] and shall apply as follows:
 - 1) Except as provided in Paragraph (2), the provisions of this section apply to a long-term care policy issued in this state on or after the effective date of the amended regulation.
 - 2) For certificates issued on or after the effective date of this section, under a group long-term care insurance policy as defined in Section [insert reference to Section 4E(1) of the Long-Term Care Insurance Model Act] that was in force at the time this amended regulation became effective, the provisions of this section shall not apply.

Section 30. Additional Standards for Benefit Triggers for Qualified Long-Term Care Insurance Contracts

- A. For purposes of this section the following definitions apply:
 - 1. "Qualified long-term care services" means services that meet the requirements of Section 7702(c)(1) of the Internal Revenue Code of 1986, as amended, as follows: necessary diagnostic, preventive, therapeutic, curative, treatment, mitigation and rehabilitative services, and maintenance or personal care services which are required by a chronically ill individual, and are provided pursuant to a plan of care prescribed by a licensed health care practitioner.
 - 2. (a) "Chronically ill individual" has the meaning prescribed for this term by section 7702B(c)(2) of the Internal Revenue Code of 1986, as amended. Under this provision, a chronically ill individual means any

individual who has been certified by a licensed health care practitioner as:

- i. Being unable to perform (without substantial assistance from another individual) at least two (2) activities of daily living for a period of at least ninety (90) days due to a loss of functional capacity; or
- ii. Requiring substantial supervision to protect the individual from threats to health and safety due to severe cognitive impairment.

Drafting Note: With respect to the activities of daily living (ADL) benefit trigger, HIPAA provides that tax-qualified contracts must take into account at least five of the six ADLs specified in Section 29B. This model regulation requires that eligibility for payment of benefits be no more restrictive than requiring a deficiency in the ability to perform not more than three ADLs, of the six listed. Thus, in this regard, a contract that complies with this regulation will also be tax-qualified. States do not need to alter their regulations from this model regulation with respect to the ADL trigger for tax-qualified contracts.

(b) The term "chronically ill individual" shall not include an individual otherwise meeting these requirements unless within the preceding twelve-month period a licensed health care practitioner has certified that the individual meets these requirements.

3. "Licensed health care practitioner" means a physician, as defined in Section 1861(r)(1) of the Social Security Act, a registered professional nurse, licensed social worker or other individual who meets requirements prescribed by the Secretary of the Treasury.
4. "Maintenance or personal care services" means any care the primary purpose of which is the provision of needed assistance with any of the disabilities as a result of which the individual is a chronically ill individual (including the protection from threats to health and safety due to severe cognitive impairment).

Drafting Note: Terms used in the definition of a "chronically ill individual," such as substantial assistance, loss of functional capacity, substantial supervision and severe cognitive impairment, are not defined by the Internal Revenue Code of 1986, as amended, although the meaning of the terms has been addressed

by Treasury Department and Internal Revenue Service guidance. The requirement that an insured be certified as a chronically ill individual at least once every 12 months by a licensed health care practitioner does not preclude an insurer from requiring more frequent assessments of an insured's condition in order to determine whether benefits are payable under a contract. However, states are also free to limit an insurer's ability to perform more frequent assessments without affecting the tax-qualified status of the contract.

Qualified long-term care insurance contracts that pay benefits upon a loss of functional capacity must include a provision for triggering benefits that is different from that found in Section 29 of this model regulation. The Internal Revenue Service has stated that the 90-day requirement under this benefit trigger does not establish a waiting period before which benefits may be paid or before which services may constitute qualified long-term care services.

Under Section 7702B of the Internal Revenue Code, as amended, only "licensed health care practitioners" can certify that an insured is a chronically ill individual. This term includes only physicians (within the meaning of Section 1861(r)(1) of the Social Security Act), registered professional nurses and licensed social workers.

Section 7702B does not preclude a contract from specifying a subset of "licensed health care practitioners" who can perform certifications, e.g., only physicians within the meaning of Section 1861(r)(1) of the Social Security Act that are approved by the insurance company. The Secretary of the Treasury may in regulations expand the types of individuals who are considered "licensed health care practitioners."

Section 7702B(c)(2) states that an individual will be considered chronically ill if he or she is certified by a licensed health care practitioner as having a level of disability similar (as determined under regulations prescribed by the Secretary of the Treasury in consultation with the Secretary of Health and Human Services) to the level of disability described in Section 7702B(c)(2)(A)(i) (Section 30C of this regulation). At present, the Secretary of the Treasury has prescribed no such standard. Federal tax law does not require a qualified long-term care insurance contract to include this benefit trigger in the contract. In addition, this model regulation does not mandate inclusion of this undefined benefit trigger in policies at the present time. If the Treasury Department prescribes an additional benefit trigger in the future, consideration will be given at that time to making appropriate amendments to this regulation.

- B. A qualified long-term care insurance contract shall pay only for qualified long-term care services received by a chronically ill individual provided pursuant to a plan of care prescribed by a licensed health care practitioner.

Drafting Note: The federal tax requirements for the term "qualified long-term care services" has been added to assist states in regulating qualified long-term care insurance contracts, which are defined in Section 7702B(b) of the Internal Revenue Code of 1986, as amended. The Internal Revenue Code of 1986 is subject to amendment by Congress and to interpretation by the Treasury Department, the Internal Revenue Service and the courts.

Since a qualified long-term care insurance contract can provide insurance coverage "only" for qualified long-term care services, and such services are ones required by a "chronically ill individual," benefits from such a contract can only be provided to an individual who is chronically ill. Federal tax law does not, however, prohibit the provision of coverage of some, but not all, qualified long-term care services. Thus, a contract may cover only nursing home services or limit benefits to those performed by eligible providers consistent with the requirements of federal tax law. Likewise, the federal tax law does not preclude a contract from specifying the need for hands-on assistance for purposes of determining whether the insured can perform an activity of daily living. Under this regulation, however, benefit triggers requiring greater degrees of impairment than the minimum standard established by federal tax law are permitted only to the extent otherwise consistent with this regulation and the model act.

- C. A qualified long-term care insurance contract shall condition the payment of benefits on a determination of the insured's inability to perform activities of daily living for an expected period of at least ninety (90) days due to a loss of functional capacity or to severe cognitive impairment.

Drafting Note: Section 7702B of the Internal Revenue Code of 1986, as amended, includes a provision for triggering benefits that is different from that found in Section 29 of this model regulation. The definitions used in the triggering of benefits in Section 7702B (substantial assistance, loss of functional capacity, substantial supervision and severe cognitive impairment) have been defined in guidance promulgated by the Department of the Treasury.

- D. Certifications regarding activities of daily living and cognitive impairment required pursuant to Subsection C shall be performed by the following licensed or certified professionals: physicians, registered professional nurses, licensed social workers,

or other individuals who meet requirements prescribed by the Secretary of the Treasury.

- E. Certifications required pursuant to Subsection C may be performed by a licensed health care professional at the direction of the carrier as is reasonably necessary with respect to a specific claim, except that when a licensed health care practitioner has certified that an insured is unable to perform activities of daily living for an expected period of at least ninety (90) days due to a loss of functional capacity and the insured is in claim status, the certification may not be rescinded and additional certifications may not be performed until after the expiration of the ninety-day period.
- F. Qualified long-term care insurance contracts shall include a clear description of the process for appealing and resolving disputes with respect to benefit determinations.